A PHASE 3 RANDOMIZED, DOUBLE-BLIND, PLACEBO-CONTROLLED STUDY TO INVESTIGATE THE EFFICACY AND SAFETY OF RITLECITINIB IN PEDIATRIC PARTICIPANTS 6 TO LESS THAN 12 YEARS OF AGE WITH SEVERE ALOPECIA AREATA

Study Intervention Number:	PF-06651600
Study Intervention Name:	Ritlecitinib
US IND Number:	131503
EU CT Number:	2024-515438-33-00
ClinicalTrials.gov ID:	Not Available
Pediatric Investigational Plan Number:	EMEA-002451-PIP01-18 EMEA-002451-PIP01-18-M01
Protocol Number:	B7981027
Phase:	3
Sponsor Legal Address:	Pfizer Inc. 66 Hudson Boulevard East New York, NY 10001 United States

Brief Title: A Phase 3 Study to Evaluate the Efficacy and Safety of Ritlecitinib in Children 6 to <12 Years of Age With Severe Alopecia Areata

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Document History

Document	**Version Date**
Protocol Amendment 4	**07 October 2025**
Protocol Amendment 3	10 April 2025
Protocol Amendment 2	06 January 2025
Protocol Amendment 1	03 September 2024
Original Protocol	18 June 2024

This amendment incorporates all revisions to date, including amendments made at the request of country health authorities and IRBs/ECs and any protocol administrative change letter(s).

Protocol Amendment Summary of Changes Table

Amendment 4 (07 October 2025)

Overall Rationale for the Amendment: Protocol modifications have been implemented to revise the estimated total blood volume and provide clarifications to eligibility criteria requests from Regulatory Authorities (EU).

Description of Change	**Brief Rationale**	**Section # and Name**
Substantial Modification(s)		
Applicable to All Countries Corrected the estimated total blood volume sampling collection from approximately 50 ml to approximately 61 ml. There is no change in the number of sample collections per SoA.	Total blood volume revised based on confirmation from the central lab vendor.	Section 8.1. Administrative Procedures
Added Exclusion Criterion #15 Hypersensitivity to the active substance(s) or to any of the excipients of the investigational product.	To address a query from Regulatory Authorities (EU) and add a specific exclusion criterion regarding hypersensitivity to any component of the study drug.	Section 5.2. Exclusion Criteria
Applicable to Countries in EU **Added Inclusion Criterion #5:**	To address a query from Regulatory Authorities (EU) and	Section 5.1. Inclusion Criteria

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For sites in EU member countries (Czech Republic, France, Italy, Poland, Spain): Minimum body weight of ≥17 kg at the time of the screening visit.	specify a minimum body weight for inclusion in the trial.	Section 10.10.1. European Union
Specific to China Updated the estimated total blood volume sampling collection from approximately 50 ml to approximately 65 ml, with no change in the number of sample collections per schedule of activities.	Total blood volume revised based on confirmation from central lab vendor and to be compliant with country-specific requirements of increased frequency of Hepatitis B virus testing every 3 months and additional blood sampling collection for HIV testing.	Section 10.10.4. China
Non-Substantial Modification(s)		
Applicable for all countries Removed the text on non-reporting of screen failure data on the CRF.	Screen failure data is required to be reported on the CRF to ensure transparent reporting of screen failure participants to meet the CONSORT publishing requirements and to respond to queries from regulatory authorities.	Section 5.4. Screen Failures
Applicable to all countries Corrected the value of proportionality constant (k) of the Modified Schwartz equation for male children <12 years of age in the protocol.	Updated equation with the correct proportionality constant value.	Section 10.7.2.1. Children (2 Years to <12 Years) — (Modified) Schwartz Equation
Applicable to all countries Provided additional guidance regarding acceptability and palatability assessment at Week 18.	To provide further clarification regarding administration of acceptability and palatability assessment at Week 18 for participants who discontinued early from the study.	Section 1.3. Schedule of Activities

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Applicable to all countries Incorporated protocol template updates	To align with current Pfizer protocol template and requirements	Section 8.4.1.1. Reporting SAEs to Pfizer Safety Section 8.4.8. Protocol-Specified Adverse Events of Special Interest Section 8.4.8.1. Lack of Efficacy Section 10.1.7. Data Quality Assurance Section 10.3.3. Recording/Reporting and Follow Up of AEs and/or SAEs During the Active Collection Period Section 10.10.1 European Union
Specific to China Added the information regarding human genetic resource samples to meet regulatory requirements**.**	As required by the Chinese regulatory agency, the vendor in China that will conduct testing and analysis of human genetic resource samples or disposal of remaining samples needs to be listed.	Section 10.10. Appendix 10: Country-Specific Requirements Section 10.10.4. China
Specific to China Clarified that a worksheet provided by Pfizer for QTcF calculation could be used if the local ECG system does not automatically measure QTcF.	To clarify for sites unable to perform automated QTcF calculation on their electrocardiogram (ECG) system, that a worksheet provided by Pfizer can be used to manually calculate QTcF to meet the protocol requirements for ECG testing.	Section 10.10. Appendix 10: Country-Specific Requirements Section 10.10.4. China

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Specific to Czech Republic Added that HBV-DNA testing will be required at screening and subsequently once every 3 months till Week 24/End of Treatment/Discontinuation Visit for those participants enrolled at sites in the Czech Republic who are HBsAg negative, HBsAb positive and HBcAb positive.	As requested by the Czech Republic Regulatory Agency.	Section 1.3 Schedule of Activities Section 5.2 Exclusion criteria #11 Section 10.2 Appendix 2: Clinical Laboratory Tests Section 10.10.5.1 Czech Republic-Specific Requirements for HBV-DNA Testing
Specific to Czech Republic Added that HCV-RNA testing will be required at screening visit and subsequently once every 3 months till Week 24/End of Treatment/Discontinuation Visit for those participants enrolled at sites in the Czech Republic who are positive for HCVAb and negative for HCV-RNA at screening**.**	As requested by the Czech Republic Regulatory Agency.	Section 1.3. Schedule of Activities Section 5.2. Exclusion criteria #11 Section 10.2. Appendix 2: Clinical Laboratory Tests Section 10.10.5.2. Czech Republic-Specific Requirements for HCV-RNA Testing
Specific to Czech Republic Added details on requiring a review by tuberculosis or infectious disease specialist for participants with specific current and prior TB test results.	As requested by the Czech Republic Regulatory Agency.	Section 5.2. Exclusion Criteria #11 Section 10.10.5.3. Czech Republic-Specific Requirements for TB at Screening

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1. PROTOCOL SUMMARY

1.1. Synopsis

Protocol Title: A Phase 3 Randomized, Double-Blind, Placebo-Controlled Study to Investigate the Efficacy and Safety of Ritlecitinib in Pediatric Participants 6 to less than 12 Years of Age With Severe Alopecia Areata

Brief Title: A Phase 3 Study to Evaluate the Efficacy and Safety of Ritlecitinib in Children 6 to <12 Years of Age With Severe Alopecia Areata

Regulatory Agency Identification Numbers:

United States (US) Investigational New Drug (IND) Number:	131503
European Union (EU) Clinical Trial (CT) Number:	EU CT number will be provided prior to protocol finalization and will be added
ClinicalTrials.gov Identification (ID):	Not Available
Pediatric Investigational Plan Number:	EMEA-002451-PIP01-18 EMEA-002451-PIP01-18-M01
Protocol Number:	B7981027
Phase:	3

Rationale:

Ritlecitinib was approved for the treatment of severe alopecia areata (AA) in adults and adolescents 12 years and older in the US, EU and other countries following an evaluation of positive benefit/risk in the indicated population. However, there are currently no approved treatment options for AA in children 6 to <12 years of age. Study B7981027 is being conducted as a part of clinical development strategy to assess efficacy and safety of ritlecitinib in pediatric participants 6 to <12 years of age with severe AA. Study B7981027 is a part of EU Pediatric Investigation Plan (PIP) and is a post marketing requirement in the US.

Objectives and Endpoints:

Objectives	Endpoints
Primary:	**Primary:**
For US and Countries Following US Analysis Plan	
• To evaluate the efficacy of ritlecitinib compared to placebo in pediatric participants with severe AA on regrowth of lost scalp hair.	• Response based on achieving an absolute Severity of Alopecia Tool (SALT) score ≤20 at Week 24.
For EU/United Kingdom (UK) and Countries Following EU/UK Analysis Plan	
• To evaluate the efficacy of ritlecitinib compared to placebo in pediatric participants with AA on regrowth of lost scalp hair.	• Response based on achieving an absolute SALT score ≤10 at Week 24.

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Objectives	Endpoints
Key Secondary:	**Key Secondary:**
For EU/UK and Countries Following EU/UK Analysis Plan	
• To evaluate the effect of ritlecitinib on patient centered outcomes (Patient's Global Impression of Change [PGI-C]).	• PGI-C response defined as a score of "moderately improved" or "greatly improved" at Week 24.
Secondary	**Secondary**
For All Countries	
• To further evaluate the efficacy of ritlecitinib compared to placebo in pediatric participants with AA on regrowth of lost scalp hair at all visits (except for that included as the primary endpoint).	• Change from baseline (CFB) in SALT score at all visits. • Response based on achieving absolute SALT score ≤10 at all visits (except for that included as the primary endpoint). • Response based on achieving absolute SALT score ≤20 at all visits (except for that included as the primary endpoint).
• To evaluate the effect of ritlecitinib on patient centered outcomes, measured by PGI-C response at all visits (except for that included as a key secondary endpoint).	• PGI-C response at all visits (except for that included as a key secondary endpoint).
• To evaluate the effect of ritlecitinib on Alopecia Areata Patient Priority Outcomes (AAPPO) scales and psychological/social impairment proxy reported (Patient-Reported Outcomes Measurement Information System [PROMIS], Behavior Rating Inventory of Executive Function®, Second Edition [BRIEF®2]), and participant reported (modified Children's Dermatology Life Quality Index [CDLQI]) at all visits.	• Response based on improvement from baseline for each AAPPO Item (11 endpoints).[a] • CFB in AAPPO activity limitation and emotional symptoms scores at all visits. • CFB in PROMIS Parent Proxy Depressive Symptoms T-score at all visits. • CFB in PROMIS Parent Proxy Anxiety Symptoms T-score at all visits. • CFB in BRIEF®2 T-scores for the 3 index scores (Behavior Regulation Index [BRI], Emotional Regulation Index [ERI], Cognitive Regulation Index [CRI]) at all visits. • CFB in modified CDLQI total score at all visits.
• To evaluate the effect of ritlecitinib versus placebo on achieving eyebrow assessment (EBA) response.	• Response based on achieving at least 2 grade improvement or a score of 3 in EBA score at all visits in participants with an abnormal EBA at baseline.*
• To evaluate the effect of ritlecitinib versus placebo on achieving eyelash assessment (ELA) response.	• Response based on achieving at least 2 grade improvement or a score of 3 in ELA score at all visits in participants with an abnormal ELA at baseline.**
• To characterize the pharmacokinetics (PK) of ritlecitinib in children with AA 6 to <12 years of age.	• Plasma concentration of ritlecitinib at 1 hr (±15 min) and 3 hr (±30 min) post-dose at Week 4 or Week 8.

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Objectives	Endpoints
• To evaluate safety and tolerability of ritlecitinib over time.	• Incidence of treatment-emergent adverse events (AEs) (including audiological and neurological treatment-emergent AEs).*** • Incidence of serious AEs (SAEs) and AEs leading to permanent discontinuation from the study.
• To evaluate acceptability and palatability of the age-appropriate formulation.	• Acceptability and palatability assessment.

a. For each of the 11 AAPPO items, among participants whose item-specific score at baseline is ≥2, a response is defined as having a score of 0 (No hair loss) or 1 (A little hair loss) for Item 1 to Item 4, a response is defined as having a score of 0 (Never) or 1 (Rarely) for Item 5 to Item 8, a response is defined as having a score of 0 (Not at all) or 1 (A little) for Item 9 to Item 11.
* Abnormal EBA at baseline is defined as an EBA score of 0, 1 or 2.
** Abnormal ELA at baseline is defined as an ELA score of 0, 1 or 2.
*** Audiological assessment includes updated audiological history, otoscopic examination, air pure tone audiometry at Week 24/Month 6. Additional audiometry assessments will be performed if clinically indicated.

Overall Design:

Study B7981027 will be conducted as a Phase 3 randomized, double-blind, placebo-controlled study investigating the safety and efficacy of ritlecitinib in pediatric participants (6 to <12 years of age) with severe AA (50% or greater scalp hair loss due to AA [ie, a SALT score of 50 or greater]).

This study will have 3 treatment arms, including 2 ritlecitinib dosage levels (50 mg and 30 mg) and 1 placebo arm.

The participants will be assessed for study eligibility at the screening visit after assent is obtained (as applicable) from the participant and informed consent is obtained through their parents or legal guardians. Participants will complete screening, be randomized then receive 1 of the 2 dosage levels of ritlecitinib or placebo, to be taken orally for a duration of 24 weeks. Participants who are withdrawn early from study intervention for any reason will remain in the study and continue the regularly scheduled visits through Week 24 and follow-up visit.

Approximately 225 participants will be randomized. A stratified randomization will be used for Alopecia Totalis/Alopecia Universalis (AT/AU and non-AT/AU).

Safety evaluations will include age-appropriate auditory screening and neurological assessments performed during the physical examinations. Audiological evaluation includes otoscopic examination, air and bone pure tone audiometry, speech audiometry and immittance audiometry at screening and Week 24/Month 6. Neuro-psychological and cognitive development evaluations include BRIEF®2 (at all visits) and Wechsler Intelligence Scale for Children® Fifth Edition (WISC-V) (at baseline) assessments, respectively. Suicidal Ideation and Behavior will be assessed throughout the study using the Columbia-Suicide Severity Rating Scale (C-SSRS) questionnaire.

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PK parameters will be evaluated using sparse sampling.

Number of Participants:

Approximately 225 participants will be enrolled in the study. At least 30% of total study population will be recruited from Europe, as required by the Pediatric Committee (PDCO).

Note: "Enrolled" means a participant's assent and/or their legally authorized representative's agreement to participate in a clinical study following completion of the informed consent/assent process and randomization to study intervention.

Study Population:

Key inclusion and exclusion criteria are listed below:

Inclusion Criteria

Participants must meet the following key inclusion criteria to be eligible for enrollment into the study:

- Participants who are 6 to <12 years old at the time of the screening visit.
- A diagnosis of AA (including AT and AU) with at least 50% scalp hair loss due to AA (ie, a SALT score of ≥50) at both the screening and baseline visits, without evidence of terminal hair regrowth within the previous 12 months.

For study participants in the EU/UK only:

- History of clinical response failure to AA treatment (such as topical, off-label pharmacologic, or hairpiece prosthetics)

Exclusion Criteria

Participants with the following characteristics/conditions will be excluded: other (non-AA) types of alopecia; any present or history of malignancies or lymphoproliferative disorders; evidence of untreated or inadequately treated active or latent Mycobacterium tuberculosis (TB) infection; history (one or more episodes) of severe or serious cytomegalovirus (CMV) infection, herpes zoster (shingles) or disseminated herpes simplex; history of any infection requiring hospitalization, parenteral antimicrobial therapy, or as otherwise judged clinically significant within prior 3 months; infection with hepatitis B virus (HBV) or hepatitis C virus (HCV).

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Study Arms and Duration:

The participants will be randomized into 3 treatment arms in a 1:1:1 ratio stratified by AT/AU and non-AT/AU.

Study Arms			
Arm Title	Arm 1: Ritlecitinib 50 mg	Arm 2: Ritlecitinib 30 mg	Arm 3: Placebo
Arm Description	Participants will receive 1 ritlecitinib 50 mg capsule once daily (QD) and 1 placebo 30 mg capsule QD orally for 24 weeks.	Participants will receive 1 ritlecitinib 30 mg capsule QD and 1 placebo 50 mg capsule QD orally for 24 weeks.	Participants will receive 1 placebo 50 mg capsule QD and 1 placebo 30 mg capsule QD orally for 24 weeks.

Ethical Considerations:

The results of previous studies of ritlecitinib in adult and adolescent populations that demonstrated clinical efficacy with acceptable safety and tolerability of the chronic 50 mg QD dose support the investigation of ritlecitinib in younger pediatric participants (6 to <12 years of age) with AA, and there is a favorable benefit-risk profile to support the rationale for this study. Considering the measures to minimize risk to participants, the potential risks associated with ritlecitinib are justified by the anticipated benefits that may be afforded to pediatric participants with severe AA. The doses to be studied have been carefully selected based on expected exposures. Based on the information from the population PK analysis, the higher dose of 50 mg is equivalent to the exposure of the approved ritlecitinib 50 mg dose in adults and adolescents and the lower dose of 30 mg is equivalent to the ritlecitinib 30 mg dose exposure in adults and adolescents.

As there are no currently approved therapies for severe AA in the age group of 6 to <12 years, this study will use placebo as the comparator. This study will provide information on efficacy and safety of ritlecitinib in 6 to <12-year-old population. Participants who successfully complete Study B7981027 may be eligible to receive ritlecitinib in an extension Study B7981028.

Based on the understanding of ritlecitinib's safety profile, the important potential risks for ritlecitinib include viral reactivation, serious and opportunistic infections, malignancy and thromboembolism. Identified risks that could have adverse outcomes include dermatologic effects (rash, acne, urticaria), decreased lymphocyte counts and decreased platelet counts. In pre-clinical adult dog studies, reversible central nervous system (CNS) findings of axonal dystrophy (with reversible hearing loss in some animals) were reported at exposures higher than those used clinically; the clinical data to date do not demonstrate evidence suggestive of neurotoxicity with ritlecitinib treatment. Appropriate mitigation strategies have been defined in this protocol to identify and manage any safety events.

Participants will be expected to commit time and may experience some discomfort while undergoing study assessments. In addition, participants of childbearing potential must agree to use appropriate contraception methods, and all participants must avoid vaccination with live attenuated replication-competent vaccines.

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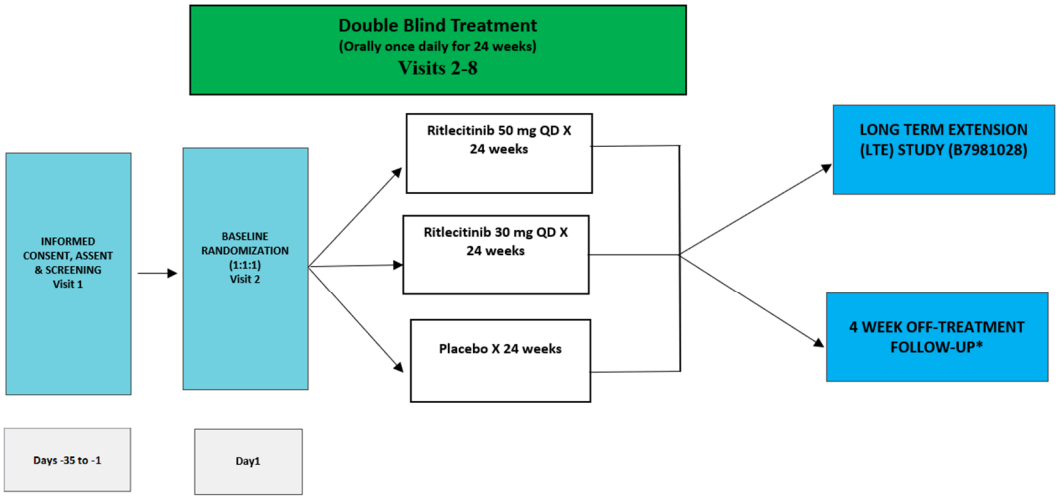
1.2. Schema

Figure 1. Study Schematic



*If the participant is ineligible for LTE study or had early study discontinuation

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1.3. Schedule of Activities

The SoA table provides an overview of the protocol visits and procedures. Refer to the STUDY ASSESSMENTS AND PROCEDURES section of the protocol for detailed information on each procedure and assessment required for compliance with the protocol.

The investigator may schedule visits (unplanned visits) in addition to those listed in the SoA table, in order to conduct evaluations or assessments required to protect the well-being of the participant.

Table 1. Study Schedule of Assessment

Protocol Activity	Screening [a]	Treatment Period						EOT[b]	EOS[c]	Notes
Visit Identifier Abbreviations used in this table may be found in Appendix 17	**1**	**2**	**3**	**4**	**5**	**6**	**7**	**8**	**9**	
Visit Day	**Day -35 to Day -1**	**Day 1 of Treatment (Baseline Randomization)**	**Day 15**	**Day 29**	**Day 57**	**Day 85**	**Day 127**	**Day 169**	**4-week FU**	• Day relative to start of study intervention (Day 1). • Follow-up contact may occur via telephone contact and must occur 28 to 35 days after administration of the final dose of study intervention.
Visit Week	**NA**		**Week 2**	**Week 4**	**Week 8**	**Week 12**	**Week 18**	**Week 24**	**Week 28**	
Visit Window	**NA**	**NA**	**±3 days**		**±7 days**					• See Section 8.4.3 for follow-up AE and SAE assessments.
ELIGIBILITY										
Informed consent/assent	X									• Informed consent and participant assent should be obtained prior to undergoing any study-specific procedures. • See Section 10.1.3 for additional information.
Inclusion/exclusion criteria	X	X								
Demographics & Medical history	X									

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Table 1. Study Schedule of Assessment

Protocol Activity	Screening [a]	Treatment Period						EOT[b]	EOS[c]	Notes
Visit Identifier Abbreviations used in this table may be found in Appendix 17	**1**	**2**	**3**	**4**	**5**	**6**	**7**	**8**	**9**	
Registration/randomization		X								• At registration, the participant enrollment number and dose level allocation are assigned.
PHYSICAL EXAMINATION, VITAL SIGNS										
Complete physical examination	X							X		• Physical examinations to be completed before administration of study intervention. • Complete physical examination consists of general appearance, skin, head, eyes, ears, nose, throat, mouth, lungs, heart, abdomen, neurologic function, extremities, back and lymph nodes. In addition, dermatological full body exam must be performed. • See Section 8.3.1 for additional information
Brief/targeted physical examination		X		X	X	X	X			• Targeted physical examination consists of skin, heart, lungs, abdomen, neurologic function, and examination of body systems where there are symptom complaints by the participant.
Weight, Height	X	X						X		• See Section 8.3.1 for additional information.
Vital signs	X	X	X	X	X	X	X	X	X	• Includes heart rate and blood pressure assessments. • See Section 8.3.2 for additional information.

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Table 1. Study Schedule of Assessment

Protocol Activity	Screening [a]	Treatment Period						EOT[b]	EOS[c]	Notes
Visit Identifier Abbreviations used in this table may be found in Appendix 17	**1**	**2**	**3**	**4**	**5**	**6**	**7**	**8**	**9**	
Tanner staging		X						X		• If EOT visit occurs before Week 24, Tanner staging should be performed only at Week 24 visit.
Audiologic examination	X							X		• Audiologic evaluation includes audiological history, otoscopic examination, air and bone pure tone audiometry, speech audiometry, and immittance audiometry. All audiological results must be available prior to Day 1 to assess eligibility. At subsequent visit (Week 24/Month 6), updated audiological history, otoscopic examination, and air pure tone audiometry will be performed; based upon results, additional audiometry assessments may be required. If EOT visit occurs before Week 24, audiological examination should be performed only at Week 24 visit. • Pediatric Audiometry Study Guide will be provided to the sites to perform audiological evaluations. All procedures must be performed as per the Pediatric Audiometry Study Guide. See Section 8.3.4 for additional information.
ECG	X			X						• See Section 8.3.5 for additional information.
LABORATORY ASSESSMENTS										• See Section 8.3.6 for additional information. • See Appendix 2 for a list of Clinical Laboratory tests to be done.

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Table 1. Study Schedule of Assessment

Protocol Activity	Screening [a]	Treatment Period						EOT[b]	EOS[c]	Notes
Visit Identifier Abbreviations used in this table may be found in Appendix 17	**1**	**2**	**3**	**4**	**5**	**6**	**7**	**8**	**9**	
										• For laboratory collection volumes, see the laboratory manual.
Clinical Laboratory Tests										
Hematology	X			X		X		X		
Serum chemistry	X			X		X		X		
QFT, hepatitis B and C, HIV, VZV IgG Ab	X									• VZV IgG Ab needed only if there is no documented evidence of 2 doses of varicella vaccine or prior exposure to VZV (positive VZV IgG Ab test result).
HBV-DNA testing	X					X		X		• Refer to Appendix 10, Section 10.10.5.1 for Czech Republic-specific requirements for HBV-DNA testing.
HCV-RNA testing	X					X		X		• Refer to Appendix 10, Section 10.10.5.2 for Czech Republic-specific requirements for HCV-RNA testing.
Collection of baseline sample for potential viral screen		X								
Urine dipstick	X			X		X		X		• Urinalysis will be performed by dipstick in the clinic and the result documented at the site level (source document); if an abnormal result is observed or the participant is symptomatic, the sample will be sent to the central laboratory. • See Appendix 2, Table 4 - Protocol-Required Laboratory Assessments for more details.

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Table 1. Study Schedule of Assessment

Protocol Activity	Screening [a]	Treatment Period						EOT[b]	EOS[c]	Notes
Visit Identifier Abbreviations used in this table may be found in Appendix 17	**1**	**2**	**3**	**4**	**5**	**6**	**7**	**8**	**9**	
Urine Pregnancy test (+ monthly 'at-home' tests between study visits) for all females ≥8 years of age only[d]	X	X	X	X	X	X	X	X	X	• Urine pregnancy test with a sensitivity of at least 25 mIU/mL will be performed at screening and throughout the study at timepoints defined in the protocol SoA. • Pregnancy screening test will be performed for all females ≥8 years of age. If, in the opinion of the investigator, pregnancy testing is not necessary for a participant above the age of 8 years, then the investigator should document why pregnancy screening was not necessary (eg, based on their level of sexual maturity). In such cases, menarchal status should be recorded in the Medical History CRF. This will also include a monthly phone call check on results of pregnancy testing between study visits.
PHARMACOKINETICS										
PK sampling				X						• Blood samples for PK will be collected at 1 hr (±15 min) and 3 hr (±30 min) post-dose at Week 4. If not collected at Week 4 PK samples must be collected at Week 8. • The dosing method should be recorded on the day of PK sampling.
STUDY PROCEDURES and ASSESSMENTS										• See Section 8 for additional information.
Study Intervention										• See Section 6 for additional information.

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Table 1. Study Schedule of Assessment

Protocol Activity	Screening [a]	Treatment Period						EOT[b]	EOS[c]	Notes
Visit Identifier Abbreviations used in this table may be found in Appendix 17	**1**	**2**	**3**	**4**	**5**	**6**	**7**	**8**	**9**	
Study intervention dispensing		X		X	X	X	X			
Study intervention administration		X	X	X	X	X	X			• See Section 6.1 for additional information.
Study intervention accountability			X	X	X	X	X	X		• See Section 6.2 for additional information.
SALT Score	X	X	X	X	X	X	X	X		
Acceptability and Palatability assessment Questionnaire			X				X			• See Appendix 12 for additional information. • If a participant is discontinued prior to Week 18 and remains in the study for regularly scheduled visits, acceptability and palatability assessment questionnaire should not be administered at Week 18.
EBA assessment	X	X	X	X	X	X	X	X		
ELA assessment	X	X	X	X	X	X	X	X		
C-SSRS suicidal ideation and behavior risk assessment	X	X	X	X	X	X	X	X		
PROMIS® Parent Proxy Anxiety symptoms	X	X	X	X	X	X	X	X		• See Section 8.2.3.3 for additional information.
PROMIS® Parent Proxy Depressive symptoms	X	X	X	X	X	X	X	X		• See Section 8.2.3.3 for additional information.
BRIEF®2	X	X	X	X	X	X	X	X		• BRIEF®2 will be used only in countries where the BRIEF®2 questionnaire is available. • See Section 8.2.3.4 for additional information.
Modified CDLQI	X	X	X	X	X	X	X	X		• See Section 8.2.3.5 for additional information.

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Table 1. Study Schedule of Assessment

Protocol Activity	Screening [a]	Treatment Period						EOT[b]	EOS[c]	Notes
Visit Identifier Abbreviations used in this table may be found in Appendix 17	**1**	**2**	**3**	**4**	**5**	**6**	**7**	**8**	**9**	
WISC-V		X								• See Section 8.2.3.6 for additional information.
PGI-C			X	X	X	X	X	X		
AAPPO	X	X	X	X	X	X	X	X		• AAPPO will be used only in countries where the AAPPO questionnaire is available.
Contraception check (women of childbearing potential only)	X	X	X	X	X	X	X	X	X	• This will also include a monthly phone call check on contraception between study visits.
CONCOMITANT TREATMENT(S)										
Prior/concomitant treatment(s)	X	X	X	X	X	X	X	X		• See Section 6.9 for additional information
Serious and nonserious AE monitoring	X	X	X	X	X	X	X	X	X	• See Section 8.4 for additional information.
BIOMARKER ASSESSMENTS										
IP-10 (CXCL-10)		X						X		• See Section 8.7.1.1 for additional information. • To be collected before dose. • If EOT visit occurs before Week 24, IP-10 should be performed only at EOT visit.
FACS-TBNK		X						X		• See Section 8.7.1.2 for additional information. If EOT visit occurs before Week 24, FACS-TBNK should be performed only at EOT visit.
Retained research sample for genetics: Prep D1.5		X								• If not collected on the designated collection day, collect at the next available time point when biospecimen are being collected in conjunction with a participant visit.

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Table 1. Study Schedule of Assessment

Protocol Activity	Screening [a]	Treatment Period						EOT[b]	EOS[c]	Notes
Visit Identifier Abbreviations used in this table may be found in Appendix 17	**1**	**2**	**3**	**4**	**5**	**6**	**7**	**8**	**9**	
										• See Section 8.6.2 for additional information.

a. All screening procedures are to be done ≤35 days before the first dose.
b. Participants who are withdrawn early from study intervention for any reasons will remain in the study and continue the regularly scheduled visits through Week 24 and follow-up visit.
c. For participants who enroll in Study B7981028 within 30 days of last dose of Study B7981027 and first visit of Study B7981028, EOS visit is not required. For participants who are ineligible to participate in Study B7981028 and for those Study B7981027 participants, for whom the gap between the last dose of Study B7981027 and first visit of Study B7981028 is >30 days, EOS visit procedures are performed 28 to 35 days after administration of the final dose of study intervention.
d. If a female participant is <8 years of age at the time of screening and turns 8 years during the study, the investigator should perform urine pregnancy tests including monthly "at-home" urine pregnancy tests for such participants at that visit and at all the remaining study visits. The site/investigator should carefully monitor participants' age during the study.

2. INTRODUCTION

Ritlecitinib (PF-06651600) is an oral JAK3 and TEC kinase inhibitor that is currently being developed as a treatment for AA and vitiligo.

2.1. Study Rationale

Ritlecitinib was approved for the treatment of severe AA in adults and adolescents 12 years of age and older in the US, EU and other countries following an evaluation of positive benefit/risk in the indicated population. Studies in adults and adolescents demonstrated clinical efficacy and acceptable tolerability and safety of ritlecitinib in participants with severe AA, and the approved dose in both age groups is 50 mg QD.

Considering the high unmet need to treat AA in children, the rationale for this study is to evaluate the efficacy and safety of ritlecitinib in children aged 6 to <12 years, at the dose expected to result in exposures equivalent to those of the approved 50 mg dose in adults and adolescents. An additional lower dose will also be evaluated. The systemic exposure data generated in adult and adolescent participants (Studies B7981015, B7931005, B7981032) were used to extrapolate PK characteristics by allometric scaling in a population PK model and to derive the dose providing equivalent exposures for children 6 to <12 years old. The PK of the derived dose was confirmed in the pediatric PK and PD Study B7981031 in the age group of 6 to <12 years and used in conjunction with the data in adolescents and adults to confirm the dose recommendations for this Phase 3 pediatric Study B7981027.
Study B7981027 is being conducted as a part of clinical development strategy to assess efficacy and safety of ritlecitinib in pediatric participants 6 to <12 years of age with severe AA. Study B7981027 is a part of EU PIP and is a post marketing requirement in the US.

2.2. Background

AA is a chronic relapsing T-cell mediated autoimmune disorder characterized by non-scarring hair loss affecting children and adults across all ages, races, and sexes [1,2]. AA is associated with other immune diseases including asthma, allergic rhinitis, atopic dermatitis, and autoimmune diseases such as thyroiditis and vitiligo [2].

It is estimated that AA affects as many as 6 to 7 million individuals in the US [2] and 160 million worldwide [3]. Depending on ethnic background and area of the world, the prevalence of AA is 0.1 to 0.2%, with a calculated lifetime risk of 2% [1]. AA is a disease with significant pediatric prevalence. In the US, the overall prevalence of pediatric AA is 0.11% and the overall incidence rate of AA in children between 2009 and 2020 is 13.6 cases per 100 000 person-years. In a cohort of 5801 children with alopecia areata in the US, prevalence of AA by age peaked at age 9 years and 12 years for females and males, respectively with higher prevalence found among older children. The incidence rate of AA by age was normally distributed and peaked at age 6 years. Further, it was observed that the incidence rates of AA was much higher in pediatric patients who self-identified by race and ethnicity as Hispanic and Asian compared with those who identified as White [4].

Clinical presentation of AA can be limited to small, circular patches of scalp hair loss (patchy hair loss, alopecia focalis), involve complete loss of hair on the scalp (AT), or total loss of

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hair on the scalp and body (AU). AA involving 50% or greater scalp hair loss, including AT and AU, is considered an advanced form of alopecia, according to the US National Alopecia Areata Foundation [3]. Patchy alopecia is the most common form of AA which may develop into the more extensive and often treatment-resistant forms of AA, especially with earlier age of onset [5].

The spontaneous remission rates for AA patients presenting with <50% scalp hair loss (patchy hair loss, alopecia focalis) are 30-50% within the first 6-12 months of disease onset, and 66% of the patients will show complete regrowth of hair within 5 years [6]. However, the likelihood of complete regrowth spontaneously with AT or AU is less than 10% [7]. Further, there is evidence that AA can become refractory to JAK inhibition if it has been present for a substantial period of time. Specifically, a recent case series demonstrated that AA patients whose current episode of AT or AU had lasted more than 10 years were highly non-responsive to off-label tofacitinib treatment in comparison with patients with shorter duration [8].

Pediatric AA has been associated with a guarded long-term prognosis, with patients having multiple relapses and progression to AT or AU. Up to 50% of patients show regrowth within one year without treatment, but the course is unpredictable. Relapse is common even after apparently successful regrowth after treatment [9].

Young age at first presentation is associated with a poor prognosis. Other poor prognostic factors include extensive hair loss at presentation, including AT and AU, ophiasis pattern of hair loss, long duration of hair loss, atopy, positive family history, other autoimmune disease and nail involvement [10].

Pediatric AA can affect the QoL of patients and their family members. Depression, anxiety, and panic disorders are often observed in patients with AA and the coping mechanisms of AA patients mirror those of grief and bereavement [11-15]. A substantial body of evidence demonstrates a widespread impact of AA on the psychological health of both adult and pediatric patients with AA, including impairment in self-esteem, increased incidence of anxiety and depressive disorders and other psychological conditions [11-15], problems with social relations [16], decreased HRQoL and QoL, as well as the QoL of their families [17-25]. In a US study, children with AA had more psychological problems than those without AA. Specifically, those with AA exhibited more anxiety, depression, tendencies to withdraw, aggression, and delinquency. In addition, children with AA were more likely to exhibit somatic problems as well as problems in social relations and in attention span. Girls with AA seem to have been affected more in dimensions of total problems, anxiety/depression, and internalizing/externalizing syndromes.

Therapeutic options for children in 6 to <12 years with AA are limited. As of the writing of this protocol, there are no systemic treatments approved by Health Authorities for the treatment of AA in pediatric patients aged 6 to <12 years. While a majority of children with limited AA spontaneously recover, the variability of the disease course and unpredictable response to therapy make AA challenging to treat. Given the finding that complete scalp hair loss with duration >10 years in adults is less likely to respond to treatment, pursuing

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treatment, even if only intermittently, in adolescents or even younger patients with stable, severe AA may prevent future irreversible hair loss [26].

Ritlecitinib is the only systemic therapy with demonstrated efficacy and safety approved in adults and adolescents (12 years of age and older) with severe AA. However, its efficacy and safety in younger children with AA (6-11 years of age) have not been evaluated. Numerous other therapies have been reported, but the evidence is mostly weak [27-31]. Treatment guidelines include recommendations for topical (corticosteroids, immunotherapy and minoxidil), intralesional corticosteroids or systemic therapies (corticosteroids, corticosteroid-sparing agents such as cyclosporin and methotrexate). In children, intralesional therapies are not recommended [26,32]. The short- and long-term side effects of systemic corticosteroids are well known and potentially severe, and thus their use in children is not supported by guidelines [28]. There is only limited evidence from uncontrolled trials for the use of older immunosuppressive drugs, compounded by concerns of severe and potentially irreversible adverse effects, and therefore these therapies are not recommended for use in children [10,28,33-36].

Rationale for Ritlecitinib

CD8+ T cells, NK cells, and mast cells are involved in the pathogenesis of AA. The possible inflammatory pathways in AA include cytokines from the TH1 axis, including IFNα, IFNγ, and the IFNγ–induced protein, IP-10 [2,37]. Mouse models have shown that IL-2 and IL-15 play a role in the initiation of autoreactive CD8+ cells that attack hair follicles [5]. Several JAK3-dependent cytokines such as IL-15, IL-7, IL-9, and IL-21 have been implicated in the pathophysiology of AA. BTK, IL-12 and IL-23 may also play a role in the pathogenesis of AA [6].

Ritlecitinib is an orally bioavailable small molecule that irreversibly inhibits JAK3 with selectivity over the other 3 JAK isoforms (JAK1, JAK2 and TYK2). Ritlecitinib also irreversibly inhibits the TEC kinase family proteins (BTK, BMX, ITK, TEC, and TXK) with high selectivity over the broader kinome. JAK3 inhibition will lead to modulation of γ-common chain cytokine pathways, such as IL-7, IL-9, IL-15, and IL-21, some of which have been implicated in the pathophysiology of AA. Additionally, ritlecitinib inhibits the cytotoxic function of CD8+ T cells and NK cells which have also been implicated in the pathogenic process of AA.

Ritlecitinib has been investigated in adults (Studies B7981005, B7981015, B7981032 and B7981037) and adolescents 12 years of age and older (Studies B7981015 and B7981032). The benefit/risk profile of ritlecitinib was considered favorable for treatment of severe AA in both adults and adolescents. Ritlecitinib was safe and well tolerated in both adult and adolescent populations. The TEAE profile indicated that there were no significant risks unique to adolescents. Based on those results, ritlecitinib has been approved at the dose of 50 mg QD for the treatment of severe AA in adults and adolescents 12 years of age and older.

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Study B7981031 was conducted as an interventional PK study in the pediatric populations aged 6 to <12 years to estimate the systemic exposures ritlecitinib and support a dose selection for pediatric patients 6 to <12 years of age in Study B7981027.

Study B7981027 is a Phase 3 study in the pediatric AA population aged 6 to <12 years to evaluate the efficacy, safety, and tolerability of ritlecitinib compared to placebo. Participants who successfully complete the 24-week treatment period of the study will have the option to enter an LTE Study B7981028 if eligibility criteria are met.

2.2.1. Clinical Overview

The clinical development program of ritlecitinib is a global program and consists of completed, ongoing and planned studies in healthy participants and in patients with AA. This program includes 22 completed Phase 1 studies, 2 Phase 3 studies planned in AA, 4 Phase 3 studies planned in vitiligo and 1 planned Phase 1 relative bioavailability study of modified release formulations compared to the immediate release solution formulation. The POC for the AA indication was established in Study B7931005. B7981015 study was conducted as a single pivotal trial to support registration for treatment of moderate-to-severe. The B7981037 study was launched to increase the size of the safety database. A mechanistic safety study (B7981037) was included in the CDP to further evaluate clinical relevance of the axonal dystrophy finding in the 9-month dog toxicity studies of PF-06651600 should be investigated. The CDP for ritlecitinib in AA also includes the pediatric studies in children 6 to <12 years of age with AA (Studies B7981031, B7981027, B7981028).

A total of 491 participants were exposed to ritlecitinib in the Phase 1 healthy participant and special population studies. In completed and ongoing interventional Phase 2 and Phase 3 studies in AA and vitiligo, 1628 participants have received ritlecitinib.

Pharmacokinetic Overview of Ritlecitinib

The PK profile of ritlecitinib is characterized by rapid absorption, rapid elimination ($t_{½}$ of ~2 hours) and is approximately dose proportional. Steady state generally appears to have been reached by Day 4 for the QD regimens and Day 6 for the BID regimens based on similar median trough (pre-dose) ritlecitinib beyond Day 6. Ritlecitinib has been evaluated at single oral doses ranging from 5 mg to 800 mg and multiple oral doses ranging from 50 mg to 400 mg QD and at 100 mg and 200 mg BID for 14 days. The clearance mechanisms for ritlecitinib in humans appear to be primarily by metabolism. Less than 10% of ritlecitinib is excreted unchanged in the urine.

B7981078 was a Phase 1, randomized, open-label, crossover study in healthy adult participants to estimate the relative bioavailability of pediatric ritlecitinib sprinkled in soft foods relative to intact blend-in capsule of ritlecitinib. The study also assessed the sensory characteristics and overall palatability of sprinkled BiC by healthy participants. A group of 12 participants received each of the 5 treatments: Treatment A (ritlecitinib intact BiC 30 mg in given fasted state); Treatment B (ritlecitinib 30 mg BiC sprinkled on strawberry jam in fasted state); Treatment C (ritlecitinib 30 mg BiC sprinkled on yogurt in fasted state); Treatment D (ritlecitinib 30 mg BiC sprinkled on applesauce in fasted state); Treatment E

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(ritlecitinib intact BiC 30 mg given with high fat meal). Ritlecitinib 30 mg BiC sprinkled on strawberry jam was the most favorable. Ritlecitinib 30 mg BiC sprinkled on applesauce was more favorable than being sprinkled on yogurt.

Pharmacokinetics in Children

B7981031 was an interventional PK, PD, Phase 1, open-label study to investigate PK and PD of multiple-dose ritlecitinib in children 6 to less than 12 years of age with severe AA. The purpose of the study was to collect data to support dose selection for subsequent studies in the same population. Participants were given 20 mg ritlecitinib in one dose daily for 7 consecutive days. The primary endpoint was AUC_{24} on Day 7, and the secondary endpoints included PK parameters (C_{max}, T_{max}, CL/F, V_z/F, and half-life on Day 7).

Following multiple oral administration of 20 mg ritlecitinib capsules, plasma concentrations on Day 7 were observed at a median T_{max} of approximately 0.5 hour. Mean terminal half-life value for ritlecitinib was approximately 1.19 hours. Geometric mean values for ritlecitinib exposure parameters AUC_{24} and C_{max} were 437.5 ng•hr/mL and 208.7 ng/mL, respectively. The mean clearance was 0.0457 L/hr and mean value of distribution was approximately 0.075 L. The interparticipant variability (geometric %CV) for AUC_{24} and C_{max} was 30% to 38%, respectively.

Safety data are described in the subsequent section regarding safety results.

Clinical Overview of Ritlecitinib in Adult and Adolescent Participants With AA

B7981015 was a Phase 2b/3, randomized, double-blind, placebo-controlled, dose-ranging study to investigate ritlecitinib in both adolescent (≥12 to <18 years old) and adult (≥18 years old) participants with ≥50% scalp hair loss due to AA. The study had a maximum duration of approximately 57 weeks. This included an up to 5-week screening period, a 48-week treatment period, and a 4-week follow-up period. The treatment period was comprised of a placebo-controlled period that included a 4-week loading phase and a 20-week maintenance phase, followed by a 24-week extension phase.

Eligible participants were randomized to blinded ritlecitinib and matching placebo in a 2:2:2:2:1:1:1 (200 mg/50 mg, 200 mg/30 mg, 50 mg, 30 mg, 10 mg, placebo-200 mg/50 mg, and placebo-50 mg, respectively) manner for a total of 7 treatment sequences. All participants began dosing during the loading phase according to their assigned sequence. Following the 4-week loading phase, participants continued dosing according to their assigned sequence in the 20-week maintenance phase. At the end of the maintenance phase, placebo treated participants were advanced in a prespecified, blinded manner to one of 2 active treatment sequences for the remainder of the treatment period (through Week 48). A total of 718 participants were randomized among which 614 participants completed the study.

Ritlecitinib 200/50 mg, 200/30 mg, 50 mg and 30 mg were significantly superior to placebo at Week 24 on clinician-assessed and patient-reported endpoints related to scalp hair regrowth (including response based on absolute SALT ≤20, response based on absolute SALT ≤10, and PGI-C response). Ritlecitinib 200/50 mg, 200/30 mg, 50 mg and 30 mg were

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also nominally superior to placebo at Week 24 in producing improvement in eyebrows and eyelashes. Continued improvement in efficacy endpoints was seen between Week 24 and Week 48.

Overall, ritlecitinib was safe and well tolerated. The proportion of participants who experienced all-causality TEAEs was similar across treatment groups up to Week 24 (placebo-controlled period) and up to Week 48 (overall).

Fourteen participants experienced 16 SAEs up to Week 48. Of the 16 SAEs, 12 were considered by the investigator as unrelated to study intervention. The 4 SAEs that were considered related to study intervention in the opinion of the investigator were sepsis and empyema (both in 1 participant), breast cancer, and eczema. There were no deaths in the study.

Phase 3 Long-Term Safety Study B7981032 in AA

Study B7981032 is a Phase 3, open-label, multi-center, long-term study designed to evaluate the safety and efficacy of ritlecitinib in adults and adolescents ≥12 years of age with AA. A total of 1052 participants were randomized among which 190 participants have completed the study.

This study is ongoing.

Eligible prior participants in the index Studies B7931005 and B7981015 were given the opportunity to enroll ("rollover participants"), as well as de novo participants (ie, those who had not previously received study intervention in Study B7931005 or B7981015).

The maximum duration of participation in the study is approximately 60 months, including:

Up to 5 weeks for screening (for de novo participants and participants originating from Studies B7931005 or B7981015 with >30 days between the last dose in Studies B7931005 or B7981015 and their first visit in Study B7981032), 60 months of study intervention, and a follow-up period of 4 weeks after the last dose of study intervention.

Overall, ritlecitinib was safe and well tolerated in the adolescent population. In general, there was a lower proportion of adolescents with SAEs, severe AEs and AEs of interest relative to adults (>18 years of age). The TEAEs suggest that there were no risks unique to adolescents, except for a higher proportion of adolescents with acne (which is not unexpected considering that acne is generally more common in adolescents than adults).

Safety in Pediatric Population of 6 to <12 Years of Age

In Study B7981031, of the 15 participants who received study treatment, there were 4 all causality AEs reported by 3 (20.0%) participants. All the AEs were mild or moderate in severity. One participant permanently discontinued from the study due to TEAE of urticaria. No deaths, SAEs or severe AEs were reported in this study. The detailed safety information from the Study B7981031 can be found in the PF-06651600 (Ritlecitinib) 2023, v10.0 IB.

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2.3. Benefit/Risk Assessment

More detailed information about the known and expected benefits and risks and reasonably expected AEs of ritlecitinib may be found in the IB, which is the SRSD for this study. Refer to the Study Intervention(s) table in Section 6.1 for a complete description of SRSDs, for this study.

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2.3.1. Risk Assessment

Potential Risk of Clinical Significance	Summary of Data/Rationale for Risk	Mitigation Strategy
Study Intervention(s) **Ritlecitinib**		
Reductions in platelet counts and lymphocyte counts	The data from the ritlecitinib studies (B7981015 and B7981032) in adults and adolescent participants revealed a modest decrease in lymphocyte counts that were more apparent during treatment with loading doses of 200 mg QD ritlecitinib but were not considered clinically meaningful. Median decrease of platelets from baseline, which remained stable, was observed. There were no serious or severe AEs and 2 participants met protocol-specified discontinuation criteria for decreased platelets. There were no bleeding events associated with decreased platelets.	Loading doses of ritlecitinib will not be used in this study, and the highest dose utilized will be 50 mg QD (equivalent to 50 mg QD exposure in adults and adolescents). Clinical laboratory test results will be monitored and there are pre-specified eligibility, monitoring and discontinuation criteria (See Section 7) based on laboratory test results.
Serious and opportunistic infections	Ritlecitinib is an immunomodulator and, as such, can be associated with the potential risk of infections (including serious infections), opportunistic infections, and viral reactivation. The data from the ritlecitinib studies (B7981015 and B7981032) do not suggest a meaningful increase in the incidence of serious infections overall compared to placebo nor exhibit a dose response. There were 7 events of opportunistic infections in 7 participants: 2 participants in study B7981032 (both multidermatomal herpes zoster), 1 participant in study B7981037 (multidermatomal herpes zoster), 3 participants in Study B7981005 (1 event of cytomegalovirus infection and 2 events of	Participants with a history of recurrent infections and/or latent infections (including participants with evidence of active, latent or inadequately treated infection with TB) and immunocompromised participants will be excluded. Participants known to be infected with hepatitis B or hepatitis C or with a history of herpes zoster or disseminated herpes simplex are excluded from studies with ritlecitinib (See Section 5.2). The occurrence of infections will be monitored in this study, and there are discontinuation criteria for serious infections. See Section 7. Concomitant use with other systemic immunomodulators is prohibited (see Section 6.9).

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Potential Risk of Clinical Significance	Summary of Data/Rationale for Risk	Mitigation Strategy
Study Intervention(s) **Ritlecitinib**		
	herpes zoster), and 1 participant in Study B7981007 (cytomegalovirus infection)	
Dermatologic effects (rash, acne, urticaria).	In animals, ritlecitinib administration was associated with focal skin inflammation, generalized red skin, skin papules, and localized inflammation in hair follicles. In ritlecitinib clinical studies, one participant experienced a severe TEAE of rash macular. The other events of acne and rashes were mild and moderate in severity. Urticaria led to permanent discontinuation from study or study drug (up to Week 48) in 3 participants (1 participant in 200/30 mg and 2 participants in 50 mg) in Study B7981015 and 1 participant in 20 mg in Study B7981031.	Medical history includes any history of skin rash prior to enrollment in and during Study B7981027. Participants reporting an unexplained skin rash will be examined by a dermatologist according to local guidelines for formal comprehensive skin evaluations which may include a swab for culture to assist diagnosis. Rash to be treated in accordance with relevant guidelines. Participants who experience serious or severe drug-related rash may be discontinued from the study at the discretion of the investigator or sponsor.
Malignancy and lymphoproliferative disorders.	In chronic animal toxicology studies, ritlecitinib was not associated with the development of neoplasms or lymphoproliferative disorders. Lymphoma and other malignancies have been observed in patients treated with JAK inhibitors. Malignancies, including NMSC, were observed in clinical studies in adult participants with ritlecitinib. There was no evidence of an increased risk of malignancies, including NMSC. However, the clinical data are limited to assess the potential incidence of malignancies following longer term exposure to ritlecitinib. Long-term safety evaluations are ongoing	Participants with any present malignancies or history of malignancies, history of any lymphoproliferative disorder such as EBV related lymphoproliferative disorder, history of lymphoma, history of leukemia, or signs and symptoms suggestive of current lymphatic or lymphoid disease are not eligible for this study.

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Potential Risk of Clinical Significance	Summary of Data/Rationale for Risk	Mitigation Strategy
Study Intervention(s) **Ritlecitinib**		
Thromboembolic events	Thromboembolic events have been reported with JAK inhibitors approved for inflammatory diseases. 1 event of venous thromboembolism (pulmonary embolism) and 5 events of arterial thromboembolism (acute myocardial infarction) have been reported in adult patients receiving ritlecitinib in clinical studies.	Participants with any medical condition that may increase the risk of study participation or, in the investigator's judgment, make the participant inappropriate for the study are not eligible for this study.
Audiological and neurological effects	In pre-clinical dog studies, ritlecitinib administration for 9 months resulted in reversible CNS findings of axonal dystrophy, with reversible hearing loss in some animals. The exposure margins for axonal dystrophy with and without functional BAEP threshold and waveform changes were 33x and 7.4x, respectively, relative to the 50 mg human dose. In a juvenile rat toxicity study, oral administration of ritlecitinib from postnatal day 10 to 60 (comparable to infant through adolescence human age) was not associated with effects on the nervous system. The clinical data to date do not demonstrate evidence suggestive of neurotoxicity with ritlecitinib treatment.	Participants with pre-existing hearing loss regardless of hearing loss type, or middle ear disease such as otitis media will be excluded from the study (see Section 5.2). The audiometric assessment will be undertaken at screening and at Week 24/EOT. If there is a clinically meaningful, treatment-emergent decline in hearing resulting in an AE, the participant must be promptly evaluated by a medically qualified specialist to assess for possible causes. Evaluation results should then be discussed with the sponsor to determine if the participant should be discontinued from the study. Discontinued participants must be followed up off-treatment with appropriate testing at regular intervals, until hearing returns to baseline or is determined to be clinically stable (see Section 8.3.4). Neurologic assessment will be performed at screening and throughout the study. Participants with clinically meaningful changes from baseline in neurologic signs or symptoms will be referred to a neurologist to undergo a formal neurologic evaluation (see Section 8.3.3.3).
Potential fetal risk and potential risk if ritlecitinib is secreted into human milk.	In animals, ritlecitinib was associated with fetal skeletal variations, fetal skeletal and visceral malformations, and lower fetal body weights. It is not known whether ritlecitinib is secreted into human milk.	Breastfeeding and pregnant women are not eligible to participate in the study. Ritlecitinib should not be administered to breastfeeding women and exposure during breastfeeding or pregnancy should be reported to Pfizer Safety (see Environmental Exposure, Exposure During Pregnancy

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Potential Risk of Clinical Significance	Summary of Data/Rationale for Risk	Mitigation Strategy
Study Intervention(s) **Ritlecitinib**		
		or Breastfeeding, and Occupational Exposure and Appendix 4: Contraception Guidance). Urine pregnancy test with a sensitivity of at least 25 mIU/mL will be performed at screening and throughout the study at timepoints defined in the protocol SoA. Pregnancy screening test will be performed for all females ≥8 years of age. If, in the opinion of the investigator, pregnancy testing is not necessary for a participant above the age of 8 years, then the investigator should document why screening was not necessary (eg, based on their level of sexual maturity). In such cases, menarchal status should be recorded in the Medical History CRF. Pregnancy screening test (urine) and regular urine pregnancy testing throughout the study (at timepoints defined in the protocol SoA) will be performed for all post-menarchal females. Site personnel will contact participants (or their legally authorized representative, if appropriate) via telephone between study visits to check on contraception and results of pregnancy testing and the contact/results will be recorded in participant source documentation and CRF. WOCBP who are unwilling or unable to use contraception as defined in the study protocol will be excluded (see Appendix 4: Contraception Guidance).

2.3.2. Benefit Assessment

The results of the nonclinical studies together with the clinical experience obtained to date with ritlecitinib support the continued investigation of this therapy for the treatment of AA in the pediatric population aged 6 to <12 years. Clinical efficacy (eg, SALT ≤20 response) was demonstrated by ritlecitinib 50 mg and 30 mg doses in adult and adolescent participants with AT/AU and non-AT/AU. The 50 mg dose in this study will provide exposure equivalent to that of the 50 mg dose in adults and adolescents, and the 30 mg dose will provide exposure equivalent to that of the ritlecitinib 30 mg dose in adults and adolescents.

All participants may also benefit from protocol participation by gaining knowledge about their health status through study tests and safety assessments (eg, physical examinations, laboratory assessments, vital sign measurements, etc.) conducted at regular intervals during the trial. Also, after completion of this study, participants may qualify to be screened for the planned 3-year, long-term, open-label Study B7981028.

2.3.3. Overall Benefit/Risk Conclusion

Taking into account the measures to minimize risk to study participants, the potential risks identified in association with ritlecitinib are justified by the anticipated benefits that may be afforded to participants with severe AA.

3. OBJECTIVES, ENDPOINTS, AND ESTIMANDS

Objectives	Endpoints	Estimands
Primary:	**Primary:**	**Primary:**
For US and Countries Following US Analysis Plan		
• To evaluate the efficacy of ritlecitinib compared to placebo in pediatric participants with severe AA on regrowth of lost scalp hair.	• Response based on achieving an absolute SALT score ≤20 at Week 24.	• The difference in proportions of participants with the SALT ≤20 response at Week 24 between each ritlecitinib dose and placebo groups in pediatric AA participants defined by the inclusion and exclusion criteria.
For EU/UK and Countries Following EU/UK Analysis Plan		
• To evaluate the efficacy of ritlecitinib compared to placebo in pediatric participants with AA on regrowth of lost scalp hair.	• Response based on achieving an absolute SALT score ≤10 at Week 24.	• The difference in proportions of participants with the SALT ≤10 response at Week 24 between each ritlecitinib dose and placebo groups in pediatric AA participants defined by the inclusion and exclusion criteria.

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Objectives	Endpoints	Estimands
Key Secondary:	**Key Secondary:**	**Key Secondary:**
For EU/UK and Countries Following EU/UK Analysis Plan		
• To evaluate the effect of ritlecitinib on patient centered outcomes (PGI-C).	• PGI-C response defined as a score of "moderately improved" or "greatly improved" at Week 24.	• The difference in proportions of participants with the PGI-C response at Week 24 between each ritlecitinib dose and placebo groups in pediatric AA participants defined by the inclusion and exclusion criteria.
Secondary	**Secondary**	**Secondary**
For All Countries		
• To further evaluate the efficacy of ritlecitinib compared to placebo in pediatric participants with AA on regrowth of lost scalp hair at all visits (except for that included as the primary endpoint).	• CFB in SALT score at all visits. • Response based on achieving absolute SALT score ≤10 at all visits (except for that included as the primary endpoint). • Response based on achieving absolute SALT score ≤20 at all visits (except for that included as the primary endpoint).	• The difference in means/proportions in the endpoints at all scheduled time points (except for that included as the primary endpoints) between each ritlecitinib dose and placebo groups in pediatric AA participants defined by the inclusion and exclusion criteria.
• To evaluate the effect of ritlecitinib on patient centered outcomes, measured by PGI-C response at all visits (except for that included as a key secondary endpoint).	• PGI-C response at all visits (except for that included as a key secondary endpoint).	• The difference in proportions of participants with the PGI-C response at all scheduled time points (except for that included as key secondary endpoint) between each ritlecitinib dose and placebo groups in pediatric AA participants defined by the inclusion and exclusion criteria.

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Objectives	Endpoints	Estimands
• To evaluate the effect of ritlecitinib on AAPPO scales and psychological/social impairment proxy reported (PROMIS, BRIEF®2), and participant reported (modified CDLQI) at all visits.	• Response based on improvement from baseline for each AAPPO Item (11 endpoints).[a] • CFB in AAPPO activity limitation and emotional symptoms scores at all visits. • CFB in PROMIS Parent Proxy Depressive Symptoms T-score at all visits. • CFB in PROMIS Parent Proxy Anxiety Symptoms T-score at all visits. • CFB in BRIEF®2 T-scores for the 3 index scores (BRI, ERI, CRI) at all visits. • CFB in modified CDLQI total score at all visits.	• The difference in proportions/ means in the endpoints at all scheduled time points between each ritlecitinib dose and placebo groups in pediatric AA participants defined by the inclusion and exclusion criteria.
• To evaluate the effect of ritlecitinib versus placebo on achieving EBA response.	• Response based on achieving at least 2 grade improvement or a score of 3 in EBA score at all visits in participants with an abnormal EBA at baseline.*	• The difference in proportions of participants with the EBA response at all scheduled time points between each ritlecitinib dose and placebo groups in pediatric AA participants defined by the inclusion and exclusion criteria.
• To evaluate the effect of ritlecitinib versus placebo on achieving ELA response.	• Response based on achieving at least 2 grade improvement or a score of 3 in ELA score at all visits in participants with an abnormal ELA at baseline.**	• The difference in proportions of participants with the ELA response at all scheduled time points between each ritlecitinib dose and placebo groups in pediatric AA participants defined by the inclusion and exclusion criteria.
• To characterize the PK of ritlecitinib in children with AA 6 to <12 years of age.	• Plasma concentration of ritlecitinib at 1 hr (±15 min) and 3 hr (±30 min) post-dose at Week 4 or Week 8.	• Not applicable.
• To evaluate safety and tolerability of ritlecitinib over time.	• Incidence of treatment-emergent AEs (including audiological and neurological treatment-emergent AEs).*** • Incidence of SAEs and AEs leading to permanent discontinuation from the study.	• Not applicable.

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Objectives	Endpoints	Estimands
• To evaluate acceptability and palatability of the age-appropriate formulation.	• Acceptability and palatability assessment.	• Not applicable.
Tertiary (Exploratory):	**Tertiary (Exploratory):**	**Tertiary (Exploratory):**
• To evaluate the effect of ritlecitinib versus placebo on achieving a ≥75% improvement in SALT ($SALT_{75}$) and a SALT score of 0 at all visits.	• Response based on achieving a ≥75% improvement from baseline in SALT ($SALT_{75}$) at all visits. • Response based on achieving a SALT score of 0 at all visits.	• Not applicable.

a. For each of the 11 AAPPO items, among participants whose item-specific score at baseline is ≥2, a response is defined as having a score of 0 (No hair loss) or 1 (A little hair loss) for Item 1 to Item 4, a response is defined as having a score of 0 (Never) or 1 (Rarely) for Item 5 to Item 8, a response is defined as having a score of 0 (Not at all) or 1 (A little) for Item 9 to Item 11.
* Abnormal EBA at baseline is defined as an EBA score of 0, 1 or 2.
** Abnormal ELA at baseline is defined as an ELA score of 0, 1 or 2.
*** Audiological assessment includes updated audiological history, otoscopic examination, and air pure tone audiometry at Month 6. Additional audiometry assessments will be performed if clinically indicated.

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4. STUDY DESIGN

4.1. Overall Design

Study B7981027 will be a Phase 3 randomized, double-blind, placebo-controlled study to investigate the safety, efficacy and tolerability of ritlecitinib in pediatric participants (6 to <12 years of age) with 50% or greater scalp hair loss due to AA measured by a SALT score of 50 or greater (severe AA).

This study will include 2 ritlecitinib dosage levels (50 mg and 30 mg) and matching placebo. The participants will be assessed for study eligibility at the screening visit after assent is obtained (as applicable) from the participant and informed consent is obtained through their parents or legal guardians. Participants will complete screening, be randomized then receive 1 of the 2 dosage levels of ritlecitinib or placebo, to be taken orally for 24 weeks.

Approximately 225 participants will be randomized. At least 30% of total study population will be recruited from Europe, as required by PDCO. A stratified randomization will be used for AT/AU (AT/AU and non-AT/AU). Within each of the 2 strata, participants will be randomized in a 1:1:1 ratio to the 2 dosage levels of ritlecitinib or placebo.

The ritlecitinib doses were informed by results of Study B7981031; a 50 mg dose to match exposures of 50 mg and a 30 mg dose to match exposures of 30 mg in adults and adolescents in Study B7981015.

Study visits to assess safety and efficacy will be conducted as summarized in the SoA (Section 1.3).

Safety monitoring will be performed to identify and monitor important identified or potential risks (viral reactivation, serious infections and opportunistic infections, malignancy, thromboembolism, dermatologic effects [rash, acne, urticaria], decreased lymphocyte counts, and decreased platelet counts), as well as audiological and neurological effects.

Safety evaluations will also include age-appropriate auditory screening as well as neurological assessments performed during the physical examinations. Audiologic evaluation includes audiological history, otoscopic examination, air and bone pure tone audiometry, speech audiometry, and immittance audiometry at screening. At Month 6, updated audiological history, otoscopic examination, air pure tone audiometry will be performed; based upon results, additional audiometry assessments may be required. Refer to the Pediatric Audiometry Study Guide for details on auditory evaluation. All procedures must be performed as per the Pediatric Audiometry Study Guide.

Neuro-psychological and cognitive development evaluations include BRIEF®2 (at all visits) and WISC-V (at baseline) assessments, respectively. WISC–V will be conducted only at baseline in the B7981027 study and will also be assessed at the end of long-term extension (LTE) Study B7981028 for participants rolling over to Study B7981028.

For any AEs or changes of potential concern, referral to a specialist for further evaluation is required.

PK parameters using sparse sampling will be evaluated to contribute to the population PK model.

Participants completing the 24-week treatment period of the study may have the option to enter LTE Study B7981028, if the eligibility criteria are met. Participants who complete the 24-week treatment period of the study and are ineligible for the LTE study will undergo a 4-week off-treatment follow-up period. Participants who are withdrawn early from study intervention for any reason will remain in the study and continue the regularly scheduled visits through Week 24 and follow-up visits. At the time of discontinuation from study intervention, all efficacy, safety and PRO assessments planned at the scheduled Week visit and the EOT visit will be performed together.

4.2. Scientific Rationale for Study Design

4.2.1. Patient Input Into Design

Surveys of children and parents were conducted to evaluate preferences and feasibility of the study, including study drug administration, blood draws, and clinic visit duration. All participants indicated that they were comfortable with taking pills as a form of medication. Most of the survey participants indicated that they were apprehensive about blood withdrawal, but they could tolerate the procedure. Majority of the survey participants responded that they were comfortable with study visits and about 93% participants preferred to watch TV or movies during visits for best clinical trial experience.

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4.2.2. Diversity of Study Population

The diversity strategy will include selection of sites with the potential to support recruitment of racially and ethnically diverse pediatric populations across geographically diverse countries. Reasonable attempts will be made to enroll participants that are representative of the patient population that will be treated with ritlecitinib in clinical practice.

4.2.3. Rationale for Comparator

In order to establish clinical efficacy of ritlecitinib in children 6 to <12 years of age with severe AA, the study will use placebo as the comparator. As there are currently no approved therapies for severe AA in children 6 to <12 years of age, this study will not include an active comparator arm.

4.2.4. Use of Adjudication Committees

This protocol will use independent endpoint adjudication committees to determine whether certain investigator-reported safety events meet the definition of disease-related safety endpoints, using predefined endpoint criteria. Further information about this endpoint adjudication committee is provided in the charter, including a description of the scope of the committee's responsibilities, the process and definitions to be utilized by the committee for adjudication, and communication plan including timelines.

The following adjudication committees will be used:

- Opportunistic Infection Review Committee (OIRC),
- Cardiovascular Event Adjudication Committee (CVEAC),
- Malignancy Adjudication Committee (MAC), and
- Neurosafety Event Adjudication Committee (NSEAC).

Events requiring review may be identified by:

- Pfizer Study Team or designee during the review of participant data listings, or
- Site monitors during their routine review of a participant's study records.

Additional types of events for review and adjudication may be identified by Pfizer. The Pfizer Study Team or designee will notify the relevant study site of any such events should they be identified. Investigators will be responsible for obtaining and submitting the documentation to be reviewed by the safety event adjudication committee. Documentation includes, but is not limited to, the following: hospital discharge summaries, operative reports, clinic notes, ECGs, diagnostic tests, pathology reports, autopsy reports and death certificate information, as applicable. These documents may be reviewed by Pfizer.

Further information about these safety event adjudication committees is provided in respective charters, including a description of the scope of the committee's responsibilities,

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the process and definitions to be utilized by the committee for adjudication, and communication plan including timelines.

Additional safety event adjudication committees may be established during the study to standardize additional safety event assessments. As described above, the charter will provide specific descriptions of the scope of responsibilities and the processes and definitions used to assess specific safety events for each of the individual adjudication committees.

4.2.5. Choice of Contraception/Barrier Requirements

Human reproductive safety data are limited for ritlecitinib and not enough to evaluate the risk for treatment related birth defects. In animal studies (rats and rabbits) fetotoxicity and fetal malformations were observed at exposures equivalent to 49 to 55 times the MRHD of 50 mg QD in adults. No developmental toxicity was observed at exposures equivalent to 12 to 16 times the MRHD. In animals, ritlecitinib was associated with fetal changes in bones and some internal organs, and lower fetal body weights. Therefore, the use of a highly effective method of contraception in WOCBP is required. Male participants in the study are not required to use birth control, because ritlecitinib is not genotoxic and not likely to transfer to a partner through semen at pharmacologically relevant blood levels (see Appendix 4).

The definition of WOCBP is provided in Section 10.4.3.

4.3. Justification for Dose

Given the similar disease etiology and pathophysiology between adult and pediatric participants, as well as similar exposure-response relationship of efficacy established between adults and adolescents in Studies B7981015, B7931005, B7981032, the exposure-response for efficacy is expected to be similar between adults/adolescents and younger pediatric participants (6 to <12 years of age). Therefore, the doses to be evaluated in the current study were selected based on the consideration of the doses that are expected to provide exposure equivalents to the efficacious doses tested in adults and adolescents.

In Study B7981015, both 50 mg and 30 mg doses were efficacious and provided a significantly greater efficacy response compared to placebo. The 50 mg dose is the approved dose in adults and adolescents.

The pediatric dose matching adult and adolescent 50 mg exposure is being evaluated in Study B7981027 because this is the approved dose in adults and adolescents. In addition, the pediatric dose matching adult and adolescent 30 mg exposure is being evaluated in Study B7981027 because this was the minimum efficacious dose in Study B7981015.

Ritlecitinib PK in pediatric participants 6 to <12 years of age was evaluated in Study B7981031. The previous population PK model developed in adults and adolescents has been updated including the B7981031 study data. The updated population PK analysis results indicated that CL/F in participants 6 to <12 years of age is faster than the extrapolated CL/F from adults and adolescents using body weight based allometric scaling. The simulation results using the updated population PK model suggest that 50 mg in 6 to <12 years old participants will provide equivalent exposure to the 50 mg dose in adults and adolescents,

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and the 30 mg dose will provide equivalent exposure to the 30 mg in adults and adolescents. Thus, 50 mg and 30 mg doses were selected to be evaluated in this pediatric Phase 3 study.

4.4. End of Study Definition

The end of the study is defined as the date of the last visit of the last participant in the study.

A participant is considered to have completed the study if they have completed all periods of the study, including the last scheduled procedure shown in the SoA.

5. STUDY POPULATION

This study can fulfill its objectives only if appropriate participants are enrolled, including participants across diverse and representative racial and ethnic backgrounds. If a prescreening tool is utilized for study recruitment purposes, it will include collection of information that reflects the enrollment of a diverse participant population including, where permitted under local regulations, age, sex, race, and ethnicity. The following eligibility criteria are designed to select participants for whom participation in the study is considered appropriate. All relevant medical and nonmedical conditions should be taken into consideration when deciding whether a particular participant is suitable for this protocol.

Prospective approval of protocol deviations to recruitment and enrollment criteria, also known as protocol waivers or exemptions, is not permitted.

5.1. Inclusion Criteria

Participants are eligible to be included in this study only if all of the following criteria apply:

Age and Sex:

1. 6 to <12 years old at the time of the screening visit.

   Refer to Appendix 4 for reproductive criteria for male (Section 10.4.1) and female (Section 10.4.2) participants.

Disease Characteristics:

2. A diagnosis of AA (including AT and AU) with at least 50% scalp hair loss due to AA (ie, a SALT score of ≥50) at both screening and baseline visits, without evidence of terminal hair regrowth within the previous 12 months.

3. <u>For study participants in the EU/UK only:</u>

   - History of clinical response failure to AA treatment (such as topical, off-label pharmacologic, or hairpiece prosthetics)

Other Inclusion Criteria:

4. Documented evidence of having received varicella vaccination (2 doses), OR

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- evidence of prior exposure to VZV based on serological testing (ie, a positive VZV IgG Ab result) at screening.

5. For sites in EU member countries (Czech Republic, France, Italy, Poland, Spain): Minimum body weight of ≥17 kg at the time of the screening visit.

5.2. Exclusion Criteria

Participants are excluded from the study if any of the following criteria apply:

Medical Conditions:

1. Other (non-AA) types of alopecia, including any known congenital cause of AA.

2. Pre-existing hearing loss is excluded regardless of hearing loss type. If hearing loss is due to wax build up or acute middle ear disease such as active AOM, participants can be allowed for re-screen when wax is removed, or infection has cleared, and retest shows normal hearing. Note: recurrent AOM (defined as having at least three episodes of AOM in a period of 6 months, or four or more episodes in 12 months) is excluded.

3. Any present malignancies or history of malignancies, history of any lymphoproliferative disorder such as EBV related lymphoproliferative disorder, history of lymphoma, history of leukemia, or signs and symptoms suggestive of current lymphatic or lymphoid disease.

4. Active autoimmune disorder (other than AA)

5. Known immunodeficiency disorder.

6. Any medical or psychiatric condition or laboratory abnormality that may increase the risk of study participation or, in the investigator's judgment, make the participant inappropriate for the study.

A risk assessment must be done by a qualified mental health professional to assess whether it is safe for the participant to participate in the trial if the participant's screening information indicate any of the following:

- Suicidal ideation associated with actual intent and a method or plan at any time in their lifetime: "Yes" answer on items 4 or 5 of the C-SSRS.

- Previous history of suicidal behaviors at any time in their lifetime: "Yes" answer to any of the suicidal behavior items of the C-SSRS.

- Any lifetime history of serious or recurrent suicidal behavior (non-suicidal self-injurious behavior is not a trigger for a risk assessment unless in the investigator's judgement it is indicated).

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- Clinically significant depression per PROMIS Parent Proxy Short Form - Depressive symptoms (T-score ≥70).

- The presence of any current major psychiatric disorder that is not explicitly permitted in the inclusion/exclusion criteria.

A summary of risk assessment should be documented in the participant's source document/medical record.

If a participant presents with suicidal ideation/behavior at any time during the study, the participant's parent or legal guardian should be informed.

7. Trisomy 21

Prior/Concomitant Therapy:

8. Current use of any prohibited concomitant medication(s) or history of discontinuation of treatment with a JAK inhibitor due to a treatment related adverse event. Refer to Section 6.9.

Prior/Concurrent Clinical Study Experience:

9. Previous administration of an investigational product (drug or vaccine) within 30 days or 5 half-lives preceding the first dose of study intervention used in this study or as determined by the local requirement (whichever is longer).

Diagnostic Assessments:

10. Renal dysfunction as defined using an eCrCl normalized to BSA (units of mL/min/1.73 m²): confirmed eCrCl <60 mL/min/1.73 m² based on Modified Schwartz equation (pediatric participants 2 to <12 years of age).

    Hepatic dysfunction defined as:

    Total bilirubin >1.5 × ULN (except for Gilbert's syndrome)

    AST >1.5 × ULN

    ALT >1.5 × ULN

    Hematologic abnormalities defined as:

    ANC <1.2 × 10^9/L (<1200/mm^3)

    Hemoglobin <11.0 g/dL

    Platelet count <150 × 10^9/L (<150,000/mm^3)

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Absolute lymphocyte count of $<0.8 \times 10^9$/L ($<800/mm^3$)

11. Infections:

- Evidence of untreated or inadequately treated active or latent Mycobacterium TB infection as evidenced by a positive QFT-G test or positive or borderline SPOT ® TB (T-Spot) test performed within the 3 months prior to baseline. If the laboratory reports the test as indeterminate, the test should be repeated. If the result of the repeat test is indeterminate, a PPD test may be substituted for the QFT-G test or T-Spot test only with approval from the Pfizer Medical Monitor on a case-by-case basis.

- Refer to Appendix 10, Section 10.10.5.3 for Czech Republic-specific requirements for TB testing at screening.

- Any infection requiring hospitalization, parenteral antimicrobial therapy or judged to be opportunistic by the investigator within the 3 months prior to the first dose of IP; or infection with HIV; or active acute or chronic infection requiring treatment with oral antibiotics, antivirals, antiparasitics, antiprotozoals, or antifungals within 4 weeks prior to Day 1; or superficial skin infection within 1 week prior to Day 1.

- Infection with hepatitis B or hepatitis C viruses according to protocol-specific testing algorithm:

  a) For hepatitis B, all participants will undergo testing for HBsAg and HBcAb

     - Participants who are HBsAg positive will not be eligible for this study.

     - Participants who are HBsAg negative but HBcAb positive will be reflex tested for HBsAb.

     - Additional reflex testing for HBV DNA is also required for HBcAb positive participants in countries in which hepatitis B prevalence has been reported at a rate of >5.0% or if required by local standard of care. (Refer to Section 10.10.3.1 for Japan specific requirements.)

     - Refer to Appendix 10, Section 10.10.5.1 for Czech Republic-specific requirements for HBV-DNA testing.

  b) For hepatitis C, all participants will undergo testing HCVAb during screening:

     - Participants who are HCVAb positive will be reflex tested for HCV RNA.

     - Participants who are HCVAb and HCV RNA positive are not eligible for the study.

     - Refer to Appendix 10, Section 10.10.5.2 for Czech Republic-specific requirements for HCV-RNA testing.

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- History (1 or more episodes) of severe or serious CMV infection, herpes zoster (shingles) or disseminated herpes simplex. A history of uncomplicated herpes simplex is not exclusionary.

12. Significant trauma or major surgery within 1 month of the first dose of study intervention.

13. Within 6 weeks of first dose of study intervention: vaccination with live attenuated replication-competent vaccine*

    * Vaccination with live attenuated replication-competent vaccines is prohibited within the 6 weeks prior to the first dose of study intervention, while receiving study intervention, and for 6 weeks after the last dose of study intervention.

    - Detailed vaccination information is provided in Section 6.9.3.

Other Exclusion Criteria:

14. Investigator site staff directly involved in the conduct of the study and their family members, site staff otherwise supervised by the investigator, and sponsor and sponsor delegate employees directly involved in the conduct of the study and their family members.

15. Hypersensitivity to the active substance(s) or to any of the excipients of the investigational product.

5.3. Lifestyle Considerations

5.3.1. Contraception

The investigator or qualified designee, in consultation with the participant and parent/legal guardian, will confirm that the participant is utilizing an appropriate method of contraception for the individual participant and their partner(s) from the permitted list of contraception methods (see Appendix 4, Section 10.4.4) and will confirm that the participant has been instructed in its consistent and correct use. The investigator or designee will advise the participant and parent/legal guardian to seek advice about the donation and cryopreservation of germ cells prior to the start of study intervention, if applicable.

At time points indicated in the SoA, the investigator or designee will inform the participant and parent/legal guardian of the need to use highly effective contraception consistently and correctly and document the conversation and the participant's affirmation in the participant's chart. Participants need to affirm their consistent and correct use of at least 1 of the selected methods of contraception, considering that their risk for pregnancy may have changed since the last visit.

In addition, the investigator or designee will instruct the participant and parent/legal guardian to call immediately if the selected contraception method is discontinued and document the requirement to use an alternate protocol-specified method, including if the participant will no

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longer use abstinence as the selected contraception method, or if pregnancy is known or suspected in the participant or partner.

5.3.2. Meals and Dietary Restrictions

There are no restrictions regarding food or drink either before or after dosing. Participants should follow their regular diet.

5.3.3. Cosmetic Treatments/Applications

Following the screening visit and through 28 days after the last dose of study intervention, participants must not shave the hair on the scalp, eyebrows, or eyelashes. Participants wishing to keep their hair short are permitted to do so and an appropriate length should be discussed with the investigator to allow for assessment of hair re-growth.

In addition, the following applies to other cosmetic treatments/applications:

1. Hair transplants and tattooing of scalp, eyebrows, and eyelashes, including procedures such as microblading, are not permitted during study but these procedures performed prior to screening may be permitted, based on the investigator's opinion.

2. Hair prosthetics (eg, wigs, hair extensions) are permitted but must be removed for clinical assessments of AA at all study visits.

3. Hair dye is permitted to be used during the study, but participants should be discouraged from undergoing any hair dying process for 7 days prior to a study visit.

4. Mascara and false eyelashes are permitted but must be removed for clinical assessments of AA at all study visits. The use of false eyelashes with adhesive should be discouraged, where possible, due to the risk of eyelash loss during removal.

5.4. Screen Failures

Screen failures are defined as participants who consent to participate in the clinical study but are not subsequently enrolled in the study. A minimal set of screen failure information is required to ensure transparent reporting of screen failure participants to meet the CONSORT publishing requirements and to respond to queries from regulatory authorities. Minimal information includes demography, screen failure details, and any SAE.

Individuals who do not meet the criteria for participation in this study (screen failure) may be rescreened for not meeting exclusion criteria # 2, 9, 10 (hematological abnormalities only), 11 and 12.

Participants meeting exclusion criteria # 2, 9, 10, 11, or 12 may be rescreened after the exclusion interval is over as follows:

- exclusion criterion # 2 – after resolution of acute otitis media;

- exclusion criterion # 9 – 30 days or 5 half-lives for investigational products;

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- exclusion criterion # 10 (hematological abnormalities).

Participants meeting exclusion criteria # 10 may be rescreened after the hematological laboratory parameters return to normal.

- exclusion criterion # 11 (infection requiring hospitalization or parenteral treatment) – 3 months after the infection resolves;

- exclusion criterion # 11 (Active acute or chronic infection requiring treatment) – 28 days after the infection resolves (exception for superficial skin infections – 7 days after the infection resolves);

- exclusion criterion # 12 – 30 days after significant trauma.

At the rescreening visit, all screening procedures must be repeated (including lab tests) and all other eligibility criteria must be met.

6. STUDY INTERVENTION(S) AND CONCOMITANT THERAPY

Study interventions are all prespecified investigational and medical devices, and other interventions (eg, surgical and behavioral) intended to be administered to the study participants during the study conduct.

For the purposes of this protocol, study intervention refers to ritlecitinib and placebo. On the day of randomization, study intervention will be provided to the participant's parent/legal guardian/caregiver.

6.1. Study Intervention(s) Administered

Study Intervention(s)		
Intervention Name	Ritlecitinib (PF-06651600)	Placebo
Type	Drug	Drug
Use	Experimental	Placebo
IMP or NIMP/AxMP	IMP	IMP
Dose Formulation	Capsule for Ritlecitinib 30 mg Capsule for Ritlecitinib 50 mg	Capsule (to match Ritlecitinib 30 mg) Capsule (to match Ritlecitinib 50 mg)
Unit Dose Strengths	30 mg, 50 mg	0 mg
Dosage Levels	30 mg QD for 24 weeks 50 mg QD for 24 weeks	0 mg QD for 24 weeks

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Study Intervention(s)		
Route of Administration	Oral	Oral
Sourcing	Provided centrally by the sponsor	Provided centrally by the sponsor
Packaging and Labeling	Study intervention will be provided in HDPE bottles. Each bottle will be labeled as required per country's requirement. Participants will receive blinded, labeled supplies throughout the study.	Study intervention will be provided in HDPE bottles. Each bottle will be labeled as required per country's requirement. Participants will receive blinded, labeled supplies throughout the study.
SRSD	IB	NA
Current Name	Ritlecitinib (PF-06651600)	NA

Study Arms			
Arm Title	Arm 1: Ritlecitinib 50 mg	Arm 2: Ritlecitinib 30 mg	Arm 3: Placebo
Arm Description	Participants will receive 1 ritlecitinib 50 mg capsule QD and 1 placebo 30 mg capsule QD orally for 24 weeks.	Participants will receive 1 ritlecitinib 30 mg capsule QD and 1 placebo 50 mg capsule QD orally for 24 weeks.	Participants will receive 1 placebo 50 mg capsule QD and 1 placebo 30 mg capsule QD orally for 24 weeks.

6.1.1. Administration

Participants will receive the study intervention as outpatients. Ritlecitinib capsules and matching placebo capsules for oral administration will be dispensed in bottles. Participants and their parent/legal guardian/caregiver will be provided with clear dosing instructions. Sites will be trained on how participants should take capsules at home through an IP manual and/or other vehicle(s). Sites are responsible for communicating this information and site staff should review the dosing instructions with participants and their parent/legal guardian/caregiver at every study visit.

Participants should take the study intervention orally once daily; participants should swallow the capsule with liquid. Participants will swallow the study intervention whole and will not manipulate or chew the capsules prior to swallowing. Participants who cannot swallow the capsule can take the study intervention by opening the capsule and sprinkling the contents onto food. Participants will be encouraged to take the study intervention in the morning whenever possible. Participants should take the study intervention at approximately the same time every day. The study intervention may be taken with or without food. However, for study visit days (except at EOT visit), participants are to be instructed to refrain from dosing at home and are to take the dose at the investigational site (from the containers being returned to the site [ie, dispensed at the previous study visit]).

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If a dose is missed and the interval to the next dose is less than 8 hours, the missed dose should not be administered. If a dose is missed and the interval to the next dose is ≥8 hours, the missed dose should be administered. In the event that the participant vomits after administration of the study intervention, the dose should not be repeated and should not be taken until the next day at the regular time.

A dosing diary will be provided to record daily administration of the study intervention and any missed dose information. The diary should be completed by the participant's parent/legal guardian/caregiver.

Administration in Clinic

Administration of study intervention(s) at the site will be performed by an appropriately qualified and trained member of the study staff as allowed by local, state, and institutional guidance.

Following administration of study intervention(s) at the site, participants will be observed for at least 30 minutes by an appropriately qualified and trained member of the study staff. Appropriate medication and other supportive measures for management of a medical emergency will be available in accordance with local guidelines and institutional guidelines.

Study personnel should review dose administration requirements with the participant, as appropriate, and with delegated caregiver(s) (may include school nurse) before administration and throughout the study as necessary.

Dosing instructions should be reviewed with the parent/legal guardian/caregiver and adolescent at every clinic visit and phone contact. Study intervention must be dispensed to an adult and managed by an adult. The parent/legal guardian/caregiver must be closely involved in supervising the study intervention dosing administration and compliance for participants. If contact cannot be made with the parent/legal guardian/caregiver to review dosing instructions, then the participant will be discontinued from the study intervention. If it is found that there is lack of involvement in managing study intervention for the participant, then the participant will be discontinued from the study intervention but will remain in the study and continue the regularly scheduled visits through Week 24 and follow-up visit.

6.2. Preparation, Handling, Storage, and Accountability

1. The investigator or designee must confirm that appropriate conditions (eg, temperature) have been maintained during transit for all study interventions received and any discrepancies are reported and resolved before use of the study intervention.

2. Only participants enrolled in the study may receive study intervention and only authorized site staff may supply, prepare, and/or administer study intervention.

3. All study interventions must be stored in a secure, environmentally controlled, and monitored (manual or automated recording) area in accordance with the labeled storage conditions with access limited to the investigator and authorized site staff. At a minimum,

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daily minimum and maximum temperatures for all site storage locations must be documented and available upon request. Data for nonworking days must indicate the minimum and maximum temperatures since previously documented upon return to business.

4. Any excursions from the study intervention label storage conditions should be reported to Pfizer upon discovery along with actions taken. The site should actively pursue options for returning the study intervention to labeled storage conditions, as soon as possible. Once an excursion is identified, the study intervention must be quarantined and not used until Pfizer provides permission to use the study intervention. Specific details regarding the excursion definition and information to report for each excursion will be provided to the site in the IP manual.

5. Any storage conditions stated in the SRSD will be superseded by the storage conditions stated on the label. Site staff will instruct participants on the proper storage requirements for take-home study intervention.

6. Study interventions should be stored in their original containers.

7. The investigator, institution, head of the medical institution (where applicable), or authorized site staff is responsible for study intervention accountability, reconciliation, and record maintenance (ie, receipt, reconciliation, and final disposition records), such as the IPAL or sponsor-approved equivalent. All study interventions will be accounted for using a study intervention accountability form/record. All study intervention that is taken home by the participant, both used and unused, must be returned to the investigator by the participant. **Returned study intervention must not be re-dispensed to the participants.**

8. Further guidance and information for the final disposition of unused study interventions are provided in the IP manual. All destruction must be adequately documented. If destruction is authorized to take place at the investigator site, the investigator must ensure that the materials are destroyed in compliance with applicable environmental regulations, institutional policy, and any special instructions provided by Pfizer.

Upon identification of a product complaint, notify the sponsor within 1 business day of discovery as described in the IP manual.

6.2.1. Preparation and Dispensing

A qualified staff member will dispense the study intervention using unique container numbers via an IRT system in the bottles, provided, in quantities appropriate according to the SoA. A second staff member will verify the dispensing. The participant/caregiver should be instructed to maintain the product in the bottle provided throughout the course of dosing and return the bottle to the site at the next study visit.

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6.3. Assignment to Study Intervention

Allocation of participants to treatment groups will proceed through the use of an IRT system. Randomization will be stratified based on AT/AU (AT/AU vs non-AT/AU). Within a stratum, the block randomization method will be used to ensure a balance in the number of participants across treatment groups and the block size will be a multiple of 3 participants (corresponding to a randomization ratio of 1:1:1). The site personnel (study coordinator or specified designee) will be required to enter or select information including but not limited to the user's ID and password, the protocol number, and the participant number. The site personnel will then be provided with a randomization number corresponding to the assigned treatment group, and DU or container number(s) when study intervention is being supplied via the IRT system. The IRT system will provide a confirmation report containing the participant number, randomization number, and DU or container number assigned. The confirmation report must be stored in the site's files.

Study intervention will be dispensed at the study visits summarized in the SoA. Returned study intervention must not be re-dispensed to the participants.

The study-specific IRT reference manual and IP manual will provide the contact information and further details on the use of the IRT system.

6.4. Blinding

This is a randomized, double-blind, placebo-controlled study.

6.4.1. Blinding of Participants

Participants and their caregivers will be blinded to their assigned study intervention.

6.4.2. Blinding of Site Personnel

Investigators and other site staff will be blinded to participants' assigned study intervention.

6.4.3. Blinding of the Sponsor

Sponsor staff will be blinded to participants' assigned study intervention, except for sponsor staff involved in the assignment or distribution of study intervention and the provision noted below.

Sponsor staff who are not directly involved with the conduct of this study will prepare analyses and documentation containing unblinded data while the study is ongoing to support interactions with the E-DMC. Refer to Section 10.1.5 for additional details.

6.4.4. Sensitive Clinical Data

Sensitive clinical data are data collected in this study that have the potential to unblind a participant's treatment assignment. Access to sensitive clinical data will be restricted to authorized individuals until the study has been unblinded. The following data variables are considered sensitive clinical data: IP-10 (CXCL-10) and PK data. Refer to Sections 8.7.1.1 and 8.5.1, respectively, for additional details.

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6.4.5. Breaking the Blind

The IRT will be programmed with blind breaking instructions. In case of an emergency, the investigator has the sole responsibility for determining if unblinding of a participant's treatment assignment is warranted. Participant safety must always be the first consideration in making such a determination. If the investigator decides that unblinding is warranted, the investigator should make every effort to contact the study medical monitor prior to unblinding a participant's treatment assignment unless this could delay further management of the participant. If a participant's treatment assignment is unblinded, the sponsor must be notified within 24 hours after breaking the blind. The date and reason that the blind was broken must be recorded in the source documentation and CRF.

The study-specific IRT reference manual and IP manual will provide the contact information and further details on the use of the IRT system.

6.5. Study Intervention Compliance

Participant compliance with study intervention will be assessed at each visit. Compliance with the dosing of study intervention will be monitored and verified by delegated site personnel.

When participants are dosed at the site, they will receive study intervention directly from the investigator or designee, under medical supervision. The date and time of each dose administered in the clinic will be recorded in the source documents and recorded in the CRF. The dose of study intervention and study participant identification will be confirmed at the time of dosing by a member of the study site staff other than the person administering the study intervention. Study site personnel will examine each participant's mouth to ensure that the study intervention was ingested.

When participants self-administer study intervention(s) at home, compliance with study intervention will be assessed at each visit. Compliance will be assessed by direct questioning, counting returned capsules and review of the dosing diary during the site visits and documented in the source documents and CRF. Deviation(s) from the prescribed dosage regimen should be recorded in the CRF.

A record of the number of ritlecitinib/placebo capsules dispensed to and taken by each participant must be maintained and reconciled with study intervention and compliance records. Intervention start and stop dates, including dates for intervention delays and/or dose reductions, will also be recorded in the CRF.

If compliance is <80% between visits, the investigator or designee is to counsel the participant and the participant's parent/legal guardian/caregiver and ensure steps are taken to improve compliance. If the participant is over compliant (>120% between visits) with study intervention (intentional or accidental), the investigator or designee is to counsel the participant and the participant's parent/legal guardian/caregiver and ensure correct understanding of the study intervention dosing regimen. The investigator should contact the Pfizer Study Clinician promptly with any over compliance that may potentially impact the safe use of study intervention or that may result in an SAE.

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6.6. Dose Modification

Dose modification is not permitted during the study.

6.7. Continued Access to Study Intervention After the End of the Study

No study intervention will be provided to participants at the end of their study participation. It is expected that participants will be treated as required with standard-of-care treatments, as advised by their usual care physician. Participants completing the 24-week treatment period of the study will have the option to enter the LTE study (B7981028).

6.8. Treatment of Overdose

For this study, any total daily dose greater than 1 dose of study intervention will be considered an overdose. If a dose is missed and the interval to the next dose is ≥8 hours, the missed dose should be administered, and this will not be considered an overdose.

There is no specific treatment for an overdose.

In the event of an overdose, the investigator should:

1. Contact the study medical monitor within 24 hours.
2. Closely monitor the participant for any AEs/SAEs and laboratory abnormalities as medically appropriate and follow up until resolution, stabilization, the event is otherwise explained, or the participant is lost to follow-up (as defined in Section 7.3).
3. Document the quantity of the excess dose as well as the duration of the overdose in the CRF.
4. Overdose is reportable to Pfizer Safety **only when associated with an SAE.**

Decisions regarding dose interruptions will be made by the investigator in consultation with the study medical monitor as needed based on the clinical evaluation of the participant.

6.9. Prior and Concomitant Therapy

Medications/treatments that are taken in the screening period (after informed consent/assent is obtained and before the first dose of study intervention) as well as any medications/treatments taken for the treatment of AA at any time prior to the screening visit will be documented as prior medications/treatments. Medications/treatments taken after the first dose of study intervention has been administered will be documented as concomitant medications/treatments.

Any medication or vaccine (including over-the-counter or prescription medicines, vitamins, and/or herbal supplements) that the participant is receiving at the time of enrollment or receives during the study must be recorded along with:

- Reason for use

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- Dates of administration including start and end dates
- Dosage information including dose and frequency

The medical monitor should be contacted if there are any questions regarding concomitant or prior therapy.

6.9.1. Permitted Concomitant Medications

For the purposes of this protocol, dietary supplements are defined as vitamins, minerals, and purified food substances with pharmaceutical properties. Vitamins, minerals and purified food substances are allowed in amounts not known to be associated with adverse effects (such as hypervitaminosis).

A participant who is receiving a permitted concomitant medication for any reason must be on a locally approved medication and dose for the treated indication, and this must be documented on the CRF.

Participants are not allowed any other investigational drugs or treatments during the study.

Participants should refrain from starting new or changing doses of permitted prescription or non-prescription drugs, vitamins, and dietary supplements within 7 days or 5 half-lives (whichever is longer) prior to Day 1 and prior to study visits throughout the study, unless otherwise noted below.

Hormonal contraceptives that meet the requirements of this study are allowed to be used in participants who are WOCBP (see Appendix 4).

Participants should report any changes to permitted medications during the study to the investigator as soon as they occur. Medication changes must be documented in the participant's record and CRF.

Unless a prohibited medication or treatment, participants may be administered any other medications necessary for the treatment of concomitant medical disorders as deemed necessary by the treating physician. Following Day 1, addition of concomitant medications or any change in the dosage should be limited to those considered medically essential.

Permitted concomitant CYP3A substrates are listed in Table 2.

Table 2. Permitted Concomitant CYP3A Substrates[a]

Sensitive CYP3A Substrates[b]		Moderate Sensitive CYP3A Substrates[c]
Alfentanil	Midazolam	Alprazolam
Avanafil	Naloxegol	Atorvastatin
Buspirone	Quetiapine	Colchicine
Darifenacin	Sildenafil	Rivaroxaban

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Table 2. Permitted Concomitant CYP3A Substrates[a]

Sensitive CYP3A Substrates[b]		Moderate Sensitive CYP3A Substrates[c]
Eletriptan	Simvastatin	Tadalafil
Eplerenone	Ticagrelor	
Felodipine	Triazolam	
Lovastatin	Vardenafil	
Lurasidone		

a. Multiple doses of 200 mg once daily ritlecitinib increased the AUC_{inf} and C_{max} of midazolam, a CYP3A4 substrate, by approximately 2.7-fold and 1.8-fold, respectively. Ritlecitinib is a moderate inhibitor of CYP3A; caution should be exercised with concomitant use of ritlecitinib with CYP3A substrates where small concentration changes may lead to serious adverse reactions. Dose adjustment recommendations for the CYP3A substrate in accordance with approved product labelling should be considered.
b. Sensitive CYP3A substrates are drugs that demonstrate an increase in AUC of ≥5-fold with strong index inhibitors of a given metabolic pathway in clinical DDI studies.
c. Moderate sensitive substrates are drugs that demonstrate an increase in AUC of ≥2 to <5-fold.

Permitted concomitant CYP1A2 substrates are listed in Table 3.

Table 3. Permitted Concomitant CYP1A2 Substrates[a]

Sensitive CYP1A2 Substrates[b]	Moderate Sensitive CYP1A2 Substrates[c]
Alosetron	Clozapine
Duloxetine	Theophylline
Melatonin	Olanzapine
Ramelteon	Rasagiline
Tasimelteon	Ropivacaine
Tizanidine	Pirfenidone
	Roflumilast

a. Multiple doses of 200 mg once daily ritlecitinib increased the AUC_{inf} and C_{max} of caffeine, a CYP1A2 substrate, by approximately 2.7-fold and 1.1-fold, respectively. These increases in caffeine exposure are not considered clinically relevant. Ritlecitinib is a moderate inhibitor of CYP1A2; caution should be exercised with concomitant use of ritlecitinib with other CYP1A2 substrates where small concentration changes may lead to serious adverse reactions. Dose adjustment recommendations for the CYP1A2 substrate in accordance with approved product labelling should be considered.
b. Sensitive CYP1A2 substrates are drugs that demonstrate an increase in AUC of ≥5-fold with strong index inhibitors of a given metabolic pathway in clinical DDI studies.
c. Moderate sensitive substrates are drugs that demonstrate an increase in AUC of ≥2 to <5-fold.

6.9.2. Prohibited Concomitant Medications

Participants will abstain from all prohibited medications as described in Section 6.9.2.1. Medically necessary medications should not be discontinued without prior evaluation of acceptable alternatives, including consultation with prescribing health professional.

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Participants should be instructed at each visit to contact the study site investigator promptly if there are any intended changes or additions to concomitant medications.

All medications and treatments that are known to affect AA must be discontinued before Day 1 of the study.

During the study, if it is discovered that a participant has been taking a medication or treatment that could affect AA, the investigator should contact the sponsor for each case to determine whether the participant should be discontinued.

6.9.2.1. Prohibited Concomitant Medications/Treatments

Any of the following medications and treatment regimens specified in the timeframes outlined below are prohibited for use during the study.

NOTE: Examples provided do not represent an all-inclusive list of medications and treatments. If there is a question about whether a particular medication is prohibited, the medication should be discussed with the sponsor.

a. Within 1 week of first dose of study intervention:

   - Herbal medications with either unknown properties or pharmaceutical properties that impact PK.
   - Prohibited CYP3A substrates as described in Appendix 9 (within 7 days or 5 half-lives, whichever is longer).

b. Within 2 weeks of the first dose of study drug: topical treatments that could affect AA on areas under assessment (ie, scalp, eyebrows, eyelashes) (eg, steroid cream, medicated shampoo, minoxidil).

c. Within 4 weeks of the first dose of study intervention or 5 half-lives (if known), whichever is longer: prohibited CYP3A inducers as described in Appendix 9.

d. Within 4 weeks of the first dose of study drug: phototherapy (eg, UVB phototherapy, PUVA).

e. Within 4 weeks of the first dose of study drug: contact immunotherapy (eg, DPCP, SADBE, and DNCB).

f. Within 4 weeks of the first dose of study drug: topical irritants (eg, anthralin) and liquid nitrogen cryotherapy.

g. Within 6 weeks of the first dose of study intervention: vaccination with live attenuated replication competent vaccine.

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h. Within 8 weeks of first dose of study intervention or within 5 half-lives (if known), whichever is longer:

- Immunosuppressants (eg, cyclosporine A, azathioprine, MTX, sulfasalazine, MMF, everolimus, ibrutinib).
- Oral or injectable steroids (including intralesional injections).

i. Within 8 weeks of first dose of study drug: 5-ARIs (eg, finasteride, dutasteride), oral minoxidil, spironolactone if used specifically for AA.

j. Within 12 weeks of first dose of study intervention or 5 half-lives (if known), whichever is longer:

- Any JAK inhibitor for use in any disease indication (including topical preparation).
- Immunomodulatory biologic agents.

Note: Discontinuation of any JAK inhibitor due to a treatment-related AE is exclusionary.

k. Within 6 months of first dose of study intervention or 5 half-lives (if known), or until lymphocyte count returns to normal, whichever is longer: any B-cell-depleting agents, including but not limited to rituximab.

l. At any time: previous use of any non-B-cell selective lymphocyte-depleting agent (eg, alefacept, alemtuzumab).

This list of drugs prohibited for potential DDI concerns with the IMP may be revised during the course of the study with written notification from the sponsor to include or exclude specific drugs or drug categories for various reasons (eg, emerging DDI results for the IMP, availability of new information in literature on the DDI potential of other drugs) if the overall benefit/risk assessment is not impacted or if the changes do not significantly impact the safety of participants or the scientific value of the trial.

For participants who discontinue study intervention prior to week 24 and continue in the study for efficacy and safety assessments, the use of prohibited medications mentioned in Section 6.9.2.1 are allowed 28 days after the last dose of study intervention.

6.9.3. Vaccinations

Vaccination with live attenuated replication-competent vaccines is prohibited within the 6 weeks prior to the first dose of study intervention, while receiving study intervention, and for 6 weeks after the last dose of study intervention.

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Similarly, current routine household contact with individuals who have been vaccinated with live attenuated, replication-competent vaccines should be avoided while receiving study intervention and for 6 weeks after the last dose of study intervention.

Following vaccination with a live attenuated replication-competent vaccine, the virus may be shed in bodily fluids, including stool, and there is a potential risk that the virus may be transmitted.

Such vaccines include but are not limited to: FluMist® (intranasal influenza vaccine), attenuated rotavirus vaccine, varicella (chickenpox) vaccine, attenuated typhoid fever vaccine, oral polio vaccine, MMR vaccine, vaccinia (smallpox) vaccine, and Zostavax® (zoster vaccine live).

Live attenuated vaccines that are known not to be replication-competent in humans are permitted. Such vaccines include, but are not limited to, the Modified Vaccinia Ankara Bavarian Nordic (Jynneos®, Imvamune®, Imvanex®) smallpox and monkeypox vaccine. By contrast, the ACAM2000 smallpox and monkeypox vaccine is prohibited because it is live and replicates in humans.

Vaccines (including COVID-19 vaccines) that are not live attenuated are permitted.

Individuals receiving immunosuppressive therapy may have a diminished response to vaccination.

Vaccines (including COVID-19 vaccines) that are not live attenuated are permitted.

6.9.4. Elective Surgery

During the study, no elective surgery should occur without first consulting with the Pfizer medical monitor. Preferably, elective surgery should occur before the study or be delayed until participation in the study is completed.

The Pfizer medical monitor should be notified if a participant requires surgery (including dental surgery) during the study to determine whether the participant should discontinue from the study and/or discontinue study intervention prior to the surgical procedure. The Pfizer medical monitor should be notified as soon as possible if a participant undergoes a surgical procedure without first informing the study staff.

Participants who require elective surgery should temporarily discontinue study intervention for one week prior to the surgical procedure and remain off study intervention after the surgical procedure until sutures/staples are removed. If absorbing sutures or chemical closure methods are utilized, study intervention can be resumed when the operative site is sufficiently healed, and risk of infection is minimal. If the participant interrupts study intervention for >14 consecutive days, this must be discussed with the sponsor for possible withdrawal from the study intervention.

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7. DISCONTINUATION OF STUDY INTERVENTION AND PARTICIPANT DISCONTINUATION/WITHDRAWAL

7.1. Discontinuation of Study Intervention

It may be necessary for a participant to permanently discontinue study intervention. Discontinuation of study intervention does not represent withdrawal from the study. Participants who are withdrawn early from study intervention for any reason will remain in the study and continue the regularly scheduled visits through Week 24 and follow-up visit.

Reasons for permanent discontinuation of study intervention include the following:

7.1.1. Adverse Events

- Serious infections, defined as any infection (viral, bacterial, and fungal) requiring parenteral antimicrobial therapy or hospitalization for treatment or meeting other criteria that require the infection to be classified as SAE
- Treatment-related SAEs
- Clinically meaningful, treatment emergent declines in hearing from baseline are to be discussed with the sponsor for possible withdrawal from study intervention
- Other serious or severe AEs, at the discretion of the investigator or sponsor

7.1.2. Study Intervention Interruptions

Participants interrupting study intervention for more than 14 consecutive days are to be discussed with the sponsor for possible withdrawal from the study intervention.

7.1.3. Laboratory Abnormalities

All the following laboratory abnormalities require discontinuation from study intervention if they are confirmed by re-test. Refer to the re-testing timeframes for laboratory abnormalities in Section 8.3.3.2.

- ANC <750/mm^3 (<0.75 × 10^9/L)
- Hemoglobin <9.0 g/dL (<5.59 mmol/L or <90 g/L) or a decrease of >30% from baseline (either criterion or both).
- Platelet count <75,000/mm^3 (<75.0 × 10^9/L)
- ALC <500/mm^3 (<0.5 × 10^9/L)

NOTE: Participants with absolute lymphocyte count <500/mm^3 (0.5 × 10^9/L) will be reflex tested for FACS TBNK until the absolute lymphocyte count resolves or stabilizes at a level acceptable to the investigator and sponsor.

- CK >10 × ULN

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NOTE: In addition to re-testing CK >3 × ULN, any participant with CK >10 × ULN will have local urine dipstick testing for blood performed at the time of the CK retest. If indicated, additional testing of the urine sample will be performed at the central lab (see Appendix 2 and the laboratory manual for details).

Discontinuation criteria based on AST, ALT and bilirubin are summarized in Section 7.1.9 - Liver Injury.

7.1.4. Prohibited Medications

Participants who are treated with any prohibited medication during the study may require discontinuation.

Participants who are administered prohibited medication are to be discussed with the sponsor for possible withdrawal from the study intervention.

7.1.5. Suicidal Ideation

Participants with suicidal ideation associated with actual intent and a method or plan since last visit (a "Yes" answer on items 4, 5 of C-SSRS) or "Yes" answer to any of the suicidal behavior items of the C-SSRS must be discontinued.

The participant must be referred to a mental health professional for appropriate evaluation and treatment or immediately referred to the Emergency Room at the investigator's judgement. A summary of risk assessment should be documented in the participant's source document/medical record.

If a participant presents with suicidal ideation/behavior at any time during the study, the participant's parent(s) or legal guardian should be informed.

7.1.6. Protocol Deviation

Study intervention must be permanently discontinued and the EOT Visit completed for protocol deviations such as (but not limited to):

- Use of prohibited medication
- Not meeting all eligibility criteria (eg, incidental monitoring findings during phone calls)
- Treatment over or under-compliance

7.1.7. Withdrawal of Consent/Assent

Participants may withdraw from the study at any time. If the consent/assent is withdrawn during the treatment period, an EOT visit must be completed. If the withdrawal is complete, from all study procedures, an EOS visit study must be conducted at the same time.

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In the event of discontinuation of study intervention, it must be documented on the appropriate CRF/in the medical records whether the participant is discontinuing further receipt of study intervention or also from study procedures, further study follow-up, and/or future collection of additional information.

7.1.8. Potential Cases of Acute Kidney Injury

Participants exposed to IMP demonstrating transient or sustained increase in Screat (with decrease in Screat-based eGFR or eCrCl) require expedited evaluation to differentiate AKI from DICI. DICI is defined as transporter-mediated effect related to altered renal tubular creatinine handling without histological injury.

AKI may be due to one or more types of injury, including DIKI. Differentiation of DIKI from other causes of AKI and from DICI may require clinical, radiographic, histopathologic, and laboratory assessments, as well as nephrology consultation.

Follow-Up Assessments

The participant should return to the site for evaluation as soon as possible, preferably within 48 hours of awareness of the abnormal results.

Evaluation should include physical examination, laboratory tests, detailed medical and surgical history, review of all medications (including recreational drugs and supplements [herbal]), family history, sexual history, travel history, blood transfusion, and potential occupational exposure to chemicals.

Laboratory assessments should include simultaneous Scys and Screat tests. Estimates of eGFR, eCrCl and Screat-based eGFR and combined Screat-Scys-based eGFR should also be derived using the appropriate equation described in Appendix 7.

Assessments of urine albumin-to-creatinine ratio or urine volume may also be performed as appropriate.

If appropriate, nephrology consultation may be recommended to facilitate differentiation of renal parenchymal disease, pre-renal azotemia, and post-renal obstruction.

Differentiating Acute Kidney Injury from DICI

A confirmed Screat increase is defined as:
(i) ≥0.3 mg/dL (≥26.5 µmol/L) within 48 hours OR
(ii) confirmed Screat increase ≥1.5 times baseline (known or suspected to have occurred within the prior 7 days).

Based on the assessments performed, suspected AKI (including DIKI) may be differentiated from DICI as follows.

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	AKI (including DIKI) **Any one of the below**	**DICI**
Scys & Screat	Simultaneous, confirmed Scys increase and confirmed Screat increase	Confirmed Screat increase without confirmed increase in reflex Scys
Albuminuria	Confirmed albuminuria increase (see Appendix 7 for Grades A1 to A3 quantitation)	
Urine volume	Urine volume <0.5 mL/kg/h for 6 consecutive hours	

Regardless of the presence or absence of increase in Screat, DIKI and other causes of AKI may be suspected if new-onset or worsening albuminuria or proteinuria are detected.

All confirmed cases of clinically relevant decrease in kidney function should be considered potential cases of DIKI if no other reason for the kidney function abnormalities has been found.

7.1.9. Liver Injury

AST or ALT that meet ANY of the following:

- >3 times the upper limit of normal with at least one total bilirubin value >2 times the upper limit of normal
- >3 times the upper limit of normal accompanied by signs or symptoms consistent with hepatic injury (eg, new onset elevated PT/INR)
- Two sequential AST or ALT elevations >5 times the upper limit of normal, regardless of total bilirubin or accompanying signs or symptoms

All the above laboratory abnormalities require discontinuation from study intervention if they are confirmed by re-test. Refer to the re-testing timeframes for laboratory abnormalities in Section 8.3.3.2.

NOTE: In each case, there is a need for additional investigations, such as review of ethanol, recreational drug and dietary supplement consumption; testing for acute hepatitis A, B or C infection and biliary tract imaging should be promptly discussed with the sponsor or designee. Refer to Appendix 6 for additional details.

NOTE: See Appendix 6 for thresholds of AST, ALT, and total bilirubin laboratory abnormalities corresponding to potential cases of DILI and actions and timeframes for

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follow-up in the event of potential DILI. These cases should be promptly discussed with the sponsor or designee.

7.1.10. Pregnancy

In case of positive urine pregnancy test, the participant will have study intervention interrupted, and a serum sample collected on the same day (or as soon as possible) and submitted to the central laboratory for pregnancy testing. Female participants confirmed as pregnant during the study must permanently discontinue study intervention.

7.1.11. Other Reasons for Discontinuation

These include reasons such as death, study termination by sponsor, physician decision, medication error, and lost to follow-up.

7.2. Participant Discontinuation/Withdrawal From the Study

A participant may withdraw from the study at any time at their own request. Reasons for discontinuation from the study include the following:

- Death
- Lost to follow-up
- Physician decision
- Study terminated by sponsor
- Withdrawal of consent/assent by parent/legal guardian or by an adolescent participant who has provided assent during any phase of the study

At the time of discontinuing from the study, if possible, an EOT should be conducted. See the SoA for assessments to be collected at the time of study discontinuation and follow-up and for any further evaluations that need to be completed.

If participation in the study and treatment are discontinued at the same time, the EOT and EOS visits will be conducted together.

If a participant withdraws from the study, they may request destruction of any remaining samples taken and not tested, and the investigator must document any such requests in the site study records and notify the sponsor accordingly.

If the participant withdraws from the study and also withdraws consent/assent (see Section 7.2.1) for disclosure of future information, no further evaluations will be performed and no additional data will be collected. The sponsor may retain and continue to use any data collected before such withdrawal of consent/assent.

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7.2.1. Withdrawal of Consent/Assent

Participants who request to discontinue receipt of study intervention will remain in the study and must continue to be followed for protocol-specified follow-up procedures. The only exception to this is when a participant specifically withdraws consent/assent for any further contact with them or persons previously authorized by the participant to provide this information. Participants or the participant's parent/legal guardian/caregiver should notify the investigator in writing of the decision to withdraw consent/assent from future follow-up, whenever possible. The withdrawal of consent/assent should be explained in detail in the medical records by the investigator, as to whether the withdrawal is only from further receipt of study intervention or also from study procedures and/or posttreatment study follow-up, and entered on the appropriate CRF page. In the event that vital status (whether the participant is alive or dead) is being measured, publicly available information should be used to determine vital status only as appropriately directed in accordance with local law.

7.3. Lost to Follow-Up

A participant will be considered lost to follow-up if the participant repeatedly fails to return for scheduled visits and is unable to be contacted by the study site.

The following actions must be taken if a participant fails to return to the clinic for a required study visit:

- The site must attempt to contact the participant and reschedule the missed visit as soon as possible. Counsel the participant on the importance of maintaining the assigned visit schedule, and ascertain whether the participant wishes to and/or should continue in the study;

- Before a participant is deemed lost to follow-up, the investigator or designee must make every effort to regain contact with the participant (where possible, 3 telephone calls and, if necessary, a certified letter to the participant's last known mailing address or local equivalent methods). These contact attempts should be documented in the participant's medical record;

- Should the participant continue to be unreachable, the participant will be considered to have withdrawn from the study.

8. STUDY ASSESSMENTS AND PROCEDURES

Efforts will be made to minimize missing data in the study, including detailing the steps to prevent missing data in CRF completion guidelines, training the investigators on importance of retention of participants in the study, including a statement in consent forms to inform study participants on the scientific importance of their data and of remaining in the study even if they discontinue study intervention early. Per study protocol, participants discontinuing early from study intervention will remain in the study (off study intervention) through the end of the 24-week treatment period and follow-up visit for applicable safety and efficacy assessments.

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8.1. Administrative Procedures

The investigator (or an appropriate delegate at the investigator site) must obtain an assent from the participant and a signed and dated ICD from the participant's parent/legal guardian/caregiver before performing any study-specific procedures.

Study procedures and their timing are summarized in the SoA. Protocol waivers or exemptions are not allowed.

Adherence to the study design requirements, including those specified in the SoA, is essential and required for study conduct.

All screening evaluations must be completed and reviewed to confirm that potential participants meet all eligibility criteria. The investigator will maintain a screening log to record details of all participants screened and to confirm eligibility or record reasons for screening failure, as applicable.

Every effort should be made to ensure that protocol required tests and procedures are completed as described. However, it is anticipated that from time to time there may be circumstances outside the control of the investigator that make it unfeasible to perform the test. In these cases, the investigator must take all steps necessary to ensure the safety and well-being of the participant. When a protocol-required test cannot be performed, the investigator will document the reason for the missed test and any corrective and preventive actions that they have taken to ensure that required processes are adhered to as soon as possible. The study team must be informed of these incidents in a timely manner.

- Immediate safety concerns should be discussed with the sponsor immediately upon occurrence or awareness to determine if the participant should continue or discontinue study intervention.

- To assure consistency and reduce variability, visits should occur at approximately the same time of day throughout the study. Participants should be encouraged to attend visits in the morning and prior to the participant's dosing of study intervention as participants will receive their dose at the clinic during their study visit.

- Urine pregnancy test must be performed at each visit and every month with at-home testing (and must be negative) prior to dosing with the study intervention for females ≥8 years of age through follow-up (see Appendix 4 and Section 8.3.7).

- The patient, observer and clinician-reported outcome assessments should be completed before the other evaluations or treatments at the clinical visits whenever it is possible. Vital signs should be performed before any laboratory blood collection. All other evaluations (unless noted otherwise) do not need to be performed in any specific order.

- In certain situations, sponsor review of redacted copies of participant medical records (eg, for safety monitoring, follow-up of AEs or laboratory abnormalities, review of

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local laboratory results) may be performed, when ethically and scientifically justified and permitted by local regulations, to ensure participant safety. See Section 10.1.9 for details on the use of medical records.

Safety results that have been collected for the purposes of this study and could unblind the study will not be reported to investigative sites or other blinded personnel until the study has been unblinded.

For samples being collected and shipped, detailed collection, processing, storage, and shipment instructions and contact information will be provided to the investigator site prior to initiation of the study.

The total blood sampling volume for individual participants in this study is approximately 61 mL for the whole study. The actual collection times of blood sampling may change. Additional blood samples may be taken for safety assessments at times specified by Pfizer, provided the total volume taken during the study does not exceed 46 mL during any 4-week period.

TBV is equal to the participant's weight in kg multiplied by the estimated mL/kg (80-90 ml/kg for children). EU and WHO guidelines suggest that no more than 1% of TBV should be drawn in a 24-hour period and no more that 3% of TBV should be drawn over 4 weeks. For example, the TBV of a 6-year-old child at approximately the 5th percentile for weight, weighing 17 kg, is 1,360-1,530 mL. Per the criteria of the EU and WHO guidelines, total blood sampling volume limits for this child are 14 - 15 mL of blood in 24 hours and 41 - 46 mL of blood in 4 weeks.

Every effort will be made to limit the amount of blood collected from each participant over the duration of the study. Repeat or unscheduled samples are allowed based on clinical judgement for participant safety or for technical issues with the samples.

Psychological assessments will be conducted as described in the SoA.

Diagnosis of AA must be performed by a qualified dermatologist (board certified or equivalent) who has experience in treating adult and/or pediatric patients with AA. Severity of alopecia will also be assessed at screening (SALT) and documented in the CRF.

8.1.1. Baseline Procedures

All procedures listed in the SoA must be conducted at this visit.

8.1.2. Telehealth Visits

Telehealth visits may be used to assess participant safety and collect data points. Telehealth includes the exchange of healthcare information and services via telecommunication technologies (eg, audio, video, videoconferencing software) remotely, allowing the participant and the investigator to communicate on aspects of clinical care, including medical

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advice, reminders, education, and safety monitoring. Assessments that may be performed during a telehealth visit are described in the SoA and are as follows:

- Review and record study intervention(s), including compliance and missed doses.
- Review and record any AEs and SAEs since the last contact. Refer to Section 8.4.
- Review and record any new concomitant medications or changes in concomitant medications since the last contact.
- Review and record contraceptive method and results of pregnancy testing. Confirm that the participant is adhering to the contraception method(s) required in the protocol. Refer to Appendix 4.

Study participants must be reminded to promptly notify site staff about any change in their health status.

8.2. Efficacy Assessments

8.2.1. Rater Qualifications

For specific rating assessments, only qualified raters will be allowed to evaluate and/or rate participants in this study. The minimum qualifications a rater must meet for each study rating assessment will be outlined in the study manual provided to each participating site. The level of experience with the target population (or equivalent), specific scale experience (or equivalent), and certification required (if applicable) will be listed and used to determine whether a rater is approved for a given assessment. The rater must become certified to perform selected study assessments before they can participate in the conduct of the study. For specifically defined assessments, rater training and standardization exercises may be conducted, and written documentation will be provided by the site for each rater's certification. In return, each site will be provided written documentation outlining each rater's certification for specific study assessments. Recertification may be required at periodic intervals during the study. The raters who administer specific study assessments will be documented in a centralized location and all site staff who administer ratings will be verified in the site study documentation during the conduct of the study.

An experienced and qualified physician or healthcare professional may be permitted to perform the clinical evaluations of alopecia (ie, SALT, ELA, EBA, the C-SSRS suicidal ideation and behavior risk assessment, the Acceptability and Palatability Questionnaire, WISC-V). The Acceptability and Palatability Questionnaire is a simple assessment that can be conducted by the study coordinator or designee.

All other assessments require experienced personnel, with skills in interacting with children.

For C-SSRS, specific documented training is required.

For C-SSRS assessments, only qualified raters will be allowed to evaluate and/or rate participants in this study (see Appendix 11 for details).

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Additional details are provided in specific appendices and study manual, for each assessment.

8.2.2. Clinical Assessments

8.2.2.1. Severity of Alopecia Tool (SALT)

SALT is a quantitative assessment of alopecia severity based on scalp terminal hair loss [38]. The overall SALT score, which does not distinguish the reason for hair loss, will be collected at visits shown in the SoA.

Score parameters utilize a visual aid showing the division of the scalp hair into 4 quadrants (back, top of scalp, and both sides), with each of the 4 quadrants given an accurate determination of the % of scalp surface area covered, representing 24%, 40%, 18%, and 18% of the total scalp surface area, respectively.

Hair prosthetics (eg, wigs, hair extensions) must be removed for clinical assessments of AA at all study visits.

The assessment will be performed by a qualified dermatologist (board certified or equivalent) who has experience with AA or a delegated member of the study staff (an experienced and qualified physician or healthcare professional).

8.2.2.2. Eyelash Assessment (ELA)

ELA is an NRS developed to characterize eyelash hair loss. The numeric rating scale ranges from 0 (none) to 3 (normal) as described below. The ELA should be completed as per the SoA. ELA will be assessed in the study in those participants who have an abnormal ELA score (defined as an ELA score of 0, 1 or 2) at baseline. Refer to Section 5.3.3 for restrictions.

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Score	Description
0	None Eyelash No eyelashes of both right and left upper and lower eyelashes.
1	Minimal Eyelash Modestly or severely decreased density of and/or large gap(s) in one or both upper eyelashes.
2	Moderate Eyelash • Normal density of both upper eyelashes without gap(s), and decreased density or gap(s) is present in one or both lower eyelashes, OR • Normal density of both upper eyelashes with short gap(s), OR • Mildly decreased density of one or both upper eyelashes with or without short gap(s).
3	Normal Eyelash Normal density of both right and left upper and lower eyelashes from near medial canthus to near lateral canthus without any gap(s).

NOTE:
Density of lower eyelashes is usually less than upper eyelashes.
A short gap does not significantly distort the appearance of the eyelash(es).
Moderate Eyelash score does not require presence of lower eyelashes.

8.2.2.3. Eyebrow Assessment (EBA)

The EBA is an NRS developed to characterize eyebrow hair loss. The numeric rating scale ranges from 0 (none) to 3 (normal). The EBA should be completed as per the SoA. EBA will be assessed in the study in those participants who have an abnormal EBA score (defined as an EBA score of 0, 1 or 2) at baseline.

Score	Description
0	None Eyebrow No eyebrow hair.
1	Minimal Eyebrow • Normal or decreased density of one or both eyebrows with large gap(s). • Severely decreased density of one or both eyebrows with or without gap(s).

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Score	Description
2	Moderate Eyebrow • Normal density of both eyebrows with short gap(s) that does not significantly distort the appearance of the eyebrows, OR • Mildly decreased density of eyebrows with or without short gap(s), OR • Moderately decreased density of eyebrows without short gap(s). There is visual definition of eyebrows at a distance of 3 feet.
3	Normal Eyebrow Normal density of both right and left eyebrows spanning usual length (ie, from glabella to near temple) and width. There are no gap(s).

NOTE: Density of lateral aspect of eyebrows may be mildly less than medial eyebrows.
A short gap does not significantly distort the appearance of the eyebrow(s).

8.2.3. Patient/Observer Reported Outcomes

Participants and/or caregivers will report self-assessment outcomes during specified visits outlined in the SoA. Every effort should be made to have the participant complete all PRO questionnaires before any other evaluations.

8.2.3.1. Patient's Global Impression of Change (PGI-C)

PGI-C asks the participant to evaluate the improvement or worsening of their AA as compared to the start of the study using a single-item, "Since the start of the study, my alopecia areata has: …". The participants will select one of seven responses ranging from "greatly improved" to "greatly worsened." Participants should refer to changes from the start of this study through the end of study. The PGI-C should be completed as per the SoA.

8.2.3.2. Alopecia Areata Patient Priority Outcomes (AAPPO)

The AAPPO measure is a validated 11-item self-administered questionnaire that measures hair loss, emotional symptoms, and activity limitations over the past week. The first 4 items of the tool, which cover hair loss from the scalp, eyebrows, eyelashes, and body, ask the patient to describe the current amount of hair loss using a 5-point response scale that ranges from "no hair loss" to "complete (do not have any [insert body location/type of hair])." Items 5-8 are an assessment of emotional symptoms due to hair loss. Response choices on these items are scored from "0=never" to "4=always." Items 9-11 are an assessment of activity limitations due to hair loss, with response choices scored from 0="not at all" to 4="completely (did not do any [insert activity] because of hair loss)". The AAPPO should be completed as per the SoA.

8.2.3.3. PROMIS Parent Proxy - Anxiety Short Form 8a and Depressive Symptoms Short Form 6a

The PROMIS® is a standardized PRO measurement system which covers multiple domains of health including Physical Function, Emotional Distress, Social Role Relationships,

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Fatigue, Pain, and others. Within the Emotional Distress domain, there are components that cover Anxiety and Depressive Symptoms. For all PROMIS measures, the T-score rescales the raw score into a standardized score with a mean of 50 and a SD of 10. Therefore, a person with a T-score of 60 is one SD above the pediatric population mean, and a T-Score of 70 is two SDs above the pediatric population mean of 50. PROMIS T-scores 70 or greater are considered severe dysfunction; T-scores from 60 to 69 are considered moderate dysfunction [39].

The PROMIS Parent Proxy Short Form v3.0 – Anxiety 8a is an 8-item measure of anxiety. Each item has five response options ranging in value from 1 to 5. Raw scores are calculated by summing the values of the response to each question. The lowest possible raw score for this measure is 8 and the highest possible raw score is 40. Raw scores are transformed to T-scores using a conversion table available from the developer [39].

The PROMIS Parent Proxy Short Form v3.0 – Depressive Symptoms 6a is a 6-item measure of depressive symptoms. Each item has five response options ranging in value from 1 to 5. Raw scores are calculated by summing the values of the response to each question. The lowest possible raw score for this measure is 6 and the highest possible raw score is 30. Raw scores are transformed to T-scores using a conversion table available from the developer [39].

The PROMIS-Anxiety and PROMIS-Depressive Symptoms measures should be completed as per the SoA.

8.2.3.4. Behavior Rating Inventory of Executive Function®, Second Edition (BRIEF®2)

The BRIEF®2, that will be used in this study is an ObsRO parent/caregiver completed scale designed to assess executive behaviors in children. The BRIEF®2 will be used in this study to evaluate neuropsychological development and executive functions in children 6 to <12 years of age. The BRIEF®2 scales assess how well the child is demonstrating behavioral or emotional control, initiating activity, engaging in planful and well-organized problem solving, and monitoring one's own social success and problem-solving outcomes.

The BRIEF®2 consists of 63 items and yields scores on nine scales, with the Inhibit and Self-Monitor scales comprising the BRI, the Shift and Emotional Control scales comprising the ERI, and the Initiate, Working Memory, Plan/Organize, Task Monitor, and Organization of Materials scales comprising the CRI. The questionnaire will be completed by the parent/caregiver of the participant [40].

The BRIEF®2 should be completed as per the SoA.

8.2.3.5. Modified Children's Dermatology Life Quality Index (CDLQI)

The CDLQI is a general dermatology questionnaire designed to measure the impact of skin diseases on children's quality of life. The CDLQI is a children's version of the DLQI [41]. The CDLQI may be completed by children as young as 4 years old. The CDLQI consists of 10 items that assess participant health related quality of life (daily activities, personal relationships, symptoms and feelings, leisure, school, and treatment). Permission was granted

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by the developers of the CDLQI to modify the items to refer to “alopecia areata” instead of “skin” or “skin trouble” to avoid confusing respondents with AA.

Each item is scored from 0 to 3. Questions left unanswered are given a score of 0. Item scores are summed and expressed as usual out of a maximum score of 30.

The questionnaires are self-explanatory and can be handed to the participant who is asked to fill them in with the help of the child’s parent or legal guardian, as necessary.

The Modified CDLQI should be completed as per the SoA.

8.2.3.6. Wechsler Intelligence Scale for Children - 5th edition

The WISC-V is a clinical measure of cognitive ability/intelligence of children 6 to 16 years of age. It will be used to assess cognitive development in study participants. Responses from the WISC-V can be used to calculate scores for intellectual functioning in several cognitive domains, as well as a composite score that represents the general intellectual ability.

The WISC-V is composed of multiple subtests (primary, secondary and complementary) and index scales, with each subtest grouped into 3 general categories of index scales: primary, ancillary, and complementary.

Administration of the primary subtests is recommended for a comprehensive description of intellectual ability. The secondary and complementary subtests can be administered in addition to the primary subtests to provide a broader sampling of intellectual functioning and to yield more information.

This assessment will be administered by a qualified psychologist. Administration and Scoring Manual, Technical and Interpretive Manual will be provided to the sites [42].

The WISC-V should be completed as per the SoA in all countries where it is available. If there are exceptional individual circumstances (eg, cultural factors) such that a WISC-V assessment cannot be administered to a study participant, the site Investigator should contact the sponsor to determine if the participant is eligible for enrollment into the study. If the sponsor’s decision is that the participant can be enrolled without performing WISC-V assessments, the decision (as well as the reason/s for not performing the assessments) will be documented in the participant’s source documents/medical records and CRF.

8.3. Safety Assessments

Planned time points for all safety assessments are provided in the SoA. Unscheduled safety measurements may be obtained at any time during the study to assess any perceived safety issues.

8.3.1. Physical Examinations

A complete physical examination will include, at a minimum, assessments of the following: general appearance, skin, lymph nodes, eyes, ears, nose, throat, mouth, cardiovascular,

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respiratory, gastrointestinal, neurological and musculoskeletal systems. Height and weight will also be measured and recorded. Brief/targeted physical examination consists of skin, heart, lungs, abdomen, neurologic function, and examination of body systems where there are symptom complaints by the participant.

In addition, at baseline and EOT visits only, visual inspection of the breasts and external genitalia will be performed for the purpose of Tanner staging.

Investigators should pay special attention to clinical signs related to previous serious illnesses.

Complete and brief physical examinations are performed at various time points as per the SoA.

Physical examination findings collected during the study will be considered source record and will not be required to be reported, unless otherwise noted. Any untoward physical examination findings that are identified during the active collection period and meet the definition of an AE or SAE (Appendix 3) must be reported according to the processes in Sections 8.4.1 to 8.4.3.

8.3.2. Vital Signs

Any untoward vital sign findings that are identified during the active collection period and meet the definition of an AE or SAE (Appendix 3) must be reported according to the processes in Sections 8.4.1 to 8.4.3.

8.3.2.1. Blood Pressure and Pulse Rate

BP and PR measurements will be assessed in the supine position with a completely automated device and using a pediatric cuff of the appropriate size. Manual techniques will be used only if an automated device is not available.

BP and PR measurements should be preceded by at least 5 minutes of rest with the participant in a supine position, in a quiet setting without distractions.

BP and PR will be taken before collection for laboratory tests and consist of a single BP measurement and single PR measurement.

If the BP reading is considered abnormal, a repeat BP measurement will be taken.

8.3.2.2. Temperature and Respiratory Rate

Temperature and respiratory rate findings collected during the study will be considered source record and will not be required to be reported, unless otherwise noted.

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8.3.3. Safety Monitoring

8.3.3.1. Rash

All potential treatment-related events of rash will be followed up until resolution or if the rash persists, by agreement with Pfizer.

Relevant medical history including any history of skin rash prior to enrollment in and during this study will be collected.

Participants reporting an unexplained skin rash will be examined by a dermatologist according to local guidelines for formal comprehensive skin evaluations. A swab (for microbiological assessment) of the affected area may be taken for culture and sensitivity to assess for any bacterial, fungal, or viral pathogens, if applicable.

Investigators may take appropriate photographs of the rash. The culture results, digital images, and any additional relevant test results will be forwarded to Pfizer for review within 30 days of receipt by the investigator.

Rash will be treated in accordance with relevant guidelines. Participants who experience serious or severe drug related rash may be discontinued from the study intervention at the discretion of the investigator who may consult with the sponsor medical monitor.

8.3.3.2. Monitoring Laboratory Values

The following laboratory abnormalities require re-testing until resolution or agreement with Pfizer:

Laboratory Variable	Laboratory Value	Re-testing Timeframe*
Hematology		
ANC	<1000/mm^3 (<1 × 10^9/L)	Within 1 week
Hemoglobin	<10.0 g/dL (<6.21 mmol/L or <100 g/L) OR Decrease of 2.0 g/dL from baseline	Within 1 week
Platelet count	<100,000/mm^3 (<100 × 10^9/L)	Within 1 week
ALC	<600/mm^3 (<0.6 × 10^9/L)	Within 1 week
Serum Chemistry		
Creatine Kinase	>3 × ULN	Within 1 week
Alanine transaminase	>3 × ULN**	Within 48 hours
Aspartate transaminase	>3 × ULN**	Within 48 hours
Total Bilirubin	>1.5 × ULN**	Within 48 hours

* Based on awareness of the abnormal results.
** Participants who present with the following laboratory abnormalities should be evaluated further as potential DILI (Hy's law) cases to definitively determine the etiology of the abnormal laboratory values:

- Participants with AST/ALT and TBili baseline values within the normal range who subsequently present with AST OR ALT values >3 × ULN AND a TBili value >2 × ULN with no evidence of hemolysis and an alkaline phosphatase value <2 × ULN or not available;

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Laboratory Variable	Laboratory Value	Re-testing Timeframe*

- For participants with baseline AST OR ALT OR TBili values above the ULN, the following threshold values are used in the definition mentioned above, as needed, depending on which values are above the ULN at baseline:
  - Pre-existing AST or ALT baseline values above the normal range: AST or ALT values >2 times the baseline values AND >3 × ULN; or >8 × ULN (whichever is smaller).
  - Pre-existing values of TBili above the normal range: TBili level increased from baseline value by an amount of at least 1 × ULN or if the value reaches >3 × ULN (whichever is smaller).
- Refer to Appendix 6 (Liver Safety) for additional details regarding thresholds of laboratory abnormalities corresponding to potential cases of DILI and actions and timeframes for follow-up in the event of potential DILI.

8.3.3.3. Neurological Evaluations

Physical examinations, including neurologic assessment, will be performed at screening to assess eligibility and throughout the study. Participants with clinically meaningful changes from baseline in neurologic signs or symptoms will be referred to a neurologist to undergo a formal neurologic evaluation. Treatment-emergent potential neurosafety events of interest will be reviewed by an external independent adjudication committee.

8.3.3.4. Neuropsychological and Cognitive Development

BRIEF®2 is an ObsRO parent/caregiver completed scale designed to assess executive behaviors in children and adolescents. BRIEF®2 scales assess how well the child is demonstrating behavioral or emotional control, initiating activity, engaging in planful and well-organized problem solving, and monitoring one's own social success and problem-solving outcomes.

Cognitive development will be evaluated by the WISC-V. Refer to Section 8.2.3.6 for additional details.

8.3.3.5. Growth and Development

Additional evaluations to assess growth and development in pediatric participants will be included based on their clinical relevance (eg. pubertal staging [Tanner scale], longitudinal height and weight assessments).

8.3.4. Audiological evaluation

Audiological evaluation will be performed at timepoints described in the SoA. All participants will have a full audiological evaluation at screening or within 8 weeks prior to Day 1.

All audiological results (audiological history, otoscopic examination, air and bone pure tone audiometry, speech audiometry, and immittance audiometry) must be available prior to Day 1 to assess eligibility. At subsequent visit (Week 24/EOT), updated audiological history, otoscopic examination, and air pure tone audiometry will be performed; based upon results, additional audiometry assessments may be required. Pediatric Audiometry Study Guide will be provided to the sites to perform audiological evaluations. All procedures must be performed as per the Pediatric Audiometry Study Guide.

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If multiple audiological evaluations are required, the participant should have the evaluations performed at the same evaluation center, if possible.

If there is a clinically meaningful, treatment-emergent decline in hearing resulting in an AE, the participant must be promptly evaluated by a medically qualified specialist to assess for possible causes. Evaluation results should then be discussed with the sponsor to determine if the participant should be discontinued from the study. Discontinued participants must be followed up off-treatment with appropriate testing at regular intervals, until hearing returns to baseline or is determined to be clinically stable.

8.3.5. Electrocardiograms

ECG evaluations will be performed at timepoints described in the SoA.

Standard 12-lead ECGs will be collected at times specified in the SoA section of this protocol using an ECG system that automatically calculates the HR and measures PR, QT, QTcF, and QRS intervals. All scheduled ECGs should be performed after the participant has rested quietly for at least 5 minutes in a supine position.

ECG values of potential clinical concern are listed in Appendix 8.

8.3.6. Clinical Safety Laboratory Assessments

See Appendix 2 for the list of clinical safety laboratory tests to be performed and the SoA for the timing and frequency. All protocol-required laboratory assessments, as defined in Appendix 2, must be conducted in accordance with the laboratory manual and the SoA. Unscheduled clinical laboratory measurements may be obtained at any time during the study to assess any perceived safety issues.

The investigator must review the laboratory report, document this review, and record any clinically significant changes occurring during the study in the AE section of the CRF. Clinically significant abnormal laboratory test findings are those that are not associated with the underlying disease, unless judged by the investigator to be more severe than expected for the participant's condition.

All laboratory tests with values considered clinically significant and abnormal during participation in the study or within 4 weeks after the last dose of study intervention should be repeated until the values return to normal or baseline or are no longer considered clinically significant by the investigator or study medical monitor.

If such values do not return to normal/baseline within a period of time judged reasonable by the investigator, the etiology should be identified, and the sponsor notified.

See Appendix 6 for suggested actions and follow-up assessments in the event of potential DILI.

See Appendix 7 for instructions for laboratory testing to monitor kidney function and reporting laboratory test abnormalities.

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8.3.6.1. Alternative Facilities for Clinical Safety Laboratory Assessment

Protocol-specified safety laboratory evaluations will be conducted at a central laboratory.

If a local laboratory is used, qualified study site personnel must order, receive, and review results. Site staff must collect the local laboratory reference ranges and certifications/ accreditations for filing at the site. Laboratory test results are to be provided to the site staff as soon as possible. The local laboratory reports should be filed in the participant's source documents/medical records. Relevant data from the local laboratory report should be recorded on the CRF.

8.3.7. Pregnancy Testing

A urine or serum pregnancy test is required at screening. Following screening, pregnancy tests may be urine or serum tests, and must have a sensitivity of at least 25 mIU/mL. Pregnancy tests will be performed in all females ≥8 years of age at the times listed in the SoA. Following a negative pregnancy test result at screening, appropriate contraception must be commenced, and a second negative pregnancy test result will be required at the baseline visit prior to starting ritlecitinib. Pregnancy tests will also be done whenever 1 menstrual cycle is missed during the active treatment period (or when potential pregnancy is otherwise suspected) and at the end of the study. Pregnancy tests may also be repeated if requested by IRBs/ECs or if required by local regulations.

Pregnancy screening test will be performed for all females ≥8 years of age. If, in the opinion of the investigator, pregnancy testing is not necessary for a participant ≥8 years of age, then the investigator should document why pregnancy screening was not necessary (eg, based on their level of sexual maturity). In such cases, menarchal status should be recorded in the Medical History CRF. Pregnancy screening test (urine) and regular urine pregnancy testing throughout the study (at timepoints defined in the protocol SoA) will be performed for all post-menarchal females.

If a female participant is <8 years of age at the time of screening and turns 8 years of age during the study, the investigator should perform urine pregnancy tests including "at-home" urine pregnancy tests for such participants at that visit and at all the remaining study visits. The site/investigator should carefully monitor participants' age during the study.

8.3.7.1. At-Home Pregnancy Testing

A home urine pregnancy testing kit with a sensitivity of at least 25 mIU/mL may be used by the participant to perform the test at home, if compliant with local regulatory requirements. The results of pregnancy testing and that the participant is adhering to the contraception method(s) required in the protocol should be confirmed. The pregnancy test outcome should be documented in the participant's source documents/medical records and relevant data recorded on the CRF.

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8.3.8. Suicidal Ideation and Behavior Risk Monitoring

Suicidal Ideation and Behavior will be assessed throughout the study using the C-SSRS version for pediatric patients 6 to 11 years old (Appendix 11).

The C-SSRS (pediatric version) will be completed by the study site staff.

At screening visit, a risk assessment must be done by a qualified mental health professional to assess whether it is safe for the participant to participate in the trial if the participant's screening information indicate any of the following:

- Suicidal ideation associated with actual intent and a method or plan at any time in their lifetime: "Yes" answer on items 4 or 5 of the C-SSRS.
- Previous history of suicidal behaviors at any time in their lifetime: a "Yes" answer to any of the suicidal behavior items of the C-SSRS.
- Any lifetime history of serious or recurrent suicidal behavior (non-suicidal self-injurious behavior is not a trigger for a risk assessment unless in the investigator's judgement it is indicated).

At screening, Day 1 and any post-baseline visits, if there is a "Yes" answer on items 4, 5, or on any question in the suicidal behavior section of the C-SSRS, the participant will be discontinued from the study and referred to a mental health professional for appropriate evaluation and treatment. If the participant cannot be seen by a mental health professional within 24 hours, then the participant should be sent to a local emergency room for psychiatric assessment. A summary of risk assessment should be documented in the participant's source document/medical record.

If a participant presents with suicidal ideation/behavior at any time during the study, the participant's parent(s) or legal guardian should be informed.

8.4. Adverse Events, Serious Adverse Events, and Other Safety Reporting

The definitions of an AE and an SAE can be found in Appendix 3.

AEs may arise from symptoms or other complaints reported to the investigator by the participant (or, when appropriate, by a caregiver, surrogate, or the participant's legally authorized representative), or they may arise from clinical findings of the investigator or other healthcare providers (clinical signs, test results, etc).

The investigator and any qualified designees are responsible for detecting, documenting, and recording events that meet the definition of an AE or SAE and remain responsible to pursue and obtain adequate information both to determine the outcome and to assess whether the event meets the criteria for classification as an SAE or caused the participant to discontinue the study intervention (see Section 7.1).

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During the active collection period as described in Section 8.4.1, each participant/parent/legal guardian/legally authorized representative will be questioned about the occurrence of AEs in a nonleading manner.

In addition, the investigator may be requested by Pfizer Safety to obtain specific follow-up information in an expedited fashion.

8.4.1. Time Period and Frequency for Collecting AE and SAE Information

The time period for actively eliciting and collecting AEs and SAEs ("active collection period") for each participant begins from the time the participant provides informed consent/assent, which is obtained before undergoing any study-related procedure and/or receiving study intervention, through and including a minimum of 28 calendar days, after the last administration of the study intervention.

Follow-up by the investigator continues throughout the active collection period and until the AE or SAE or its sequelae resolve or stabilize at a level acceptable to the investigator.

When a clinically important AE remains ongoing at the end of the active collection period, follow-up by the investigator continues until the AE or SAE or its sequelae resolve or stabilize at a level acceptable to the investigator and Pfizer concurs with that assessment.

For participants who are screen failures, the active collection period ends when screen failure status is determined.

If the participant withdraws from the study and also withdraws consent/assent for the collection of future information, the active collection period ends when consent/assent is withdrawn.

If a participant permanently discontinues or temporarily discontinues study intervention because of an AE or SAE, the AE or SAE must be recorded on the CRF and the SAE reported using the CT SAE Report Form.

Investigators are not obligated to actively seek information on AEs or SAEs after the participant has concluded study participation. However, if the investigator learns of any SAE, including a death, at any time after a participant has concluded study participation, and they consider the event to be reasonably related to the study intervention, the investigator must promptly report the SAE to Pfizer using the CT SAE Report Form or via PSSA.

8.4.1.1. Reporting SAEs to Pfizer Safety

All SAEs occurring in a participant during the active collection period as described in Section 8.4.1 are reported to Pfizer Safety immediately upon awareness and under no circumstance should this exceed 24 hours, as indicated in Appendix 3. The investigator will submit any updated SAE data to the sponsor within 24 hours of these data being available.

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8.4.1.2. Recording Nonserious AEs and SAEs on the CRF

All nonserious AEs and SAEs occurring in a participant during the active collection period, which begins after obtaining informed consent/assent as described in Section 8.4.1, will be recorded on the AE section of the CRF.

The investigator is to record on the CRF all directly observed and all spontaneously reported AEs and SAEs reported by the participant.

As part of ongoing safety reviews conducted by the sponsor, any nonserious AE that is determined by the sponsor to be serious will be reported by the sponsor as an SAE. To assist in the determination of case seriousness, further information may be requested from the investigator to provide clarity and understanding of the event in the context of the clinical study.

8.4.2. Method of Detecting AEs and SAEs

The method of recording, evaluating, and assessing causality of AEs and SAEs and the procedures for completing and transmitting SAE reports are provided in Appendix 3.

Care will be taken not to introduce bias when detecting AEs and/or SAEs. Open-ended and nonleading verbal questioning of the participant is the preferred method to inquire about AE occurrences.

8.4.3. Follow-Up of AEs and SAEs

After the initial AE or SAE report, the investigator is required to proactively follow each participant at subsequent visits/contacts. For each event, the investigator must pursue and obtain adequate information until resolution, stabilization, the event is otherwise explained, or the participant is lost to follow-up (as defined in Section 7.3).

In general, follow-up information will include a description of the event in sufficient detail to allow for a complete medical assessment of the case and independent determination of possible causality. Any information relevant to the event, such as concomitant medications and illnesses, must be provided. In the case of a participant death, a summary of available autopsy findings must be submitted as soon as possible to Pfizer Safety.

Further information on follow-up procedures is provided in Appendix 3.

8.4.4. Regulatory Reporting Requirements for SAEs

Prompt notification by the investigator to the sponsor of an SAE is essential so that legal obligations and ethical responsibilities toward the safety of participants and the safety of a study intervention under clinical investigation are met.

The sponsor has a legal responsibility to notify both the local regulatory authority and other regulatory agencies about the safety of a study intervention under clinical investigation. The sponsor will comply with country-specific regulatory requirements relating to safety reporting to the regulatory authority, IRBs/ECs, and investigators.

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Investigator safety reports must be prepared for SUSARs according to local regulatory requirements and sponsor policy and forwarded to investigators as necessary.

An investigator who receives SUSARs or other specific safety information (eg, summary or listing of SAEs) from the sponsor will review and then file it along with the SRSD(s) for the study and will notify the IRB/EC, if appropriate according to local requirements.

8.4.5. Environmental Exposure, Exposure During Pregnancy or Breastfeeding, and Occupational Exposure

Environmental exposure occurs when a person not enrolled in the study as a participant receives unplanned direct contact with or exposure to the study intervention. Such exposure may or may not lead to the occurrence of an AE or SAE. Persons at risk for environmental exposure include healthcare providers, family members, and others who may be exposed. An environmental exposure may include EDP, EDB, and occupational exposure.

Any such exposures to the study intervention under study are reportable to Pfizer Safety within 24 hours of investigator awareness.

8.4.5.1. Exposure During Pregnancy

An EDP occurs if:

- A female participant is found to be pregnant while receiving or after discontinuing study intervention.
- A female nonparticipant is found to be pregnant while being exposed or having been exposed to study intervention because of environmental exposure. Below are examples of environmental EDP:

  A female family member or healthcare provider reports that she is pregnant after having been exposed to the study intervention by ingestion, inhalation, or skin contact.

The investigator must report EDP to Pfizer Safety within 24 hours of the investigator's awareness, irrespective of whether an SAE has occurred. The initial information submitted should include the anticipated date of delivery (see below for information related to termination of pregnancy).

- If EDP occurs in a participant/participant's partner, the investigator must report this information to Pfizer Safety using the CT SAE Report Form/ an EDP Supplemental Form or via PSSA, regardless of whether an SAE has occurred. Details of the pregnancy will be collected after the start of study intervention and until 28 days after last dose.
- If EDP occurs in the setting of environmental exposure, the investigator must report information to Pfizer Safety using the CT SAE Report Form/ an EDP Supplemental Form or via PSSA. Since the exposure information does not pertain to the participant

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enrolled in the study, the information is not recorded on a CRF; however, a copy of the completed report is maintained in the investigator site file.

Follow-up is conducted to obtain general information on the pregnancy and its outcome for all EDP reports with an unknown outcome. The investigator will follow the pregnancy until completion (or until pregnancy termination) and notify Pfizer Safety of the outcome as a follow-up to the initial report. In the case of a live birth, the structural integrity of the neonate can be assessed at the time of birth. In the event of a termination, the reason(s) for termination should be specified and, if clinically possible, the structural integrity of the terminated fetus should be assessed by gross visual inspection (unless pre-procedure test findings are conclusive for a congenital anomaly and the findings are reported).

Abnormal pregnancy outcomes are considered SAEs. If the outcome of the pregnancy meets the criteria for an SAE (ie, ectopic pregnancy, spontaneous abortion, intrauterine fetal demise, neonatal death, or congenital anomaly in a live-born baby, a terminated fetus, an intrauterine fetal demise, or a neonatal death), the investigator should follow the procedures for reporting SAEs. Additional information about pregnancy outcomes that are reported to Pfizer Safety as SAEs follows:

- Spontaneous abortion including miscarriage and missed abortion should be reported as an SAE;
- Neonatal deaths that occur within 1 month of birth should be reported, without regard to causality, as SAEs. In addition, infant deaths after 1 month should be reported as SAEs when the investigator assesses the infant death as related or possibly related to exposure to the study intervention.

Additional information regarding the EDP may be requested by the sponsor. Further follow-up of birth outcomes will be handled on a case-by-case basis (eg, follow-up on preterm infants to identify developmental delays). In the case of paternal exposure, the investigator will provide the participant with the Pregnant Partner Release of Information Form to deliver to his partner. The investigator must document in the source documents that the participant was given the Pregnant Partner Release of Information Form to provide to his partner.

8.4.5.2. Exposure During Breastfeeding

An EDB occurs if:

- A female participant is found to be breastfeeding while receiving or after discontinuing study intervention.
- A female nonparticipant is found to be breastfeeding while being exposed or having been exposed to study intervention (ie, environmental exposure). An example of environmental EDB is a female family member or healthcare provider who reports that she is breastfeeding after having been exposed to the study intervention by ingestion, inhalation, or skin contact.

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The investigator must report EDB to Pfizer Safety within 24 hours of the investigator's awareness, irrespective of whether an SAE has occurred. The information must be reported using the CT SAE Report Form or via PSSA. When EDB occurs in the setting of environmental exposure, the exposure information does not pertain to the participant enrolled in the study, so the information is not recorded on a CRF. However, a copy of the completed report is maintained in the investigator site file.

An EDB report is not created when a Pfizer drug specifically approved for use in breastfeeding women (eg, vitamins) is administered in accordance with authorized use. However, if the infant experiences an SAE associated with such a drug, the SAE is reported together with the EDB.

8.4.5.3. Occupational Exposure

The investigator must report any instance of occupational exposure to Pfizer Safety within 24 hours of the investigator's awareness using the CT SAE Report Form or via PSSA, regardless of whether there is an associated SAE. Since the information about the occupational exposure does not pertain to a participant enrolled in the study, the information is not recorded on a CRF; however, a copy of the completed report is maintained in the investigator site file.

8.4.6. Cardiovascular and Death Events

Not applicable.

8.4.7. Disease-Related Events and/or Disease-Related Outcomes Not Qualifying as AEs or SAEs

Not applicable.

8.4.8. Protocol-Specified Adverse Events of Special Interest

AESIs for this study include serious or opportunistic infections, viral reactivation, malignancy, decreases in lymphocytes and platelets, dermatologic events (including rash, acne and urticaria), thromboembolism, and select neurological and audiological events.

Protocol-specified AESIs are examined as part of routine safety data review procedures throughout the clinical trial and as part of signal detection processes. Should an aggregate analysis indicate that these prespecified events occur more frequently than expected, eg, based on epidemiological data, literature, or other data, then this will be submitted and reported in accordance with Pfizer's safety reporting requirements. Aggregate analyses of safety data will be performed on a regular basis per internal SOPs.

All protocol-specified AESIs must be reported as an AE or SAE following the procedures described in Sections 8.4.1 through 8.4.4. A protocol-specified AESI is to be recorded as an AE or SAE on the CRF. In addition, a protocol-specified AESI that is also an SAE must be reported using the CT SAE Report Form or via PSSA.

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8.4.8.1. Lack of Efficacy

The investigator must report signs, symptoms, and/or clinical sequelae resulting from lack of efficacy. Lack of efficacy or failure of expected pharmacological action is not reportable as an AE or SAE.

However, the signs, symptoms and/or clinical sequelae resulting from lack of efficacy must be collected as AE or SAE if they fulfill the definition of an AE or SAE and must be reported to Pfizer Safety only if associated with an SAE.

8.4.9. Medical Device Deficiencies

Not applicable.

8.4.10. Medication Errors

Medication errors may result from the administration or consumption of the study intervention by the wrong participant, or at the wrong time, or at the wrong dosage strength.

Medication errors are recorded and reported as follows:

Recorded on the Medication Error Page of the CRF	Recorded on the Adverse Event Page of the CRF	Reported on the CT SAE Report Form/PSSA to Pfizer Safety Within 24 Hours of Awareness
All (regardless of whether associated with an AE)	Any AE or SAE associated with the medication error	Only if associated with an SAE

Medication errors include:

- Medication errors involving participant exposure to the study intervention;
- Potential medication errors or uses outside of what is foreseen in the protocol that do or do not involve the study participant.
- The administration of expired study intervention;
- The administration of an incorrect dosage;
- The administration of study intervention that has undergone temperature excursion from the specified storage range, unless it is determined by the sponsor that the study intervention under question is acceptable for use.

Whether or not the medication error is accompanied by an AE, as determined by the investigator, such medication errors occurring to a study participant are recorded on the medication error page of the CRF, which is a specific version of the AE page and, if

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applicable, any associated serious and nonserious AE(s) are recorded on the AE page of the CRF.

8.5. Pharmacokinetics

8.5.1. Plasma for Analysis of Ritlecitinib

During all study periods, blood samples of plasma for PK analysis (~2 mL blood to yield ~0.5 mL plasma) will be collected into appropriately labeled tubes containing K_2EDTA at times specified in the SoA.

On the PK sampling visits, Week 4 (or Week 8, if not done at Week 4), participants will be asked about the dosing time on the day before the visits and the investigational site personnel will record in the data collection tool (eg, CRF). Participants will be instructed to not take their daily dose at home but rather to bring the dose to the investigational site for the PK sampling visits and they will be instructed at the site when to take their dose. The dosing time provided to each participant for each visit should be timed such that the PK sampling time occurs during the site visit. All efforts will be made to obtain the PK samples at the exact nominal time relative to dosing. For each blood sample collection, the allowable windows are as follows for time post dose: 1 hr (±15 min), and 3 hr (±30 min). The date and exact time of dosing and the sample collection will be entered onto source document and data collection tool (eg, CRF), as well as the date and time of the previous dose (one day prior to the PK sampling day). Samples obtained outside the windows specified in the SoA will be considered a protocol deviation.

The plasma will be stored in appropriately labeled screw-capped polypropylene tube at approximately -20°C or lower within 1 hour of collection.

Further details regarding the collection, processing, storage and shipping of the PK samples will be provided in the lab manual.

Samples will be analyzed using a validated analytical method in compliance with Pfizer standard operating procedures.

The PK samples must be processed and shipped as indicated to maintain sample integrity. Any deviations from the PK processing steps, including any actions taken, must be documented and reported to the sponsor. On a case-by-case basis, the sponsor may make a determination as to whether sample integrity has been compromised. Any sample deemed outside of established stability, or of questionable integrity, will be considered a protocol deviation.

As part of understanding the PK of the study intervention, samples may be used for metabolite identification and/or evaluation of the bioanalytical method. These data will be used for internal exploratory purposes and will not be included in the clinical report.

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8.5.2. Shipment of Pharmacokinetic Samples

The shipment address and assay lab contact information will be provided to the investigator site prior to initiation of the study. The central laboratory will provide collection materials and directions for packaging and shipment of samples and will forward samples to the contract analytical laboratory. The contract analytical laboratory will be provided with randomization codes so that only samples in the ritlecitinib treatment groups are assayed. Placebo samples may be assayed in the event of suspected error in participant randomization. Refer to the central lab vendor manual for further information.

8.6. Genetics

8.6.1. Specified Genetics

Specified genetic analyses are not evaluated in this study.

8.6.2. Retained Research Samples for Genetics

A 2 mL blood sample optimized for DNA isolation Prep D1.5 will be collected according to the SoA, as local regulations and IRBs/ECs allow.

Retained Research Samples may be used for research related to the study intervention(s) and AA. Genes and other analytes (eg, proteins, RNA, nondrug metabolites) may be studied using the retained samples.

See Appendix 5 for information regarding genetic research. Details on processes for collection and shipment of these samples can be found in the laboratory manual.

8.7. Biomarkers

Collection of samples for biomarker research is also part of this study.

Blood samples will be collected for measurement of serum concentrations of pharmacodynamic biomarkers as specified in the SoA.

8.7.1. Pharmacodynamic Biomarkers

Blood samples will be collected for measurement of serum and whole blood concentrations of pharmacodynamic biomarkers as specified in the SoA. Instructions for the collection and handling of biological samples will be provided in the laboratory manual or by the sponsor. The actual date and time (24-hour clock time) of each sample will be recorded.

The actual times may change, but the number of samples will remain the same. All efforts will be made to obtain the samples at the nominal time relative to dosing.

Samples collected for measurement of serum/PD biomarker concentrations will be analyzed using a bioanalytical method validated for the context of use.

Samples will be used to evaluate PD of ritlecitinib. Samples collected for PD analyses may also be used to evaluate safety or efficacy aspects related to concerns arising during or after

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the study, for evaluation of the bioanalytical method, or for other internal exploratory purposes. The exploratory results may not be reported in the CSR.

Genetic analyses will not be performed on these PD samples. Participant confidentiality will be maintained.

PD samples must be processed and shipped as indicated in the instructions provided to the investigator site to maintain sample integrity. Any deviations from the PD sample handling procedure (eg, sample collection and processing steps, interim storage or shipping conditions), including any actions taken, must be documented and reported to the sponsor. On a case-by-case basis, the sponsor may make a determination as to whether sample integrity has been compromised.

Any changes in the timing or addition of time points for any planned study assessments must be documented and approved by the relevant study team member and then archived in the sponsor and site study files but will not constitute a protocol amendment. The IRB/EC will be informed of any safety issues that require alteration of the safety monitoring scheme or amendment of the ICD.

8.7.1.1. IP-10 (CXCL-10)

IP-10 is the same analyte as CXCL-10. The CDISC CT lab test name CXCL-10 is required to be used for data capture during study and reporting.

Blood samples to provide serum for the analysis of IP-10 (CXCL-10) will be collected as local regulations and IRB/ECs allow into appropriately labeled tubes according to the times outlined in the SoA. IP-10 (CXCL-10) will not be reported to the investigator or sponsor until after completion of study. Residual samples may be used for additional analyses related to AA and/or the mechanism of action of ritlecitinib.

8.7.1.2. FACS Analysis

Blood samples for the assessment of lymphocyte subsets will be collected and will be analyzed by FACS to identify T cell, B cell, and NK cell subsets according to the times outlined in the SoA.

8.7.2. Specified Gene Expression (RNA) Research

Specified gene expression (RNA) research is not included in this study.

8.7.3. Specified Protein Research

Blood samples for protein analyses will be collected according to the times outlined in the SoA. Specified protein research is described in Section 8.7.1. Details on processes for collection and shipment of these sample(s) can be found in the laboratory manual.

8.7.4. Specified Metabolomic Research

Specified metabolomic research is not included in this study.

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8.7.5. Retained Research Samples for Biomarkers

Retained Research Samples for biomarkers will not be collected in this study.

8.8. Immunogenicity Assessments

Immunogenicity assessments are not included in this study.

8.9. Health Economics

Health economics/medical resource utilization and health economics parameters are not evaluated in this study.

9. STATISTICAL CONSIDERATIONS

Detailed methodology for summary and statistical analyses of the data collected in this study is outlined here and further detailed in the SAP, which will be maintained by the sponsor. The SAP may modify what is outlined in the protocol where appropriate; however, any major modifications of the primary endpoint definitions or their analyses will also be reflected in a protocol amendment.

9.1. Statistical Hypotheses

The primary and key secondary endpoints are region dependent and will be tested separately by region.

US and Countries Following US Analysis Plan

- H1 is to test whether the response rates of SALT ≤20 at Week 24 are the same between ritlecitinib high dose and placebo;
- H2 is to test whether the response rates of SALT ≤20 at Week 24 are the same between ritlecitinib low dose and placebo.

The familywise Type I error rate will be set to $\alpha = 0.05$ (2-sided). H1 will be tested at $\alpha = 0.05$ first and if significant, H2 will be tested at $\alpha = 0.05$.

EU/UK and Countries Following EU/UK Analysis Plan

- H1 is to test whether the response rates of SALT ≤10 at Week 24 are the same between ritlecitinib high dose and placebo;
- H2 is to test whether the response rates of SALT ≤10 at Week 24 are the same between ritlecitinib low dose and placebo;
- H3 is to test whether the response rates of PGI-C at Week 24 are the same between ritlecitinib high dose and placebo;
- H4 is to test whether the response rates of PGI-C at Week 24 are the same between ritlecitinib low dose and placebo.

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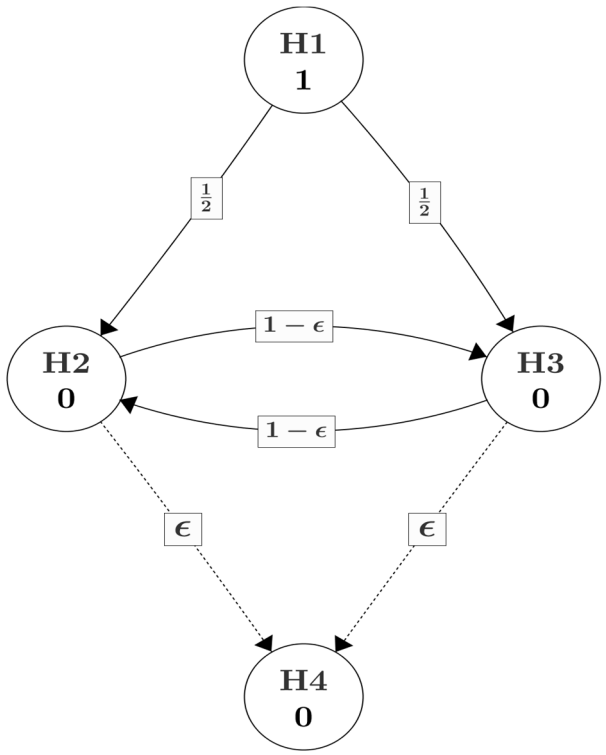
The familywise Type I error rate will be set to $\alpha = 0.05$ (2-sided). H1 will be tested at $\alpha = 0.05$ first and if significant, H2 and H3 will be tested simultaneously applying Holm's approach. If both H2 and H3 are significant, H4 will be tested at $\alpha = 0.05$. The graphical approach of Bretz et al (2009)[43] for sequentially rejective testing procedures is used to illustrate the testing strategy (Figure 2).

Figure 2. Testing Strategy for Primary and Key Secondary Endpoints for EU/UK and Countries Following EU/UK Analysis Plan



ϵ denotes an infinitesimally small weight in the graphical approach that allows for a reallocation of the significance level between families of hypotheses. When updating the transition weights in the graph, ϵ is treated in the calculations as a variable representing some fixed positive real number. For the computation of the updated significance levels or weights, we let $\epsilon \to 0$.

9.1.1. Estimands

9.1.1.1. Primary Estimand

US and Countries Following US Analysis Plan

The primary estimand (composite estimand, Estimand 1) is defined by the following attributes:

- Population: pediatric AA participants as defined by the inclusion/exclusion criteria;
- Treatment: ritlecitinib high dose, ritlecitinib low dose, and placebo;

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- Variable: binary response variable indicating a successful response if SALT score ≤20 at Week 24. A participant after an intercurrent event will be considered a non-responder for the visit of interest;

- Intercurrent events: discontinuations of study intervention due to any reason or use of prohibited medication for AA* prior to the Week 24 visit will be the intercurrent events and are captured through the variable definition;

  * Includes use of a prior or concomitant prohibited medication for AA that is thought to have a potential impact on the primary endpoint.

- Population-level summary: difference in response proportions between each ritlecitinib dose and placebo.

EU/UK and Countries Following EU/UK Analysis Plan

Estimand 1 will also be applied to the primary endpoint of SALT ≤10 response and the key secondary endpoint of PGI-C response for EU/UK and countries following the EU/UK analysis plan.

9.1.1.2. Secondary Estimands

A secondary estimand (composite estimand, Estimand 2) is defined by the following attributes:

- Population: pediatric AA participants as defined by the inclusion/exclusion criteria;

- Treatment: ritlecitinib high dose, ritlecitinib low dose, and placebo;

- Variable: continuous endpoint. For a participant with an intercurrent event, the worst observation (including the baseline value) prior to the intercurrent event will be used from the time point the intercurrent event occurs and onwards (ie, WOCF);

- Intercurrent events: discontinuation of study intervention due to any reason or use of prohibited medications for AA* prior to the Week 24 visit will be the intercurrent events and will be captured through the variable definition;

  * Includes use of a prior or concomitant prohibited medication for AA that is thought to have a potential impact on the primary endpoint.

- Population-level summary: difference in means between each ritlecitinib dose and placebo.

Estimand 2 will be used as the main estimand for continuous secondary endpoints.

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A secondary estimand (treatment policy estimand, Estimand 3) is defined by the following attributes:

- Population: pediatric AA participants as defined by the inclusion/exclusion criteria;
- Treatment: ritlecitinib high dose, ritlecitinib low dose, and placebo;
- Variable: binary endpoint;
- Intercurrent events: no intercurrent event will be defined;
- Population-level summary: difference in response proportions for binary endpoint between each ritlecitinib dose and placebo.

Estimand 3 will be used as a supportive estimand for the primary and key secondary endpoints.

9.1.2. Multiplicity Adjustment

For the primary and key secondary endpoints, to control for familywise Type I error rate, the graphical approach as described earlier in Section 9.1 will be utilized.

9.2. Analysis Sets

For purposes of analysis, the following analysis sets are defined:

Participant Analysis Set	Description
Full analysis set	All participants randomly assigned to study intervention. Participants will be analyzed according to the intervention to which they were randomized.
Safety analysis set	All participants randomly assigned to study intervention and take at least 1 dose of study intervention. Participants will be analyzed according to the intervention they actually received.
PK concentration analysis set	All enrolled participants who received at least 1 dose of ritlecitinib and in whom at least 1 concentration value is reported.

9.3. Statistical Analyses

The SAP will be developed and finalized before the database is released. This section is a summary of the planned statistical analyses of the primary and secondary endpoints.

9.3.1. General Considerations

Unless otherwise specified, the efficacy analyses will be conducted on FAS and safety analyses on SAS. Treatment groups displayed will be ritlecitinib 50 mg QD, ritlecitinib 30 mg QD and placebo. In general, number, percent and confidence interval will be

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presented for binary variables. Number, mean, standard deviation, LS mean, standard error of the LS mean, and confidence interval will be presented for all continuous variables.

Treatment comparison will be conducted for ritlecitinib 50 mg QD versus placebo and ritlecitinib 30 mg QD versus placebo. All tests will be 2-sided at $\alpha = 0.05$, and 95% CIs for treatment difference will be provided.

Analyses details for primary and key secondary endpoints are described in Section 9.3.2.2.

For continuous endpoints, change from baseline will be analyzed by an ANCOVA model at each post-baseline visit with terms for baseline value of the endpoint (if applicable), treatment group, and stratification factor of AA severity (AT/AU and non-AT/AU). All missing data will be handled by WOCF.

Binary endpoints will be analyzed using the CMH approach adjusting for the stratification factor of AA severity (AT/AU and non-AT/AU). All missing data will be considered as non-response, unless otherwise specified.

9.3.2. Primary Endpoint(s)/Estimand(s) Analysis

9.3.2.1. Definition of Endpoint(s)

US and Countries Following US Analysis Plan

The primary endpoint is response of SALT score ≤20 at Week 24.

EU/UK and Countries Following EU/UK Analysis Plan

The primary endpoint is response of SALT score ≤10 at Week 24.

9.3.2.2. Main Analytical Approach

The primary estimand is described in Section 9.1.1.1.

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US and Countries Following US Analysis Plan

SALT ≤20 responses at Week 24 will be analyzed using the CMH approach adjusting for the stratification factor of AA severity (AT/AU, non-AT/AU) using Estimand 1 on FAS. All missing data will be considered as non-response.

EU/UK and Countries Following EU/UK Analysis Plan

SALT ≤10 responses at Week 24 will be analyzed using the CMH approach adjusting for the stratification factor severity of AA (AT/AU, non-AT/AU) using Estimand 1 on FAS. All missing data will be considered as non-response.

9.3.2.3. Supplementary Analysis

For binary endpoint of SALT score ≤20 response and SALT score ≤10 response at Week 24, the estimand with treatment policy strategy (Estimand 3) plus non-response imputation for missing data on FAS will be evaluated as a supportive analysis.

Subgroup analyses will be performed for SALT score ≤20 response and SALT score ≤10 response at Week 24 by age, gender, race and ethnicity (Hispanic/Latino and not Hispanic/Latino) using Estimand 1 on FAS. All missing data will be considered as non-response.

9.3.3. Secondary Endpoint(s)/Estimand(s) Analysis

The secondary estimand is described in Section 9.1.1.2.

The methods for the key secondary endpoint PGI-C response at Week 24 will be analyzed the same as that for SALT ≤10 responses at Week 24 described in Section 9.3.2.2, using Estimand 1 as main estimand and Estimand 3 as supportive estimand. All missing data will be considered as non-response.

Subgroup analyses will be performed for PGI-C response at Week 24 by age, gender, race and ethnicity (Hispanic/Latino and not Hispanic/Latino) using Estimand 1 on FAS. All missing data will be considered as non-response.

The methods for other secondary binary endpoints will be the same as that for SALT ≤20 and SALT ≤10 responses at Week 24 and using Estimand 1 as described in Section 9.3.2.2. All missing data will be considered as non-response. No supportive estimand will be applied.

The secondary continuous endpoints will be analyzed by ANCOVA using Estimand 2 as described in Section 9.1.1.2, as the main estimand on FAS. All missing data will be handled by WOCF. No supportive estimand will be applied.

9.3.3.1. Pharmacokinetic Analyses

PK concentrations listings and summary by visit and nominal collection time will be provided. A population PK model will be developed to characterize the PK and estimate PK

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parameters. The detailed analysis plans will be described in a separate pharmacometrics analysis plan.

9.3.4. Safety Analyses

All safety analyses will be performed on the safety population. Safety data from the follow-up visit will also be included in the summaries, which is equivalent to using a treatment policy estimand.

The safety data will be summarized in accordance with Pfizer implemented CDISC Standards. All safety data will be summarized descriptively through appropriate data tabulations, descriptive statistics, categorical summaries, and graphical presentations by treatment group.

9.4. Interim Analyses

No interim analysis will be conducted for this study.

9.5. Sample Size Determination

The power is for a treatment comparison of interest upon passing the tests higher in the testing hierarchy (Section 9.1). The assumptions used for the power calculations were informed by the results from the ritlecitinib clinical trials.

US and Countries Following US Analysis Plan

A total sample size of 225 participants, with 75 participants per treatment arm (1:1:1 randomization), using normal approximation with unpooled estimate of variance, will provide 95% power to detect a treatment difference of 20% between the high dose (or low dose) and placebo for the primary endpoint of SALT ≤20 response at Week 24, assuming a 5% placebo response, at the 2-sided α-level of 0.05 (for each comparison).

EU/UK and Countries Following EU/UK Analysis Plan

A total sample size of 225 participants, with 75 participants per treatment arm (1:1:1 randomization), using normal approximation with unpooled estimate of variance, will provide: a) 95% power to detect a treatment difference of 20% between the high dose and placebo for the primary endpoint of SALT ≤10 response at Week 24, assuming a 5% placebo response, at the 2-sided α-level of 0.05; b) 91% power to detect a treatment difference of 20% between the low dose and placebo for the primary endpoint of SALT ≤10 response at Week 24, assuming a 5% placebo response, at the 2-sided α-level of 0.025; c) 99% power to detect a treatment difference of 35% between the high dose and placebo for the key secondary endpoint of PGI-C response at Week 24, assuming a 20% placebo response, at the 2-sided α-level of 0.025; and d) >99% power to detect a treatment difference of 35% between the low dose and placebo in the key secondary endpoint of PGI-C response at Week 24, assuming a 20% placebo response, at the 2-sided α-level of 0.05.

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10. SUPPORTING DOCUMENTATION AND OPERATIONAL CONSIDERATIONS

10.1. Appendix 1: Regulatory, Ethical, and Study Oversight Considerations

10.1.1. Regulatory and Ethical Considerations

This study will be conducted in accordance with the protocol and with the following:

- Consensus ethical principles derived from international guidelines, including the Declaration of Helsinki and CIOMS International Ethical Guidelines;
- Applicable ICH GCP guidelines;
- Applicable laws and regulations, including applicable privacy laws.

The protocol, protocol amendments, ICD, SRSD(s), and other relevant documents (eg, advertisements) must be reviewed and approved by the sponsor, submitted to an IRB/EC by the investigator, and reviewed and approved by the IRB/EC before the study is initiated.

Any amendments to the protocol will require IRB/EC approval before implementation of changes made to the study design, except for changes necessary to eliminate an immediate hazard to study participants.

Protocols and any substantial amendments to the protocol will require health authority approval prior to initiation except for changes necessary to eliminate an immediate hazard to study participants.

The investigator will be responsible for the following:

- Providing written summaries of the status of the study to the IRB/EC annually or more frequently in accordance with the requirements, policies, and procedures established by the IRB/EC;
- Notifying the IRB/EC of SAEs or other significant safety findings as required by IRB/EC procedures;
- Providing oversight of the conduct of the study at the site and adherence to requirements of 21 CFR, ICH GCP guidelines, the IRB/EC, European regulation 536/2014 for clinical studies, European Medical Device Regulation 2017/745 for clinical device research, and all other applicable local regulations.

10.1.1.1. Reporting of Safety Issues and Serious Breaches of the Protocol or ICH GCP

In the event of any prohibition or restriction imposed (ie, clinical hold) by an applicable regulatory authority in any area of the world, or if the investigator is aware of any new information that might influence the evaluation of the benefits and risks of the study intervention, Pfizer should be informed immediately.

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In addition, the investigator will inform Pfizer immediately of any urgent safety measures taken by the investigator to protect the study participants against any immediate hazard, and of any serious breaches of this protocol or of the ICH GCP guidelines that the investigator becomes aware of.

10.1.2. Financial Disclosure

Investigators and sub-investigators will provide the sponsor with sufficient, accurate financial information as requested to allow the sponsor to submit complete and accurate financial certification or disclosure statements to the appropriate regulatory authorities. Investigators are responsible for providing information on financial interests during the course of the study and for 1 year after completion of the study.

10.1.3. Informed Consent/Assent Process

The investigator or qualified designee will explain the nature of the study to the participant and their parent(s)/legal guardian and answer all questions regarding the study. The investigator may not delegate obtaining consent to a local healthcare professional.

The participant and their parent(s)/legal guardian should be given sufficient time and opportunity to ask questions and to decide whether or not to participate in the trial.

When consent is obtained from a participant's parent(s)/legal guardian, the participant's assent (affirmative agreement) must be subsequently obtained when the participant has the capacity to provide assent, as determined by the IRB/EC. If the investigator determines that a participant's decisional capacity is so limited they cannot reasonably be consulted, then, as permitted by the IRB/EC and consistent with local regulatory and legal requirements, the participant's assent may be waived with source documentation of the reason assent was not obtained. If the study participant does not provide their own assent, the source documents must record why the participant did not provide assent (for example, the child is not of assenting age per local regulations or policies), how the investigator determined that the person signing the consent was the participant's parent(s)/legal guardian, the consent signer's relationship to the study participant, and that the participant's assent was obtained or waived. If assent is obtained verbally, it must be documented in the source documents.

If study participants are minors who reach the age of majority or if a child reaches the age of assent (per local IRB/EC requirements) during the study, as recognized under local law, the child or adolescent must then provide the appropriate assent or consent to document their willingness to continue in the study. For an adolescent who reaches the age of consent, parental consent would no longer be valid. If the enrollment of emancipated minors is permitted by the IRB/EC and local law, the participant must provide documentation of legal status to give consent without the permission of a legally authorized representative.

Participants and their parent(s)/legal guardian must be informed that their participation is voluntary. The participant's parent(s)/legal guardian will be required to sign a statement of informed consent that meets the requirements of 21 CFR 50, local regulations, ICH guidelines, HIPAA requirements, where applicable, and the IRB/EC or study center.

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The investigator must ensure that each study participant's parent(s)/legal guardian and the study participant as applicable are fully informed about the nature and objectives of the study, the sharing of data related to the study, and possible risks associated with participation, including the risks associated with the processing of the participant's personal data.

The participant's parent(s)/legal guardian must be informed that the participant's personal study-related data will be used by the sponsor in accordance with local data protection law. The level of disclosure must also be explained to the participant's parent(s)/legal guardian.

The participant's parent(s)/legal guardian must be informed that the participant's medical records may be examined by Clinical Quality Assurance auditors or other authorized personnel appointed by the sponsor, by appropriate IRB/EC members, and by inspectors from regulatory authorities.

The investigator further must ensure that each study participant's parent(s)/legal guardian is fully informed about their right to access and correct their child's personal data and to withdraw consent for the processing of their child's personal data, keeping in mind the privacy rights that may restrict access of older adolescents' medical records by their parent(s)/legal guardian in certain regions.

The source documentation must include a statement that written informed consent, and as applicable, assent was obtained before the participant was enrolled in the study and the date the written consent/assent was obtained. The authorized person obtaining the informed consent must also sign the ICD.

Parent(s)/legal guardian and the participant must be reconsented to the most current version of the ICD(s)/assent during their participation in the study as required per local regulations.

A copy of the ICD(s) and assent, if written, must be provided to the parent(s)/legal guardian and the participant.

A participant who is rescreened is not required to sign another ICD if the rescreening occurs within 90 days from the previous ICD signature date.

10.1.4. Data Protection

All parties will comply with all applicable laws, including laws regarding the implementation of organizational and technical measures to ensure protection of participant data.

Participants' personal data will be stored at the study site in encrypted electronic and/or paper form and will be password protected or secured in a locked room to ensure that only authorized study staff have access. The study site will implement appropriate technical and organizational measures to ensure that the personal data can be recovered in the event of disaster. In the event of a potential personal data breach, the study site will be responsible for

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determining whether a personal data breach has in fact occurred and, if so, providing breach notifications as required by law.

To protect the rights and freedoms of participants with regard to the processing of personal data, participants will be assigned a single, participant-specific numerical code. Any participant records or data sets that are transferred to the sponsor will contain the numerical code; participant names will not be transferred. All other identifiable data transferred to the sponsor will be identified by this single, participant-specific code. The study site will maintain a confidential list of participants who participated in the study, linking each participant's numerical code to their actual identity and medical record ID. In case of data transfer, the sponsor will protect the confidentiality of participants' personal data consistent with the clinical study agreement and applicable privacy laws.

Information technology systems used to collect, process, and store study-related data are secured by technical and organizational security measures designed to protect such data against accidental or unlawful loss, alteration, or unauthorized disclosure or access.

The sponsor maintains SOPs on how to respond in the event of unauthorized access, use, or disclosure of sponsor information or systems.

10.1.5. Committees Structure

10.1.5.1. Data Monitoring Committee

This study will use an E-DMC. The E-DMC is independent of the study team and includes only external members. The E-DMC charter describes the role of the E-DMC in more detail.

The E-DMC will be responsible for ongoing monitoring of the safety of participants in the study according to the charter. The recommendations made by the E-DMC will be forwarded to the appropriate authorized Pfizer personnel for review and final decision. Pfizer will communicate such decisions, which may include summaries of aggregate analyses of endpoint events and of safety data that are not endpoints, to regulatory authorities and investigators, as appropriate.

10.1.6. Dissemination of Clinical Study Data

Pfizer fulfills its commitment to publicly disclose clinical study results through posting the results of studies on www.clinicaltrials.gov (ClinicalTrials.gov), the EudraCT/CTIS, and/or www.pfizer.com, and other public registries and websites in accordance with applicable local laws/regulations. In addition, Pfizer reports study results outside of the requirements of local laws/regulations pursuant to its SOPs.

In all cases, study results are reported by Pfizer in an objective, accurate, balanced, and complete manner and are reported regardless of the outcome of the study or the country in which the study was conducted.

www.clinicaltrials.gov

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Pfizer posts clinical trial results on www.clinicaltrials.gov for Pfizer-sponsored interventional studies (conducted in patients) that evaluate the safety and/or efficacy of a product, regardless of the geographical location in which the study is conducted. These results are submitted for posting in accordance with the format and timelines set forth by US law.

EudraCT/CTIS

Pfizer posts clinical trial results on EudraCT/CTIS for Pfizer-sponsored interventional studies in accordance with the format and timelines set forth by EU requirements following the end of the study globally.

www.pfizer.com

Pfizer posts CSR synopses and plain-language study results summaries on www.pfizer.com for Pfizer-sponsored interventional studies at the same time the corresponding study results are posted to www.clinicaltrials.gov. CSR synopses will have personally identifiable information anonymized.

Documents within marketing applications

Pfizer complies with applicable local laws/regulations to publish clinical documents included in marketing applications. Clinical documents include summary documents and CSRs including the protocol and protocol amendments, sample CRFs, and SAPs. Clinical documents will have personally identifiable information anonymized.

Data sharing

Pfizer provides researchers secure access to participant-level data or full CSRs for the purposes of "bona-fide scientific research" that contributes to the scientific understanding of the disease, target, or compound class. Pfizer will make data from these trials available 18 months after study completion. Participant-level data will be anonymized in accordance with applicable privacy laws and regulations. CSRs will have personally identifiable information anonymized.

Data requests are considered from qualified researchers with the appropriate competencies to perform the proposed analyses. Research teams must include a biostatistician. Data will not be provided to applicants with significant conflicts of interest, including individuals requesting access for commercial/competitive or legal purposes.

10.1.7. Data Quality Assurance

All participant data relating to the study will be recorded on printed or electronic CRF unless transmitted to the sponsor or designee electronically (eg, laboratory data). The investigator is responsible for verifying that data entries are accurate and correct by physically or electronically signing the CRF.

Guidance on completion of CRFs will be provided in the CRF Completion Requirements document.

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The investigator must ensure that the CRFs are securely stored at the study site in encrypted electronic and/or paper form and are password protected or secured in a locked room to prevent access by unauthorized third parties.

QTLs are predefined parameters that are monitored during the study. Important deviations from the QTLs and any remedial actions taken will be summarized in the CSR.

The investigator must permit study-related monitoring, audits, IRB/EC review, and regulatory agency inspections and provide direct access to source records and documents. This verification may also occur after study completion. It is important that the investigator(s) and their relevant personnel are available during the monitoring visits and possible audits or inspections and that sufficient time is devoted to the process.

Monitoring details describing strategy, including definition of study-critical data items and processes (eg, risk-based initiatives in operations and quality such as risk management and mitigation strategies and analytical risk-based monitoring), methods, responsibilities, and requirements, including handling of noncompliance issues and monitoring techniques (central, virtual, or on-site monitoring), are provided in the data management plan and monitoring plan maintained and utilized by the sponsor or designee.

The sponsor or designee is responsible for the data management of this study, including quality checking of the data.

Records and documents, including signed ICDs, pertaining to the conduct of this study must be retained by the investigator for up to 25 years or another retention period that is consistent with the ICD and clinical trial agreement after study completion unless local regulations or institutional policies require a longer retention period. No records may be destroyed during the retention period without the written approval of the sponsor. No records may be transferred to another location or party without written notification to the sponsor. The investigator must ensure that the records continue to be stored securely for as long as they are maintained.

When participant data are to be deleted, the investigator will ensure that all copies of such data are promptly and irrevocably deleted from all systems.

The investigator(s) will notify the sponsor or its agents immediately of any regulatory inspection notification in relation to the study. Furthermore, the investigator will cooperate with the sponsor or its agents to prepare the investigator site for the inspection and will allow the sponsor or its agent, whenever feasible, to be present during the inspection. The investigator site and investigator will promptly resolve any discrepancies that are identified between the study data and the participant's medical records. The investigator will promptly provide copies of the inspection findings to the sponsor or its agent. Before response submission to the regulatory authorities, the investigator will provide the sponsor or its agents with an opportunity to review and comment on responses to any such findings.

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10.1.8. Source Documents

Source documents provide evidence for the existence of the participant and substantiate the integrity of the data collected. Source documents are filed at the investigator site.

Data reported on the CRF or entered in the eCRF that are from source documents must be consistent with the source documents or the discrepancies must be explained. The investigator may need to request previous medical records or transfer records, depending on the study. Also, current medical records must be available.

Definition of what constitutes a source document and its origin can be found in the Source Document Locator, which is maintained by the sponsor.

Description of the use of the computerized system is documented in the data management plan, which is maintained by the sponsor.

The investigator must maintain accurate documentation (source record) that supports the information entered in the CRF.

The sponsor or designee will perform monitoring to confirm that data entered into the CRF by authorized site personnel are accurate, complete, and verifiable from source documents; that the safety and rights of participants are being protected; and that the study is being conducted in accordance with the currently approved protocol and any other study agreements, ICH GCP guidelines, and all applicable regulatory requirements.

10.1.9. Use of Medical Records

In certain situations, sponsor review of redacted copies of participant medical records for safety monitoring, follow-up of AEs or laboratory abnormalities, review of local laboratory results may be performed, where ethically and scientifically justified and permitted by local regulations, to ensure participant safety.

Due to the potential for a participant to be re-identified from their medical records, the following actions must be taken when medical records are sent to the sponsor or sponsor designee:

- The investigator or site staff must redact personal information from the medical record. The personal information includes, but is not limited to, the following: participant <u>names or initials</u>, participant <u>dates</u> (eg, birth date, date of hospital admission/discharge, date of death), participant <u>identification numbers</u> (eg, Social Security number, health insurance number, medical record number, hospital/institution identifier), participant <u>location information</u> (eg, street address, city, country, postal code, IP address), participant <u>contact information</u> (eg, telephone/fax number, email address).

- Each medical record must be transmitted to the sponsor or sponsor designee using systems with technical and organizational security measures to ensure the protection of personal data (eg, Florence is the preferred system if available).

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- There may be unplanned situations where the sponsor may request medical records (eg, sharing medical records so that the sponsor can provide study-related advice to the investigator). The medical records should be submitted according to the procedure described above.

10.1.10. Study and Site Start and Closure

The study start date is the date of the first participant's first visit.

The sponsor designee reserves the right to close the study site or terminate the study at any time for any reason at the sole discretion of the sponsor, including (but not limited to) regulatory authority decision, change in opinion of the IRB/EC, or change in benefit-risk assessment. Study sites will be closed upon study completion. A study site is considered closed when all required documents and study supplies have been collected and a study-site closure visit has been performed.

The investigator may initiate study-site closure at any time upon notification to the CRO if requested to do so by the responsible IRB/EC or if such termination is required to protect the health of study participants.

Reasons for the early closure of a study site by the sponsor may include but are not limited to:

- Failure of the investigator to comply with the protocol, the requirements of the IRB/EC or local health authorities, the sponsor's procedures, or the ICH GCP guidelines;
- Inadequate recruitment of participants by the investigator;
- Discontinuation of further study intervention development.

If the study is prematurely terminated or suspended, the sponsor shall promptly inform the investigators, the ECs/IRBs, the regulatory authorities, and any CRO(s) used in the study of the reason for termination or suspension, as specified by the applicable regulatory requirements. The investigator shall promptly inform the participant and should assure appropriate participant therapy and/or follow-up.

Study termination is also provided for in the clinical study agreement. If there is any conflict between the contract and this protocol, the contract will control as to termination rights.

10.1.11. Publication Policy

For multicenter trials, the primary publication will be a joint publication developed by the investigator and Pfizer reporting the primary endpoint(s) of the study covering all study sites. The investigator agrees to refer to the primary publication in any subsequent publications. Pfizer will not provide any financial compensation for the investigator's participation in the preparation of the primary congress abstract, poster, presentation, or primary manuscript for the study.

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Investigators are free to publish individual center results that they deem to be clinically meaningful after publication of the overall results of the study or 12 months after primary completion date or study completion at all sites, whichever occurs first, subject to the other requirements described in this section.

The investigator will provide Pfizer an opportunity to review any proposed publication or any other type of disclosure of the study results (collectively, "publication") before it is submitted or otherwise disclosed and will submit all publications to Pfizer 30 days before submission. If any patent action is required to protect intellectual property rights, the investigator agrees to delay the disclosure for a period not to exceed an additional 60 days upon request from Pfizer. This allows Pfizer to protect proprietary information and to provide comments, and the investigator will, on request, remove any previously undisclosed confidential information before disclosure, except for any study intervention or Pfizer-related information necessary for the appropriate scientific presentation or understanding of the study results. For joint publications, should there be disagreement regarding interpretation and/or presentation of specific analysis results, resolution of, and responsibility for, such disagreements will be the collective responsibility of all authors of the publication.

For all publications relating to the study, the investigator and Pfizer will comply with recognized ethical standards concerning publications and authorship, including those established by the International Committee of Medical Journal Editors. The investigator will disclose any relationship with Pfizer and any relevant potential conflicts of interest, including any financial or personal relationship with Pfizer, in any publications. When applicable, editorial or technical support provided by a third party and paid for by Pfizer, or provided by a Pfizer employee, may be a reportable transfer of value under the Sunshine Act for US licensed physicians or other healthcare professionals. All authors will have access to the relevant statistical tables, figures, and reports (in their original format) required to develop the publication.

10.1.12. Sponsor's Medically Qualified Individual

The sponsor will designate a medically qualified individual (MQI, also known as the medical monitor) to advise the investigator on study-related medical questions. The contact information for the study medical monitor is documented in the Study Team Contact List located in the Investigator Site File.

Participants are provided with a Pfizer study information card at the time of informed consent, which includes contact information for their investigator in case of study-related medical questions. The study information card contains, at a minimum, (a) study number, (b) participant's study identification number, and (c) principal investigator contact information.

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10.2. Appendix 2: Clinical Laboratory Tests

The following safety laboratory tests will be performed at times defined in the SoA section of this protocol. Additional laboratory results may be reported on these samples as a result of the method of analysis or the type of analyzer used by the clinical laboratory, or as derived from calculated values; for example: calculation of estimated kidney function (ie, Schwartz eGFR [children only], as standard lab safety test for clinical studies). These additional tests would not require additional collection of blood. Unscheduled clinical laboratory measurements may be obtained at any time during the study to assess any perceived safety issues at the investigator's discretion. All laboratory tests with clinically important changes from Baseline that are identified after the first dose of study intervention will be followed until the laboratory value stabilizes.

Table 4. Protocol-Required Laboratory Assessments

Hematology	Chemistry	Local Dipstick Urinalysis	Other Tests
Hemoglobin Hematocrit RBC count Platelet count WBC count Total neutrophils (Abs) Eosinophils (Abs) Monocytes (Abs) Basophils (Abs) Lymphocytes (Abs)[a] Reticulocyte Count (Abs)	BUN or Urea Creatinine Glucose Calcium Sodium Potassium AST ALT Total bilirubin Direct bilirubin Indirect bilirubin Alkaline phosphatase Creatine kinase[l] Follow-up testing for potential DILI cases[m]	pH Glucose Protein Blood Ketones Nitrites Leukocyte esterase Urobilinogen Urine bilirubin Microscopy[g] Urine Culture[h]	Pregnancy Test (as applicable)[b] HIV[c] HBsAg HBcAb HBsAb[d] HBV DNA[e] HCVAb HCV RNA[f] VZV IgG Ab[n] QFT-G test or T-Spot test[i] Viral screen[j] IP-10 (CXCL-10) FACS TBNK Swabs[k]

a. Participants with absolute lymphocyte counts <500/mm^3 (<0.5 × 10^9/L) will be reflex-tested for FACS TBNK until the absolute lymphocyte count resolves or stabilizes at a level acceptable to the investigator and the sponsor.
b. Pregnancy tests (urine) for females ≥8 years. In the event that urine pregnancy tests are not permitted at an institution, serum pregnancy tests can be utilized.
c. HIV testing per local regulations. Study participants who are positive for HIV antibodies will be screen failed.
d. HBsAb will be performed as reflex testing for any study participant who is HBsAg negative but HBcAb positive. For Japan only: In addition to HBsAg and HBcAb, HBsAb testing will be performed at screening for participants in Japan rather than as a reflex test. For Japan specific requirements see Section 10.10.3
e. HBV DNA testing will be performed at Week 12 for participants who were enrolled with a positive HBcAb and a negative HBV DNA in those regions for which Hep B prevalence has been reported at a rate of >5.0% or if required by local standard of care. For Czech Republic-specific requirements, see Appendix 10, Section 10.10.5.1.
f. HCV RNA will be performed as reflex testing for any study participant who is HCVAb positive. For Czech Republic-specific requirements, see Appendix 10, Section 10.10.5.2.
g. Only if urine dipstick is positive for blood, nitrites, leukocyte esterase, and/or protein.
h. Urine culture will be performed if urinalysis is positive for nitrite and/or leukocyte esterase or if clinically indicated.
i. The QFT-G test is preferred; however, the T-Spot test is also permitted. A negative PPD test can be substituted for the QFT-G test or T-Spot test only under specific circumstances.

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Table 4. Protocol-Required Laboratory Assessments

Hematology	Chemistry	Local Dipstick Urinalysis	Other Tests

j. A serum sample will be collected at baseline and submitted to the central lab. The sample will be stored and analyzed at a later date only at the sponsor's request. In certain cases of suspected viral infection (eg, disseminated herpes zoster or varicella), the sponsor may request to analyze the sample to determine if the participant had exposure to that virus.
k. When required in cases of skin rash adverse events.
l. If CK> 3 × ULN, retesting will be done. In addition to re-testing CK >3 × ULN, any participant with CK >10 × ULN will have local urine dipstick testing for blood performed. If the dipstick result is positive, the same urine sample will be sent to the central lab for urinalysis and RBC microscopy (refer to the laboratory manual for details).
m. In cases of suspected potential DILI, follow-up testing should be performed.
n. Needed only if there is no documented evidence of 2 doses of varicella vaccine or prior exposure to VZV (positive VZV IgG Ab test result).

The investigator must review the laboratory report, document this review, and record any clinically relevant changes occurring during the study in the AE section of the CRF.

Laboratory test results that could unblind the study and have been collected for the purpose of the study will not be reported to investigator sites or other blinded personnel until the study has been unblinded.

QuantiFERON®-TB Gold Test

QFT-G is an in vitro diagnostic test used to identify in vitro responses to peptide antigens that are associated with Mycobacterium tuberculosis infection. QFT-G is an indirect test for M. tuberculosis infection (including disease) and is intended for use in conjunction with risk assessment, imaging, and other medical and diagnostic evaluations. The QFT-G test is the preferred testing method and will be processed by the central laboratory. A QFT-G test is preferred for participants with a prior BCG vaccination.

Test results will be reported as positive, negative, or borderline/indeterminate. In the case of an indeterminate/borderline IGRA result, repeat test may be permitted for the purpose of determining eligibility of participants to enroll in this study as noted in the exclusion criteria.

The procedure for using this test and interpreting results is described fully in the laboratory manual, which will be provided to investigators.

T-SPOT®TB testing

While QuantiFERON® is the preferred IGRA testing method, the T-SPOT®TB test is acceptable as the TB test. If the test results are indeterminate/borderline, the test should be repeated. If the result of the repeat test is indeterminate/borderline, the participant may be tested using the Mantoux/PPD skin test with Pfizer medical monitor approval.

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Mantoux/Purified Protein Derivative (PPD) Tuberculin Test

If the TB results are indeterminate/borderline after the second IGRA test, a PPD test may be substituted for the IGRA only following consultation with the Pfizer clinician or medical monitor on a case-by-case basis. The test consists of an intracutaneous injection of 5 TU PPD in 0.1 mL of solution on the volar aspect of the forearm, using a short beveled 26- or 27-gauge needle (Mantoux test). After the tuberculin test is administered, the test area will be evaluated by a qualified healthcare professional, per local guidelines, 48 to 72 hours later. The test is positive if the induration diameter is ≥5 mm at 48 to 72 hours post injection.

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10.3. Appendix 3: Adverse Events: Definitions and Procedures for Recording, Evaluating, Follow-Up, and Reporting

10.3.1. Definition of AE

AE Definition
• An AE is any untoward medical occurrence in a patient or clinical study participant, temporally associated with the use of study intervention, whether or not considered related to the study intervention. • Note: An AE can therefore be any unfavorable and unintended sign (including an abnormal laboratory finding), symptom, or disease (new or exacerbated) temporally associated with the use of study intervention.

Events <u>Meeting</u> the AE Definition
• Any abnormal laboratory test results (hematology, clinical chemistry, or urinalysis) or other safety assessments (eg, ECG, radiological scans, vital sign measurements), including those that worsen from baseline, considered clinically significant in the medical and scientific judgment of the investigator. Any abnormal test results that meet any of the conditions below must be recorded as an AE: • Is associated with accompanying symptoms. • Requires additional diagnostic testing or medical/surgical intervention. • Leads to a change in study dosing (outside of any protocol-specified dose adjustments) or discontinuation from the study, significant additional concomitant drug treatment, or other therapy. • Exacerbation of a chronic or intermittent preexisting condition, including an increase in either frequency and/or intensity of the condition. • New condition detected or diagnosed after study intervention administration, even though it may have been present before the start of the study. • Signs, symptoms, or the clinical sequelae of a suspected drug-drug interaction. • Signs, symptoms, or the clinical sequelae of a suspected overdose of either study intervention or a concomitant medication. Overdose per se will not be reported as an AE or SAE unless it is an intentional overdose taken with possible suicidal/self-harming intent. Such overdoses should be reported regardless of sequelae.

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Events NOT Meeting the AE Definition
• Any clinically significant abnormal laboratory findings or other abnormal safety assessments that are associated with the underlying disease, unless judged by the investigator to be more severe than expected for the participant's condition. • The disease/disorder being studied or expected progression, signs, or symptoms of the disease/disorder being studied, unless more severe than expected for the participant's condition. • Medical or surgical procedure (eg, endoscopy, appendectomy): the condition that leads to the procedure is the AE. • Situations in which an untoward medical occurrence did not occur (social and/or convenience admission to a hospital). • Anticipated day-to-day fluctuations of preexisting disease(s) or condition(s) present or detected at the start of the study that do not worsen.

10.3.2. Definition of an SAE

An SAE is defined as any untoward medical occurrence that, at any dose, meets one or more of the criteria listed below:
a. Results in death
b. Is life-threatening The term "life-threatening" in the definition of "serious" refers to an event in which the participant was at risk of death at the time of the event. It does not refer to an event that hypothetically might have caused death if it were more severe.
c. Requires inpatient hospitalization or prolongation of existing hospitalization In general, hospitalization signifies that the participant has been admitted (usually involving at least an overnight stay) at the hospital or emergency ward for observation and/or treatment that would not have been appropriate in the physician's office or outpatient setting. Complications that occur during hospitalization are AEs. If a complication prolongs hospitalization or fulfills any other serious criteria, the event is serious. When in doubt as to whether "hospitalization" occurred or was necessary, the AE should be considered serious. Hospitalization for elective treatment of a preexisting condition that did not worsen from baseline is not considered an AE.

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d. Results in persistent or significant disability/incapacity • The term disability means a substantial disruption of a person's ability to conduct normal life functions. • This definition is not intended to include experiences of relatively minor medical significance, such as uncomplicated headache, nausea, vomiting, diarrhea, influenza, and accidental trauma (eg, sprained ankle), that may interfere with or prevent everyday life functions but do not constitute a substantial disruption.
e. Is a congenital anomaly/birth defect
f. Is a suspected transmission via a Pfizer product of an infectious agent, pathogenic or nonpathogenic The event may be suspected from clinical symptoms or laboratory findings indicating an infection in a participant exposed to a Pfizer product. The terms "suspected transmission" and "transmission" are considered synonymous. These cases are considered unexpected and handled as serious expedited cases by pharmacovigilance personnel. Such cases are also considered for reporting as product defects, if appropriate.
g. Other situations: • Medical or scientific judgment should be exercised by the investigator in deciding whether SAE reporting is appropriate in other situations, such as significant medical events that may jeopardize the participant or may require medical or surgical intervention to prevent one of the other outcomes listed in the above definition. These events should usually be considered serious. • Examples of such events include invasive or malignant cancers, intensive treatment in an emergency room or at home for allergic bronchospasm, blood dyscrasias or convulsions that do not result in hospitalization, or development of drug dependency or drug abuse.

10.3.3. Recording/Reporting and Follow-Up of AEs and/or SAEs During the Active Collection Period

AE and SAE Recording/Reporting
The table below summarizes the requirements for recording AEs on the CRF and for reporting SAEs to Pfizer Safety throughout the active collection period. These requirements are delineated for 3 types of events: (1) SAEs; (2) nonserious AEs; and (3)

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exposure to the study intervention under study during pregnancy or breastfeeding, and occupational exposure.

It should be noted that reporting SAE information to Pfizer Safety is not the same as recording on the AE page of the CRF. When the same data are collected, these 2 reporting methods must be completed in a consistent manner. AEs should be recorded using concise medical terminology and the same AE term should be used on both the CRF and the reporting method used for reporting of SAE information to Pfizer Safety.

Safety Event	**Recorded on the CRF**	**Reported to Pfizer Safety**	**Reported Within 24 Hours of Awareness**
SAE	All	All	Yes
Nonserious AE	All	None	No
Exposure to the study intervention under study during pregnancy or breastfeeding	All AEs or SAEs associated with EDP or EDB Note: Instances of EDP or EDB not associated with an AE or SAE are not captured in the CRF	All instances of EDP are reported (whether or not there is an associated SAE) All instances of EDB are reported (whether or not there is an associated SAE)	Yes
Environmental or occupational exposure to the product under study to a nonparticipant (not involving EDP or EDB)	None. Exposure to a study nonparticipant is not collected on the CRF	The exposure (whether or not there is an associated AE or SAE) must be reported	Yes

- When an AE or SAE occurs, it is the responsibility of the investigator to review all documentation (eg, hospital progress notes, laboratory reports, and diagnostic reports) related to the event.
- The investigator will then record all relevant AE or SAE information in the CRF.

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- It is **not** acceptable for the investigator to send photocopies of the participant's medical records to Pfizer Safety when the reporting method is paper (eg, in lieu of completion of the CT SAE Report Form/AE or SAE CRF page).

- There may be instances when copies of medical records for certain cases are requested by Pfizer Safety. In this case, all participant identifiers, with the exception of the participant number, will be redacted on the copies of the medical records before submission to Pfizer Safety. Refer to Section 10.1.9 for actions that must be taken when medical records are sent to the sponsor or sponsor designee.

- The investigator will attempt to establish a diagnosis of the event based on signs, symptoms, and/or other clinical information. Whenever possible, the diagnosis (not the individual signs/symptoms) will be documented as the AE or SAE.

Assessment of Intensity

The investigator will make an assessment of intensity for each AE and SAE reported during the study and assign it to 1 of the following categories:

- Mild: A type of AE that is usually transient and may require only minimal treatment or therapeutic intervention. The event does not generally interfere with usual ADL.

- Moderate: A type of AE that is usually alleviated with additional specific therapeutic intervention. The event interferes with usual ADL, causing discomfort, but poses no significant or permanent risk of harm to the research participant.

- Severe: A type of AE that interrupts usual ADL, or significantly affects clinical status, or may require intensive therapeutic intervention.

An event is defined as "serious" when it meets at least 1 of the predefined outcomes as described in the definition of an SAE, NOT when it is rated as severe.

Assessment of Causality

- The investigator is obligated to assess the relationship between study intervention and each occurrence of each AE or SAE. The investigator will use clinical judgment to determine the relationship.

- A "reasonable possibility" of a relationship conveys that there are facts, evidence, and/or arguments to suggest a causal relationship, rather than a relationship cannot be ruled out.

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- Once the causality assessment is determined, the investigator must document it in the CRF and, for safety reporting purposes, document the relatedness as "Yes" (for related) or "No" (for unrelated).
- Alternative causes, such as underlying disease(s), concomitant therapy, and other risk factors, as well as the temporal relationship of the event to study intervention administration, will be considered and investigated.
- The investigator will also consult the IB and/or product information, for marketed products, in their assessment.
- For each AE or SAE, the investigator **must** document in the medical notes that they have reviewed the AE or SAE and have provided an assessment of causality.
- There may be situations in which an SAE has occurred, and the investigator has minimal information to include in the initial report to the sponsor. However, **it is very important that the investigator always make an assessment of causality for every event before the initial transmission of the SAE data to the sponsor.**
- The investigator may change their opinion of causality in light of follow-up information and send an SAE follow-up report with the updated causality assessment.
- The causality assessment is one of the criteria used when determining regulatory reporting requirements.
- If the investigator does not know whether or not the study intervention caused the event, then the event will be handled as "related to study intervention" for reporting purposes, as defined by the sponsor. In addition, if the investigator determines that an SAE is associated with study procedures, the investigator must record this causal relationship in the source documents and CRF, and report such an assessment in the dedicated section of the selected reporting method and in accordance with the SAE reporting requirements.

Outcome Assessment

Record the appropriate outcome of the event in relation to the participant's status:

- Fatal: The termination of life as a result of an AE.
- Not Recovered/Not Resolved: One of the possible results of an AE outcome that indicates that the event has not improved or recuperated.
- Recovered/Resolved: One of the possible results of an AE outcome that indicates that the event has improved or recuperated.

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- Recovered/Resolved with Sequalae: One of the possible results of an AE outcome where the participant recuperated but retained pathological conditions resulting from the prior disease or injury.
- Recovering/Resolving: One of the possible results of an AE outcome that indicates that the event is improving.
- Unknown: Not known, not observed, not recorded, or refused.

Follow-Up of AEs and SAEs

- The investigator is obligated to perform or arrange for the conduct of supplemental measurements and/or evaluations, as medically indicated or as requested by the sponsor, to elucidate the nature and/or causality of the AE or SAE as fully as possible. This may include additional laboratory tests or investigations, histopathological examinations, or consultation with other healthcare providers.
- If a participant dies during participation in the study or during a recognized follow-up period, the investigator will provide Pfizer Safety with a copy of any postmortem findings, including histopathology.
- New or updated information will be recorded in the originally submitted documents.
- The investigator will submit any updated SAE data to the sponsor within 24 hours of receipt of the information.

10.3.4. Reporting of SAEs.

SAE Reporting to Pfizer Safety via an Electronic DCT

- The primary mechanism for reporting an SAE to Pfizer Safety will be the electronic DCT (eg, eSAE or PSSA).
- If the electronic system is unavailable, then the site will use the paper SAE report form (see next section) to report the event within 24 hours.
- The site will enter the SAE data into the electronic DCT (eg, eSAE or PSSA) or paper form (as applicable) as soon as the data become available.
- After the study is completed at a given site, the electronic DCT will be taken off-line to prevent the entry of new data or changes to existing data.
- If a site receives a report of a new SAE from a study participant or receives updated data on a previously reported SAE after the electronic DCT has been taken off-line,

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then the site can report this information on a paper SAE form (see next section) or to Pfizer Safety by telephone.

SAE Reporting to Pfizer Safety via the CT SAE Report Form
• Facsimile transmission of the CT SAE Report Form is the back-up method to transmit this information to Pfizer Safety in case PSSA is unavailable for more than 24 hours. • In circumstances when the facsimile is not working, an alternative method should be used, eg, secured (Transport Layer Security) or password-protected email. If none of these methods can be used, notification by telephone is acceptable with a copy of the CT SAE Report Form sent by overnight mail or courier service. • Initial notification via telephone does not replace the need for the investigator to complete and sign the CT SAE Report Form pages within the designated reporting time frames.

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10.4. Appendix 4: Contraceptive and Barrier Guidance

10.4.1. Male Participant Reproductive Inclusion Criteria

No contraception methods are required for male participants in this study, as the calculated safety margin is ≥100-fold between the estimated maternal exposure due to seminal transfer and the NOAEL for serious manifestations of developmental toxicity in nonclinical studies.

10.4.2. Female Participant Reproductive Inclusion Criteria

The criteria below are part of Inclusion Criterion 1 (Age and Sex; Section 5.1) and specify the reproductive requirements for including female participants. Refer to Section 10.4.4 for a complete list of contraceptive methods permitted in the study.

A female participant is eligible to participate if she is not pregnant or breastfeeding; and at least 1 of the following conditions applies:

- Is not a WOCBP (see definition in Section 10.4.3).

  OR

- Is a WOCBP and agrees to use a highly effective contraceptive method (failure rate of <1% per year) during the intervention period and for at least 28 days after the last dose of study intervention, which corresponds to the time needed to eliminate any reproductive safety risk of the study intervention(s). If a highly effective, user-dependent method is chosen, she agrees to concurrently use an effective barrier method of contraception. The investigator should evaluate the effectiveness of the contraceptive method in relationship to the first dose of study intervention.

The investigator is responsible for review of medical history, menstrual history, and recent sexual activity to decrease the risk for inclusion of a woman with an early undetected pregnancy.

10.4.3. Woman of Childbearing Potential

A woman is considered fertile following menarche and until becoming postmenopausal unless permanently sterile (see below).

If fertility is unclear (eg, amenorrhea or oligomenorrhea) and a menstrual cycle cannot be confirmed before the first dose of study intervention, additional evaluation should be considered.

Women in the following categories are not considered WOCBP:

1. Premenarchal

2. Premenopausal female with 1 of the following:

   - Documented hysterectomy;

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- Documented bilateral salpingectomy;
- Documented bilateral oophorectomy.

For individuals with permanent infertility due to a medical cause other than the above (eg, mullerian agenesis, androgen insensitivity), investigator discretion should be applied to determining study entry.

Note: Documentation for any of the above categories can come from the site personnel's review of the participant's medical records, medical examination, or medical history interview. The method of documentation should be recorded in the participant's medical record for the study.

10.4.4. Contraception Methods

Contraceptive use by women should be consistent with local availability/regulations regarding the use of contraceptive methods for those participating in clinical trials.

The following contraceptive methods are appropriate for this study:

<u>Highly Effective Methods That Have Low User Dependency</u>

1. Implantable progestogen-only hormone contraception associated with inhibition of ovulation.
2. Intrauterine device.
3. Intrauterine hormone-releasing system.
4. Bilateral tubal occlusion or bilateral tubal ligation.

<u>Highly Effective Methods That Are User Dependent</u>

5. Combined (estrogen- and progestogen-containing) hormonal contraception associated with inhibition of ovulation, used in combination with a barrier method:
   - Oral + barrier*
   - Intravaginal + barrier*
   - Transdermal + barrier*
6. Progestogen only hormone contraception associated with inhibition of ovulation used in combination with a barrier method:
   - Oral + barrier*
   - Injectable + barrier*

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* Acceptable barrier methods to be used concomitantly with options 5 or 6 for the study include any of the following:

- Male or female condom with or without spermicide;
- Cervical cap, diaphragm, or sponge with spermicide;
- Male condom and female condoms should not be used together (due to risk of failure with friction).

7. Sexual abstinence

- Sexual abstinence is considered a highly effective method only if defined as refraining from heterosexual intercourse during the entire period of risk associated with the study intervention. The reliability of sexual abstinence needs to be evaluated in relation to the duration of the study and the preferred and usual lifestyle of the participant.

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10.5. Appendix 5: Genetics

Use/Analysis of DNA

- Genetic variation may impact a participant's response to study intervention, susceptibility to, and severity and progression of disease. Therefore, where local regulations and IRBs/ECs allow, a blood sample will be collected for DNA analysis.
- The scope of the genetic research may be narrow (eg, 1 or more candidate genes) or broad (eg, the entire genome), as appropriate to the scientific question under investigation.
- The samples may be analyzed as part of a multistudy assessment of genetic factors involved in the response to ritlecitinib or study interventions of this class to understand treatments for the disease(s) under study or the disease(s) themselves.
- The results of genetic analyses may be reported in the CSR or in a separate study summary, or may be used for internal decision making without being included in a study report.
- The sponsor will store the DNA samples in a secure storage space with adequate measures to protect confidentiality.
- The samples will be retained as indicated:
    - Retained samples will be stored indefinitely or for another period as per local requirements.
- Participants may withdraw their consent for the storage and/or use of their Retained Research Samples at any time by making a request to the investigator; in this case, any remaining material will be destroyed. Data already generated from the samples will be retained to protect the integrity of existing analyses.
- Samples for genetic research will be labeled with a code. The key between the code and the participant's personally identifying information (eg, name, address) will be held securely at the study site.

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10.6. Appendix 6: Liver Safety: Suggested Actions and Follow-Up Assessments

Potential Cases of Drug-Induced Liver Injury

Humans exposed to a drug who show no sign of liver injury (as determined by elevations in transaminases) are termed "tolerators," while those who show transient liver injury but adapt are termed "adaptors." In some participants, transaminase elevations are a harbinger of a more serious potential outcome. These participants fail to adapt and therefore are "susceptible" to progressive and serious liver injury, commonly referred to as DILI. Participants who experience a transaminase elevation above 3 × ULN should be monitored more frequently to determine if they are "adaptors" or are "susceptible."

LFTs are not required as a routine safety monitoring procedure in this study. However, should an investigator deem it necessary to assess LFTs because a participant presents with clinical signs/symptoms, such LFT results should be managed and followed as described below.

In the majority of DILI cases, elevations in AST and/or ALT precede T bili elevations (>2 × ULN) by several days or weeks. The increase in T bili typically occurs while AST/ALT is/are still elevated above 3 × ULN (ie, AST/ALT and T bili values will be elevated within the same laboratory sample). In rare instances, by the time T bili elevations are detected, AST/ALT values might have decreased. This occurrence is still regarded as a potential DILI. Therefore, abnormal elevations in either AST OR ALT in addition to T bili that meet the criteria outlined below are considered potential DILI (assessed per Hy's law criteria) cases and should always be considered important medical events, even before all other possible causes of liver injury have been excluded.

The threshold of laboratory abnormalities for a potential DILI case depends on the participant's individual baseline values and underlying conditions. Participants who present with the following laboratory abnormalities should be evaluated further as potential DILI (Hy's law) cases to definitively determine the etiology of the abnormal laboratory values:

- Participants with AST/ALT and T bili baseline values within the normal range who subsequently present with AST OR ALT values ≥3 × ULN AND a T bili value ≥2 × ULN with no evidence of hemolysis and an alkaline phosphatase value <2 × ULN or not available.

- For participants with baseline AST **OR** ALT **OR** T bili values above the ULN, the following threshold values are used in the definition mentioned above, as needed, depending on which values are above the ULN at baseline:

  - Preexisting AST or ALT baseline values above the normal range: AST or ALT values ≥2 times the baseline values AND ≥3 × ULN; or ≥8 × ULN (whichever is smaller).

  - Preexisting values of T bili above the normal range: T bili level increased from baseline value by an amount of ≥1 × ULN **or** if the value reaches ≥3 × ULN (whichever is smaller).

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Rises in AST/ALT and T bili separated by more than a few weeks should be assessed individually based on clinical judgment; any case where uncertainty remains as to whether it represents a potential Hy's law case should be reviewed with the sponsor.

The participant should return to the investigator site and be evaluated as soon as possible, preferably within 48 hours from awareness of the abnormal results. This evaluation should include laboratory tests, detailed history, and physical assessment.

In addition to repeating measurements of AST and ALT and T bili for suspected Hy's law cases, additional laboratory tests should include albumin, CK, direct and indirect bilirubin, GGT, PT/INR, eosinophils (%), and alkaline phosphatase. Consideration should also be given to drawing a separate tube of clotted blood and an anticoagulated tube of blood for further testing, as needed, for further contemporaneous analyses at the time of the recognized initial abnormalities to determine etiology. A detailed history, including relevant information, such as review of ethanol, acetaminophen/paracetamol (either by itself or as a coformulated product in prescription or over-the-counter medications), recreational drug, or supplement (herbal) use and consumption, family history, sexual history, travel history, history of contact with a jaundiced person, surgery, blood transfusion, history of liver or allergic disease, and potential occupational exposure to chemicals, should be collected. Further testing for acute hepatitis A, B, C, D, and E infection, total bile acids, liver imaging (eg, biliary tract), and collection of serum samples for acetaminophen/paracetamol drug and/or protein adduct levels may be warranted.

All cases demonstrated on repeat testing as meeting the laboratory criteria of AST/ALT and T bili elevation defined above should be considered potential DILI (Hy's law) cases if no other reason for the LFT abnormalities has yet been found. **Such potential DILI (Hy's law) cases are to be reported as SAEs, irrespective of availability of all the results of the investigations performed to determine etiology of the LFT abnormalities.**

A potential DILI (Hy's law) case becomes a confirmed case only after all results of reasonable investigations have been received and have excluded an alternative etiology.

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10.7. Appendix 7: Kidney Safety Monitoring Guidelines

10.7.1. Laboratory Assessment of Change in Kidney Function and Detection of Kidney Injury

Standard kidney safety monitoring requires assessment of baseline and postbaseline Screat measurement to estimate kidney function [Screat-based eGFR] or creatinine clearance [eCrCl]). Baseline and postbaseline Scys makes it feasible to distinguish AKI from other causes of Screat increase. If Screat increase is confirmed after baseline, then reflex measurement of Scys is indicated.

PEDIATRICS: Currently, no Screat plus Scys eGFR equations have been universally adopted for pediatrics. Therefore, comparison of baseline Screat and Scys to post-baseline Screat and reflex Scys are utilized to support differentiation of AKI from DICI.

Regardless of whether kidney function monitoring tests are required as a routine safety monitoring procedure in the study, if the investigator or sponsor deems it necessary to further assess kidney safety and quantify kidney function, then these test results should be managed and followed per standard of care.

10.7.2. Age-Specific Kidney Function Calculation Recommendations

10.7.2.1. Children (2 Years to <12 Years) — (Modified) Schwartz Equation

eCrCl normalized to BSA (mL/min/1.73 m^2)

eCrCl = (K × Ht) / Screat

Ht in cm; Screat in mg/dL.

K (proportionality constant): Child < 12 years: K = 0.55.

10.7.3. Adverse Event Grading for Kidney Safety Laboratory Abnormalities

AE grading for decline in kidney function (ie, eGFR or eCrCl) will be according to KDIGO criteria for pediatric participants.

KDIGO criteria grade (G)	**Study Population**	**G1**	**G2**	**G3**	**G4**	**G5**
Decreased Kidney Function due to either Acute or Chronic Kidney Injury	Pediatric participants eCrCl (mL/min)	≥90	≥60 to 89	30 to 59	15 to 29	<15 OR dialysis indicated

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KDIGO albuminuria (A) criteria	**A1**	**A2**	**A3**
Albumin-to-creatinine ratio (ACR)	<30 mg/g OR <3 mg/mmol	30 to 300 mg/g OR 3 to 30 mg/mmol	>300 mg/g OR >30 mg/mmol

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10.8. Appendix 8: ECG Findings of Potential Clinical Concern

ECG Findings That May Qualify as AEs
• Marked sinus bradycardia (rate <40 bpm) lasting minutes. • New PR interval prolongation >280 ms. • New prolongation of QTcF to >480 ms (absolute). • New prolongation of QTcF by >60 ms from baseline. • New-onset atrial flutter or fibrillation, with controlled ventricular response rate: ie, rate <120 bpm. • New-onset type I second-degree (Wenckebach) AV block of >30-second duration. • Frequent PVCs, triplets, or short intervals (<30 seconds) of consecutive ventricular complexes.
ECG Findings That May Qualify as SAEs

• QTcF prolongation >500 ms.
• Absolute value of QTcF >450 ms AND QTcF change from baseline >60 ms.
• New ST-T changes suggestive of myocardial ischemia.
• New-onset LBBB (QRS complex >120 ms).
• New-onset right bundle branch block (QRS complex >120 ms).
• Symptomatic bradycardia.
• Asystole
  • In awake, symptom-free participants in sinus rhythm, with documented asystolic pauses ≥3 seconds or any escape rate <40 bpm, or with an escape rhythm that is below the AV node;
  • In awake, symptom-free participants with atrial fibrillation and bradycardia with 1 or more asystolic pauses of at least 5 seconds or longer.
• Atrial flutter or fibrillation, with rapid ventricular response rate: rapid = rate >120 bpm.

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- Sustained supraventricular tachycardia (rate >120 bpm) ("sustained" = short duration with relevant symptoms or lasting >1 minute).
- Ventricular rhythms >30 second duration, including idioventricular rhythm (HR <40 bpm), accelerated idioventricular rhythm (HR 40 bpm to <100 bpm), and monomorphic/polymorphic ventricular tachycardia (HR >100 bpm [such as torsades de pointes]).
- Type II second-degree (Mobitz II) AV block.
- Complete (third-degree) heart block.

ECG Findings That Qualify as SAEs

- Change in pattern suggestive of new myocardial infarction.
- Sustained ventricular tachyarrhythmias (>30-second duration).
- Second- or third-degree AV block requiring pacemaker placement.
- Asystolic pauses requiring pacemaker placement.
- Atrial flutter or fibrillation with rapid ventricular response requiring cardioversion.
- Ventricular fibrillation/flutter.
- At the discretion of the investigator, any arrhythmia classified as an adverse experience.

The major events of potential clinical concern listed above are recommended as "alerts" or notifications from the core ECG laboratory to the investigator and Pfizer study team, and not to be considered as all-inclusive of what is to be reported as AEs/SAEs.

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10.9. Appendix 9: Prohibited Concomitant Medications That May Result in DDI

The prohibited concomitant medications listed below should not be taken with ritlecitinib for the period of time at least equal to, the required washout period listed in the table, and throughout the conduct of the study.

The Pfizer study team is to be notified of any prohibited medications taken during the study. After consulting with the sponsor, the investigator will make a judgment on the ongoing participation of any participant with prohibited medication use during the study.

This list of drugs prohibited for potential DDI concerns with the IMP may be revised during the course of the study with written notification from the sponsor to include or exclude specific drugs or drug categories for various reasons (eg, emerging DDI results for the IMP, availability of new information in literature on the DDI potential of other drugs) if the overall benefit/risk assessment is not impacted or if the changes do not significantly impact the safety of participants or the scientific value of the trial.

This is not an all-inclusive list. Site staff should consult with the sponsor or designee with any questions regarding potential DDI.

Study personnel should stay current and consult with their pharmacy for concomitant medications that are moderate to potent CYP3A inducers or sensitive or moderate sensitive CYP3A substrates or sensitive or moderate sensitive CYP1A2 substrates. If a medication is a sensitive or moderate sensitive CYP3A substrate or sensitive or moderate sensitive CYP1A2 substrate and is not listed in Table 5, Table 2 or Table 3 as prohibited or permitted, consultation with the sponsor is required.

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Table 5. Prohibited Concomitant CYP3A Inducers and Substrates

Sensitive CYP3A Substrates[a]	Moderate Sensitive CYP3A Substrates[b]	Moderate to Potent CYP3A Inducers[c]
Dasatinib	Aprepitant	Avasimibe[d]
Dronedarone	Eliglustat	Bosentan
Ebastine	Pimozide	Barbiturates
Lomitapide	Rilpivirine	Carbamazepine[d]
Nisoldipine		Efavirenz
Sirolimus		Etravirine
Tacrolimus		Mitotane[d]
Tolvaptan		Modafinil
		Nafcillin
		Phenobarbital[d]
		Phenytoin[d]
		Rifamycins (eg, Rifabutin, Rifampin, Rifapentine)[d]
		St. John's Wort[d]
		Talviraline

a. Sensitive CYP3A substrates are drugs that demonstrate an increase in AUC of ≥5-fold with strong index inhibitors of a given metabolic pathway in clinical DDI studies.
b. Moderate sensitive substrates are drugs that demonstrate an increase in AUC of ≥2 to <5-fold.
c. All prohibited drugs that are CYP3A inducers require at least a 28-day or 5-half-lives (whichever is longer) washout prior to the first dose of study intervention.
In a situation where appropriate medical care of a participant requires the use of a prohibited CYP3A inducer: Moderate to potent inducers of CYP3A are not permitted in the study EXCEPT in emergency situations. **Note:** Mitotane is not permitted for any duration due to its long half-life; if necessary mitotane may be used in an emergency, however the participant will then be discontinued from the study.
Topical (including skin or mucous membranes) application of antimicrobial and antifungal medications is permitted.
d. Notated as potent inducers.

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10.10. Appendix 10: Country-Specific Requirements

10.10.1. European Union

This study will be conducted in compliance with Regulation (EU) CTR No 536/2014. The recruitment plans for each EU Member State concerned are included in the respective Recruitment and Informed Consent Procedure documents.

The sponsor will notify EU Member States concerned of the following:

- Annual safety reports via submission to CTIS
- Any SUSAR via reporting to the EudraVigilance database no later than 7 days or 15 days for a fatal or life-threatening SUSAR or non-fatal or non-life-threatening SUSAR, respectively, after becoming aware of that event. A SUSAR is defined as a serious adverse reaction, the nature, severity, or outcome of which is not consistent with the reference safety information.
- Any unexpected event that affects the benefit risk-profile of the study, but are not SUSARs, no later than 15 days of becoming aware of that event
- Any serious breach, as described in Section 10.1.1.1, no later than 7 days of becoming aware of that breach
- Any urgent safety measure, as described in Section 10.1.1.1, no later than 7 days of the measure being taken
- Any inspection report of a third-country authority concerning the study

Records and documents, including signed ICDs, pertaining to the conduct of this study must be retained by the investigator for 25 years or longer if required by other European Union law.

For sites in EU member countries (Czech Republic, France, Italy, Poland, Spain): Participants should have a minimum body weight of ≥17 kg at the time of the screening visit.

10.10.2. France

Contrat Unique

1. GCP Training

   Before enrolling any participants, the investigator and any subinvestigators will complete the Pfizer-provided Good Clinical Practice training course ("Pfizer GCP Training") or training deemed equivalent by Pfizer. Any investigators who later join the study will do the same before performing study-related duties. For studies of applicable duration, the investigator and subinvestigators will complete Pfizer GCP Training or equivalent every 3 years during the term of the study, or more often if there are significant changes to the ICH GCP guidelines or course materials.

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2. Study Intervention

   No participants or third-party payers will be charged for study intervention.

3. Urgent Safety Measures

   In addition, the investigator will inform Pfizer immediately of any urgent safety measures taken by the investigator to protect the study participants against any immediate hazard, and of any serious breaches of this protocol or of ICH GCP that the investigator becomes aware of.

4. Termination Rights

   Pfizer retains the right to discontinue development of ritlecitinib at any time.

10.10.3. Japan

10.10.3.1. Japan-Specific Requirements for HBV Testing

At screening, HBsAg, HBcAb and HBsAb will be tested for participants in Japan:

a. If all 3 tests are negative, the participant is eligible for study inclusion.

b. If HBsAg is positive, the participant must be excluded from participation in the study.

c. If HBsAg is negative, HBcAb is positive and HBsAb is negative, the participant must be excluded from participation in the study.

d. If HBsAg is negative, HBcAb is negative and HBsAb is positive, and prior HBV vaccination is unequivocally documented, the participant is eligible for the study and does not require HBV DNA monitoring during the study.

e. If HBsAg is negative, HBcAb is negative and HBsAb is positive, and no unequivocal documentation of prior HBV vaccination is available, the participant is required to undergo HBV DNA reflex testing:

   i. If HBV DNA is detected, the participant must be excluded from participation in the study;

   ii. If HBV DNA is undetectable, the participant is eligible for study inclusion. If the participant is included in the study, for subsequent visits HBV DNA testing must be performed once every 3 months.

f. If HBsAg is negative, HBcAb is positive and HBsAb is positive, the participant is required to undergo HBV DNA reflex testing:

   i. If HBV DNA is detected, the participant must be excluded from participation in the study;

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ii. If HBV DNA is undetectable, the participant is eligible for study inclusion. If the participant is included in the study, for subsequent visits HBV DNA testing must be performed once every 3 months.

10.10.4. China

The total blood sampling volume for individual participants in this study in China is approximately 65 ml.

Retained research samples for genetics (Section 8.6.2) and retained research samples for biomarkers (Section 8.7.5) will not be collected for participants in China.

Participants with HBsAg positive: not eligible for study.

Participants with HBsAg negative and HBcAb positive, will take reflex test of HBsAb. Additional reflex testing for hepatitis B virus DNA testing (HBV DNA) is also required for HBcAb positive participants if:

A. HBsAb is negative or HBV DNA is detected: not eligible for study inclusion.

B. HBsAb is positive and HBV DNA is undetectable: eligible for study inclusion.

Based on the guideline for prevention and treatment of chronic hepatitis B (2022) in China:

- All patients receiving chemotherapy, targeted treatment or immunosuppressants should be screened for HBsAg, HBsAb and HBcAb before initiating such treatment.
- For those having positive HBsAg or (and) HBcAb, HBV DNA should be further tested. If HBV DNA is undetectable, HBV DNA may be monitored every 1 to 3 months.
- HBV DNA testing will be performed at least every 3 months for participants who were enrolled with a positive HBcAb and an undetectable HBV DNA.

HGRAC
In compliance with Human Genetic Resource Administration of China (HGRAC) regulations, below is the list of vendors in China that will conduct testing and analysis of human genetic resource samples or disposal of remaining samples.

Testing Name	Testing Analysis and Unit	Destruction Unit
QFT-G	PPD Laboratories Suzhou Co., Ltd.	Zhangjiagang Huarui Hazardous Waste Disposal Center Co., Ltd.
Hematology	PPD Laboratories Suzhou Co., Ltd.	Zhangjiagang Huarui Hazardous Waste Disposal Center Co., Ltd.

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FACS TBNK	PPD Laboratories Suzhou Co., Ltd.	Zhangjiagang Huarui Hazardous Waste Disposal Center Co., Ltd.
Hematology and Potential DILI	PPD Laboratories Suzhou Co., Ltd.	Zhangjiagang Huarui Hazardous Waste Disposal Center Co., Ltd.

Electrocardiograms
Standard 12-lead ECGs will be collected at times specified in the SoA section of this protocol using an ECG system that automatically calculates the HR and measures PR, QT, QTcF, and QRS intervals. If the local ECG system does not automatically measure QTcF, the site will perform the QTcF calculation manually using a sponsor provided worksheet and enter the value into the CRF.

10.10.5. Czech Republic

10.10.5.1. Czech Republic-Specific Requirements for HBV-DNA Testing

- Hepatitis B testing will be performed per Exclusion Criterion #11.
- In addition, HBV-DNA testing will be performed as reflex testing for those participants who are HBsAg negative, HBsAb positive and HBcAb positive at screening.
- Participants with detectable HBV-DNA will be excluded from participating in the study.
- Participants with undetectable HBV-DNA are eligible for participation in the study.
- If the participant is included in the study, for subsequent visits, HBV-DNA testing must be performed according to the SoA.
- If HBV-DNA is positive at any time after the screening visit, the participant must be discontinued from study intervention and followed at all regularly scheduled visits through Week 28 (EOS).

10.10.5.2. Czech Republic-Specific Requirements for HCV-RNA Testing

- Hepatitis C testing will be performed per Exclusion Criterion #11.
- In addition, for participants who are HCVAb positive and have undetectable HCV-RNA at screening who are included in the study, HCV-RNA testing must be performed according to the SoA at subsequent visits.

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- If HCV-RNA is positive at any time after the screening visit, the participant must be discontinued from study intervention and followed at all regularly scheduled visits through Week 28 (EOS).

10.10.5.3. Czech Republic-Specific Requirements for TB at Screening

In addition to the criteria outlined in Section 5.2, Exclusion Criterion #11, the following participants must have written approval by a TB specialist or an infectious diseases specialist to initiate ritlecitinib (ie, the study intervention in this study). This documentation must be provided prior to randomization and starting study intervention in this study, with the approval stored in the site's source documents:

- Participants who have previously been adequately treated for latent or active TB infection.
- Participants currently being treated for latent TB infection.
- Participants initiating treatment for latent TB infection diagnosed during the screening period of this study.

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10.11. Appendix 11: C-SSRS

Suicidal Ideation and Behavior will be assessed at screening using the C-SSRS (Columbia Suicide Severity Rating Scale – pediatric version, baseline assessment) version for pediatric patients 6 to 11 years old.

There are 2 versions of the C-SSRS: "C-SSRS – Children's Baseline" and "C-SSRS - Children's Since Last Visit".

The person conducting the assessment must be either:

a. A Mental Health Professional or

b. A study staff member with appropriate training and certification (Completion of training curriculum as required by the Columbia Lighthouse Project)

For participants who had suicidal ideation associated with actual intent and a method or plan at any time in their lifetime (a "Yes" answer on items 4 or 5 of the C-SSRS), or a previous history of suicidal behaviors at any time in their lifetime (a "Yes" answer to any of the suicidal behavior items of the C-SSRS) or any lifetime history of serious or recurrent suicidal behavior, a risk assessment must be performed and documented by a qualified mental health professional to assess whether it is safe for the participant to participate in the trial. A summary of risk assessment should be documented in the participant's source document/medical record.

If a participant presents with suicidal ideation/behavior at any time during the study, the participant's parent(s) or legal guardian should be informed.

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10.12. Appendix 12: Acceptability and Palatability Questionnaire

Instructions:

Part A is to be completed by site personnel on Day 15 and Day 127 of the study. Part B of this questionnaire is to be completed by the participant with participant's parent/caregiver assistance if required, on Day 15 and Day 127 after taking the dose of study medication at the clinic.

Questions should be answered shortly after dose administration (within approximately 30 minutes).

Part A (to be completed by site personnel):

Participant Study ID	
Questionnaire completed by	☐ Participant ☐ Parent/Caregiver
Dose	One PF-06651600-15 Size 3 capsule and one PF-06651600-15 Size 4 capsule per day

Part B (to be completed by the participant or participant's parent/caregiver):

1. Were you able to complete administration of the medication as instructed?

   ☐ Yes

   ☐ No

2. How did you take the medicine today?

   ☐ Swallowing capsules whole (answer questions 3, 4 and 7)

   ☐ Opening the capsules and sprinkling the contents on to food (answer questions 5, 6 and 7)

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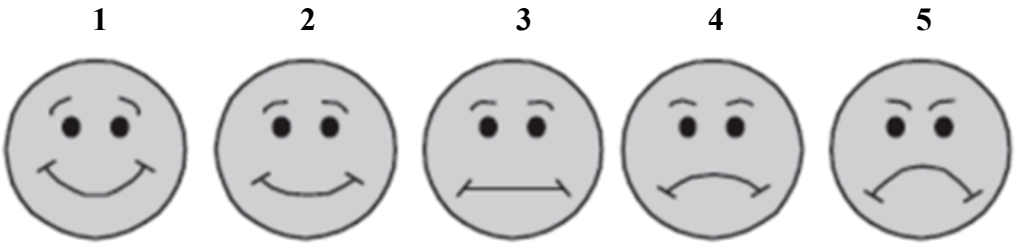
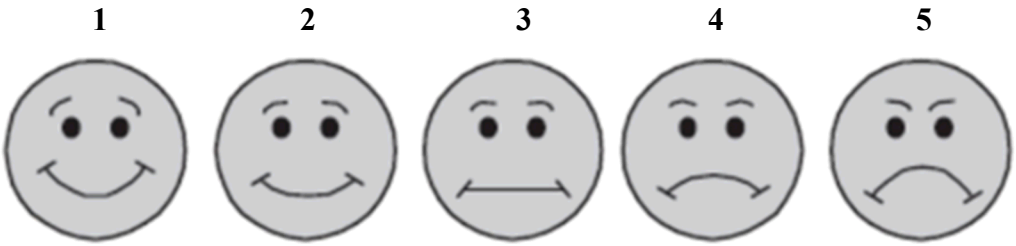
3. How easy is it to swallow the smaller capsule? Please circle one:



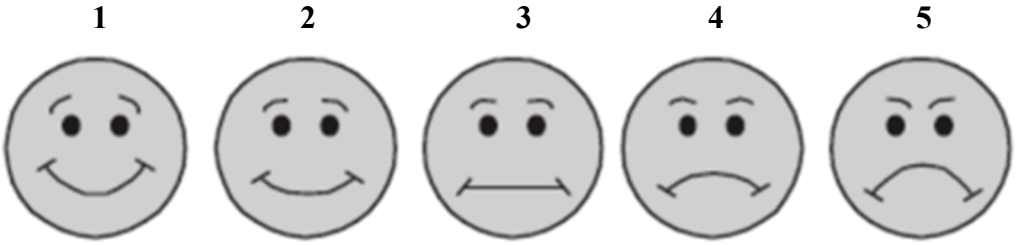
4. How easy is it to swallow the larger capsule? Please circle one:



5. What food did you sprinkle the medicine on today?

   ☐ Apple Sauce
   ☐ Yogurt
   ☐ Strawberry Jam

6. What did you think of the taste of the food and medicine preparation? Please circle one:



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7. How would you rate your overall experience taking the medication today? Please circle one:

1 **2** **3** **4** **5**

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10.13. Appendix 13: Tanner Stages of Development [44], [45], [46]

Classification of Sexual Maturity Stages in Girls [44], [45]:

For females, circle one sexual maturity stage for each domain (ie, pubic hair and breasts):

Sexual Maturity Stage	Pubic Hair	Sexual Maturity Stage	Breasts
1	Preadolescent	1	Preadolescent
2	Sparse, lightly pigmented, straight, medial border of labia	2	Breast and papilla elevated as small mound; areolar diameter increased
3	Darker, beginning to curl, increased amount	3	Breast and areola enlarged, no contour separation
4	Coarse, curly, abundant but amount less than adult	4	Areola and papilla form secondary mound
5	Adult feminine triangle, spread to medial surface of thighs	5	Mature, nipple projects, areola part of general breast contour

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Classification of Sexual Maturity Stages in Boys [45], [46]:

For males, circle one sexual maturity stage for each domain (ie, pubic hair and genitalia):

Sexual Maturity Stage	Pubic Hair	Sexual Maturity Stage	Genitalia
1	None	1	Preadolescent
2	Scanty, long, slightly pigmented	2	Slight enlargement in penis Enlarged scrotum, pink, texture altered
3	Darker, starts to curl, small amount	3	Longer penis and larger scrotum
4	Resembles adult type but less in quantity; coarse, curly	4	Larger penis; glans and breadth increase in size Larger and dark scrotum
5	Adult distribution, spread to medial surface of thighs	5	Adult size

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10.14. Appendix 14: Pediatric Anthropomorphic Measurements and Vital Signs

Normal Pediatric Height/Weight

At minimum, a baseline height and weight must be collected for all pediatric participants. Consider the growth and development of the study population, and that more frequent repeat height and weight measurements may be needed for infants and small children. These measurements may be important for dosing or future assessment of pediatric parameters, such as blood pressure.

For US-based studies, CDC recommends that health care providers:

- Use the WHO growth standards (https://www.cdc.gov/growthcharts/who_charts.htm) to monitor growth for infants and children ages 0 to 2 years of age in the US.
- Use the CDC growth charts (https://www.cdc.gov/growthcharts/cdc_charts.htm) for children age 2 years and older in the U.S.

PALS Vital Sign Parameters

https://www.ncbi.nlm.nih.gov/books/NBK553213/#!po=83.3333 (Tables 2 and 7)

https://www.acls-pals-bls.com/algorithms/pals/

Normal Pediatric Temperature

Temperature measurement in Pediatrics/Canada (Table 1):
https://www.ncbi.nlm.nih.gov/pmc/articles/PMC2819918/#!po=50.0000

Normal Pediatric Pulse

American Academy of Pediatrics (table of heart rate for various age groups while awake and asleep):

https://www.healthychildren.org/English/health-issues/conditions/heart/Pages/Irregular-Heartbeat-Arrhythmia.aspx

Normal Pediatric Blood Pressure by Age and Height for Girls and Boys (Page 3 and 4)

NIH: A Pocket Guide to Blood Pressure Measurement in Children

https://www.nhlbi.nih.gov/files/docs/bp_child_pocket.pdf

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10.15. Appendix 15: Blood Sampling in Pediatric Participants

Best Practice for Pediatric Blood Collection

Examples of best practices for pediatric blood collection which should be included in lab manuals and training materials include:

- Ensure that the child is well-hydrated prior to blood collection.
- Keep the child warm throughout the process of preparing for and drawing blood.
- Engage caregivers to determine what is the most appropriate way to soothe their child.
- Use a lidocaine-based topical anesthetic (eg, lidocaine 4% cream) as needed prior to blood collection to decrease potential discomfort to the participant. The anesthetic must not contain PG. The skin must be thoroughly cleansed prior to blood sample collection. It is recommended that 2 venipuncture sites be prepared with local anesthetic in case the first attempt is not successful.
- Adhere strictly to a limit on the number of times a pediatric patient may be stuck. If no satisfactory sample has been collected after 2 attempts, seek a second opinion to decide whether to make a further attempt or cancel the tests.

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10.16. Appendix 16: Protocol Amendment History

Amendment 1 (03 September 2024)

Overall Rationale for the Amendment: The purpose of this amendment was to align the protocol with the comments received from the FDA. Substantial and non-substantial protocol modifications have been implemented to i) incorporate recommendations from the FDA's review of the B7981027 study protocol, ii) provide more clarity to study investigators, iii) comply with the CTR protocol requirements, and iv) make minor clarifications.

This Summary of Changes table incorporates all the revisions to the "Final Protocol" version (dated 18 June 2024) and implemented with the "Protocol Amendment 1" version, dated 03 September 2024.

The substantial and non-substantial modifications are listed below.

Description of Change	Brief Rationale	Section # and Name
Substantial Modifications		
Updated the primary endpoint (for US and countries following US analysis plan) to "**Response based on achieving an absolute SALT score ≤20 at Week 24.**" Updated the primary estimand. Removed the key secondary endpoint for US and countries following US analysis plan.	To align with FDA's recommendation to use SALT ≤20 response at Week 24 as the primary endpoint.	Section 1.1: Synopsis – Objectives & Endpoints Section 3: Objectives and Endpoints
Added the following text: Participants who are withdrawn early from study intervention for any reason will remain in the study and continue the regularly scheduled visits through Week 24 and follow-up visit.	To align with FDA's recommendation to continue all participants in the study through the end of the treatment period for safety and efficacy assessments.	Section 1.1: Synopsis-Overall Design Section 1.3: Schedule of Activities (SoA) - Footnote Section 4.1: Overall Design Section 7.1: Discontinuation of Study Intervention
Updated the study sample size to 225, with 75 participants in each treatment arm.	To account for uncertainty regarding response rates due to the paucity of	Section 1.1. Synopsis, Section 4.1. Overall Design, Section 9.5.

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Description of Change	Brief Rationale	Section # and Name
	clinical data in the age group of 6-11 years.	Sample Size Determination
Non-substantial Modifications		
Specified the timepoints for audiological examination in the study: **screening and Week 24**/Month 6.	To provide more clarity to study investigators on the timepoints when audiologic examination is required to be performed during the study.	Section 1.1: Synopsis – Overall Design Section 4.1: Overall Design
Updated the exclusion criterion based on the CDRS-R to T-score ≥55.	To align with the clinical interpretation of the CDRS-R T-Score, with the T-score of ≥55 suggesting the possible presence of a depressive disorder.	Section 5.2: Exclusion criterion #6
Added the text in bold: If contact cannot be made with the parent/legal guardian/caregiver to review dosing instructions **then the participant will be discontinued from the study intervention.** If it is found that there is lack of involvement in managing study intervention for the participant, then the participant will be discontinued from the study intervention **but will remain in the study and continue the regularly scheduled visits through Week 24 and follow-up visit.**	To align with FDA's recommendation to continue all participants in the study through the end of the treatment period for safety and efficacy assessments.	Section 6.1.1: Administration
Added PK data as sensitive clinical data.	To provide more clarity to investigators on what is considered as sensitive clinical data in B7981027 study.	Section 6.4.4: Sensitive Clinical Data
Added the following text: For participants who discontinue study intervention prior to Week 24 and continue in the study for	To provide clarity for investigators on use of prohibited medications for participants who	Section 6.9.2.1: Prohibited Concomitant

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Description of Change	Brief Rationale	Section # and Name
efficacy and safety assessments, the use of prohibited medications mentioned in Section 6.9.2.1 is allowed 28 days after the last dose of study intervention.	discontinue study intervention but will continue the regularly scheduled visits through Week 24 and follow-up visit.	Medications/Treatments
Removed the following reasons for discontinuation from the study. • AE • Lack of efficacy • Non-compliance with study drug • Pregnancy • Protocol deviation • Medication error with associated AE • No longer meets eligibility criteria • Any other condition requiring permanent discontinuation from the study • Suicidal Ideation.	To align with FDA's recommendation to continue all participants in the study through the end of the treatment period for safety and efficacy assessments.	Section 7.2: Participant Discontinuation/Withdrawal from the Study
Added the text on minimizing missing data in the study.	To align with FDA's recommendation to describe how missing data will be prevented or minimized in the study.	Section 8: STUDY ASSESSMENTS AND PROCEDURES
The PROMIS Parent Proxy Short Form v2.0 – Depressive Symptoms 6a: Updated the text to state correctly that the highest possible raw score is 3**0**.	Typographical error corrected	Section 8.2.3.3: PROMIS Parent Proxy - Anxiety Short Form 8a and Depressive Symptoms Short Form 6a
Updated the SoA table and section on ECGs to include the visit timepoints.	To align with FDA's recommendation to evaluate ECGs at the screening visit and at a visit early in the study.	Section 1.3: SoA - Electrocardiogram Section 8.3.5: Electrocardiograms

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Description of Change	Brief Rationale	Section # and Name
Moved the following text on hypothesis testing for **US and Countries Following US Analysis Plan**, which was applicable to all countries, under **EU/UK and Countries Following EU/UK Analysis Plan** due to the change in the primary endpoint and the associated hypothesis testing for **US and Countries Following US Analysis Plan**: The familywise Type I error rate will be set to $\alpha = 0.05$ (2-sided). H1 will be tested at $\alpha = 0.05$ first and if significant, H2 and H3 will be tested simultaneously applying Holm's approach. If both H2 and H3 are significant, H4 will be tested at $\alpha = 0.05$. The graphical approach of Bretz et al (2009) for sequentially rejective testing procedures is used to illustrate the testing strategy.	Updated as a result of the FDA's recommendation to use SALT ≤20 response at Week 24 as the primary endpoint	Section 9.1: Statistical Hypotheses
Updated the primary estimand text for **US and Countries Following US Analysis Plan**	To align with FDA's recommendation to use SALT ≤20 response at Week 24 as the primary endpoint.	Section 9.1.1.1: Primary Estimand
Updated the definition of secondary estimand (composite estimand, Estimand 2 and treatment policy estimand, Estimand 3).	To align with FDA's recommendation to use the worst-case carried forward approach for continuous endpoints.	Section 9.1.1.2: Secondary Estimand
Corrected the lower dose of ritlecitinib from 20 mg to 25 mg.	Typographical error corrected	Section 9.3.1: General Considerations
Added the following text: For continuous endpoints, change from baseline will be analyzed by an ANCOVA model at each post-baseline visit with terms for baseline	To align with FDA's recommendation to update the analysis method for continuous endpoints.	Section 9.3.1: General Considerations

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Description of Change	Brief Rationale	Section # and Name
value (if applicable), treatment group, and stratification factor of AA severity (AT/AU and non-AT/AU). All missing data will be handled by WOCF.		
Updated the text to describe the analytical approach of the changed primary estimand (SALT ≤20 responses at Week 24) for US and Countries Following US Analysis Plan. Updated the text on subgroup analyses for SALT score ≤20 response and SALT score ≤10 response at Week 24.	To align with FDA's recommendation to use SALT ≤20 response at Week 24 as the primary endpoint and to specify subgroup analyses in the study.	Section 9.3.2: Primary Endpoint(s)/Estimand(s) Analysis
Updated the text on analysis of secondary endpoints/estimands and added text on handling of missing data.	To align with FDA's recommendation to describe the handling of missing data in the study.	Section 9.3.3: Secondary Endpoint(s)/Estimand(s) Analysis
Replaced the section with the text from the new protocol template.	Per requirement in the new protocol template dated 01 May 2024.	Section 10.1.12 Sponsor's Medically Qualified Individual
Included the mandatory text required for studies conducted in EU.	To comply with the EU CTR protocol requirements	Section 10.1.6. Dissemination of Clinical Study Data Section 10.10.1: European Union

Amendment 2 (06 January 2025)

Overall Rationale for the Amendment: Protocol modifications have been implemented to i) modify the doses of ritlecitinib in B7981027 study protocol, ii) incorporate updates from the latest version of Ritlecitinib Investigator's Brochure, iii) incorporate recommendations from the sponsor advisory committee on assessment of suicidal ideation and behavior in the pediatric population aged 6 to <12 years in B7981027 study, and iv) make minor edits.

Description of Change	Brief Rationale	Section # and Name
Substantial Modifications		

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Description of Change	Brief Rationale	Section # and Name
Modified the lower and higher doses of ritlecitinib from 25 mg and 45 mg to 30 mg and 50 mg, respectively.	To optimize the doses investigated in this study given the approved 50 mg dose in adults/adolescents, the 30 mg dose studied in B7981015, and the updated information from the population PK model-based additional simulations.	Throughout the protocol.
Non-substantial Modifications		
Updated the text on risk assessment.	To align with the latest version (V11, dated 14 Nov 2024) of Ritlecitinib Investigator's Brochure.	Section 2.3.1: Risk Assessment
Updated the text on assessment of suicidal ideation and behavior, where applicable in the protocol. Changed the screening C-SSRS lookback period from 5 years to lifetime. Added text: A summary of risk assessment should be documented in the participant's source document/medical record. Added text: If a participant presents with suicidal ideation/behavior at any time during the study, the participant's parent or legal guardian should be informed.	To align with internal sponsor advisory committee recommendations on assessment of suicidal ideation and behavior in pediatric population aged 6 to <12 years.	Section 5.2: Exclusion Criteria #6 Section 7.1.5: Suicidal Ideation Section 8.3.8: Suicidal Ideation and Behavior Risk Monitoring Section 10.11. Appendix 11: C-SSRS
Updated the text to correctly state that pregnancy screening test will be performed for all females ≥8 years of age	Typographical error corrected.	Section 1.3: Schedule of Activities Section 2.3.1: Risk Assessment (Potential fetal risk and potential risk if ritlecitinib is

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Description of Change	Brief Rationale	Section # and Name
		secreted into human milk.) Section 8.3.7: Pregnancy Testing
Added text on pregnancy testing for participants who are <8 years of age at screening and turn 8 years of age during the study.	To provide additional instructions on pregnancy testing.	Table 1: Study Schedule of Assessments (footnote) Section 8.3.7: Pregnancy Testing
Deleted the text "Participants may be rescreened after the infection resolves".	To avoid duplication of the text.	Section 5.2; Exclusion Criteria-#11 - Infections
Added psychologist to a list of individuals who can administer the WISC-V. Added text clarifying that if under exceptional circumstances a participant cannot be administered the WISC-V, the investigator can contact the sponsor to determine if the participant can be enrolled without performing the WISC-V.	To provide additional specifications to sites/investigators on the administration of WISC-V at Day 1/baseline visit.	Section 8.2.3.6: Wechsler Intelligence Scale for Children - 5th edition
Updated the Acceptability and Palatability Questionnaire: a) Added question #1 in the updated questionnaire. b) The repeated question (#4) in the previous version of the questionnaire has been deleted.	To include a metric that is indicative of acceptability of study intervention and to align with the requirements of internal data standards.	Section 10.13. Appendix 13: Acceptability and Palatability Questionnaire

Amendment 3 (10 April 2025)

Overall Rationale for the Amendment: Protocol modifications have been implemented to i) change the screening tool used to exclude participants with clinically significant depression from enrollment. ii) incorporate updates on changed urine myoglobin testing to urine dipstick testing for CK >10 × ULN. iii) make minor edits.

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Description of Change	Brief Rationale	Section # and Name
Non-substantial Modifications		
Exclusion criterion #6 has been updated to replace the CDRS-R with the PROMIS Parent Proxy Short Form - Depressive Symptom Scale, as well as removal of the CDRS-R where applicable.	Change made to be consistent with sponsor internal guidance on assessment of depression, using a validated parent rated screening tool.	Section 5.2: Exclusion criterion #6 Section 1.3: Schedule of Activities Section 8.2.1: Rater Qualifications Section 10.12 Appendix 12: CDRS-R
Updated urine myoglobin testing to urine dipstick testing for CK >10 × ULN.	An IVDR (In Vitro Diagnostics Regulation) compliant test for urine myoglobin is not available from the central laboratory in this study. Therefore, it is replaced by a urine dipstick test, which allows detection of myoglobinuria based on a positive dipstick test result for blood in the absence of both hematuria and hemoglobinuria.	Section 7.1.3: Laboratory Abnormalities Section 10.2 Appendix 2, Table 4
Included text on use of a dosing diary	To support existing methods to determine compliance and accountability of study intervention.	Section 6.1.1: Administration Section 6.5: Study Intervention Compliance
Updated the version numbers of PROMIS Parent Proxy Short Form – Anxiety 8a and PROMIS Parent Proxy Short Form – Depressive Symptoms 6a	To be consistent with sponsor internal guidance on use of latest versions.	Section 8.2.3.3: PROMIS Parent Proxy - Anxiety Short Form 8a and Depressive Symptoms Short Form 6a
Updated the text on BRIEF®2	BRIEF®2 is not specifically used in this study for the purpose of screening or monitoring. Accordingly: An incorrect statement was corrected.	Section 8.2.3.4: Behavior Rating Inventory of Executive Function®,

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Description of Change	Brief Rationale	Section # and Name
Non-substantial Modifications		
	An incorrect statement was removed.	Second Edition (BRIEF®2) Section 8.3.3.4: Neuropsychological and Cognitive Development
Updated the qualification of WISC-V rater	To align with the rater qualification recommended by the WISC-V publisher.	Section 8.2.3.6: Wechsler Intelligence Scale for Children - 5th edition
Incorporated protocol template updates.	To align with current Pfizer protocol template and requirements.	Section 8.4.1.1: Reporting Nonserious AEs and SAEs to Pfizer Safety Section 10.1.3: Informed Consent/Assent Process Section 10.1.11: Publication Policy Section 10.3.3: Recording/Reporting and Follow Up of AEs and/or SAEs During the Active Collection Period Section 10.10.1: European Union
Editorial changes	Correction of typographical errors	Throughout the protocol

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10.17. Appendix 17: Abbreviations

The following is a list of abbreviations that may be used in the protocol.

Abbreviation	Term
5-ARIs	5α-Reductase inhibitors
AA	alopecia areata
AAPPO	alopecia areata patient priority outcomes
A1 to A3	albuminuria (KDIGO albuminuria severity standardization)
Abs	absolute
ACR	albumin-to-creatinine ratio
ADL	activity/activities of daily living
AE	adverse event
AESI	adverse event of special interest
AKI	acute kidney injury
ALC	absolute lymphocyte count
ALT	alanine aminotransferase
ANC	absolute neutrophil count
ANCOVA	analysis of covariance
AOM	acute otitis media
AST	aspartate aminotransferase
AT	alopecia totalis
AU	alopecia universalis
AUC	area under the concentration-time curve
AUC_{24}	area under the concentration-time curve from time zero to 24 hours
AUC_{inf}	area under concentration-time curve to infinity
AV	atrioventricular
AxMP	auxiliary medicinal product
BAEP	brainstem auditory evoked potential
BCG	Bacille Calmette-Guérin
BiC	blend-in capsule
BMX	bone marrow tyrosine kinase on chromosome X
BP	blood pressure
bpm	beats per minute
BRI	Behavior Regulation Index
BRIEF®2	Behavior Rating Inventory of Executive Function®, Second Edition
BSA	body surface area
BTK	Bruton's Tyrosine Kinase
BUN	blood urea nitrogen
CD	Crohn's disease
CDISC	Clinical Data Interchange Standards Consortium
CDLQI	Children's Dermatology Life Quality Index
CDP	clinical development program

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Abbreviation	Term
CDRS	Childhood depression Rating Scale
CDRS-R	Childhood depression Rating Scale - Revised
CFB	change from baseline
CFR	Code of Federal Regulations
CI	confidence interval
CIOMS	Council for International Organizations of Medical Sciences
CK	creatine kinase
CL/F	apparent clearance
C_{max}	maximum observed concentration
CMH	Cochran-Mantel Haenszel
CMV	cytomegalovirus
CONSORT	Consolidated Standards of Reporting Trials
COVID-19	coronavirus disease 2019
CRF	case report form
CRI	Cognitive Regulation Index
CRO	contract research organization
C-SSRS	Columbia-Suicide Severity Rating Scale
CSR	Clinical Study Report
CT	computed tomography/clinical trial
CTIS	Clinical Trial Information System
CV	coefficient of variation
CVEAC	Cardiovascular Event Adjudication Committee
CYP	cytochrome P450
DCT	data collection tool
DDI	drug-drug interaction
DICI	drug-induced creatinine increase
DIKI	drug-induced kidney injury
DILI	drug-induced liver injury
DNCB	2,4-dinitrochlorobenzene
DPCP	diphenylcyclopropenone
DU	dispensable unit
EBA	eyebrow assessment
EBV	Epstein-Barr virus
EC	ethics committee
ECG	electrocardiogram or electrocardiography
eCrCl	estimated creatinine clearance
eCRF	electronic case report form
EDB	exposure during breastfeeding
E-DMC	External Data Monitoring Committee
EDP	exposure during pregnancy
eGFR	estimated glomerular filtration rate
ELA	eyelash assessment
EOT	end of treatment

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Abbreviation	Term
EOS	end of study
ERI	Emotion Regulation Index
eSAE	electronic serious adverse event
EU	European Union
EudraCT	European Union Drug Regulating Authorities Clinical Trials (European Clinical Trials Database)
FACS	Fluorescence-activated cell sorting
FACS TNBK	Fluorescence-activated cell sorting lymphocyte subsets T, B, and NK
FAS	full analysis set
FDA	Food and Drug Administration
FU	follow-up
G1 to G5	Grade (KDIGO eGFR category standardization)
GCP	Good Clinical Practice
GGT	gamma-glutamyl transferase
HBcAb	hepatitis B core antibody
HBsAb	hepatitis B surface antibody
HBsAg	hepatitis B surface antigen
HBV	hepatitis B virus
HBV DNA	hepatitis B virus DNA
HCV	hepatitis C virus
HCVAb	hepatitis C antibody
HCV RNA	hepatitis C RNA
HDPE	high-density polyethylene
HGRAC	Human Genetic Resources Administration of China
Hep B	hepatitis B
HIPAA	Health Insurance Portability and Accountability Act
HIV	human immunodeficiency virus
HR	heart rate
Ht	height
IB	Investigator's Brochure
ICD	informed consent document
ICH	International Council for Harmonisation of Technical Requirements for Pharmaceuticals for Human Use
ID	identification
IFN	interferon
IGRA	Interferon-Gamma Release Assays
IL	interleukin
IMP	investigational medicinal product
IND	Investigational New Drug
INR	international normalized ratio
IP	internet protocol
IPAL	Investigational Product Accountability Log

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Abbreviation	Term
IRB	Institutional Review Board
IRT	Interactive Response Technology
ITK	Interleukin-2 inducible T-cell kinase
JAK	janus kinase
JTC	jump-to-control
k	proportionality constant
KDIGO	Kidney Disease: Improving Global Outcomes
LBBB	left bundle branch block
LFT	liver function test
LSM	Least squares mean
LTE	long-term extension
MAC	Malignancy Adjudication Committee
MAR	missing at random
MMF	mycophenolate mofetil
MMR	measles, mumps, rubella
MQI	medically qualified individual
MRHD	maximum recommended human dose
MTX	methotrexate
NA	not applicable
NIMP	noninvestigational medicinal product
NK	natural killer
NMSC	non-melanoma skin cancer
NOAEL	no observed adverse effect level
NSEAC	Neuro safety Event Adjudication Committee
ObsRO	observer-reported outcomes
OIRC	Opportunistic Infection Review Committee
PD	pharmacodynamic(s)
PDCO	Pediatric Committee
PGI-C	patient's global impression of change
PI	principal investigator
PIP	Pediatric Investigation Plan
PK	pharmacokinetic(s)
PG	propylene glycol
PPD	purified protein derivative
PR	pulse rate
PRO	Patient Reported Outcome
PROMIS	Patient-Reported Outcomes Measurement Information System
PROMIS measures	PROMIS Parent Proxy Anxiety and Depressive Symptoms Short forms
PSSA	Pfizer's Serious AE Submission Assistant
PT	prothrombin time
PUVA	Psoralen Ultraviolet A
PVC	premature ventricular contraction

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Abbreviation	Term
QD	once daily
QFT-G	interferon alpha release assay
QoL	quality of life
QTc	corrected QT interval
QTcF	QTc corrected using Fridericia's formula
QTL	quality tolerance limit
RBC	red blood cell
RNA	ribonucleic acid
SADBE	squaric acid dibutylester
SAE	serious adverse event
SALT	Severity of Alopecia Tool
SAP	Statistical Analysis Plan
Screat	serum creatinine
Scys	serum cystatin C
SE	standard error
SoA	schedule of activities
SOP	standard operating procedure
SRSD	Single Reference Safety Document
SUSAR	Suspected Unexpected Serious Adverse Reaction
$t_{½}$	terminal half-life
TB	tuberculosis
TBV	total blood volume
T bili	total bilirubin
TEAE	treatment-emergent adverse event
TEC	tyrosine kinase expressed in hepatocellular carcinoma
TH1	type 1 helper T cell
T_{max}	time to reach C_{max}
TXK	tyrosine kinase expressed in T cells
TU	tuberculin unit
UK	United Kingdom
ULN	upper limit of normal
US	United States
UTI	urinary tract infection
UVB	Ultraviolet B
V_z/F	apparent volume of distribution after oral dose
VZV	varicella zoster virus
WBC	white blood cell
WHO	World Health Organization
WISC-V	Wechsler Intelligence Scale for Children® Fifth Edition
WOCBP	woman/women of childbearing potential
WOCF	worst observation carried forward

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